HHN-Henderson Hospital

1050 W. Galleria Drive
Henderson, NV 89011

(702) 963-7000

Patient Information

Patient Name: GARCIA, ARMANDO
Home Address: 6347 RUBYLYN AVE
LAS VEGAS, NV 891221111
Home Phone: 3236906321
Employer Name:
Employer Phone:

Sex: Male
DOB: 06/02/1972
Age: 52 Years
Religion: Roman Catholic
SSN: XXX-XX-5302
Race: Unknown/Other Race

Guarantor Information

Guarantor Name: GARCIA, ARMANDO
Patient's Reltn:
Billing Address: 6347 RUBYLYN AVE
LAS VEGAS, NV 891221111
Billing Phone: 3236906321
Employer Name:
Employer Phone:

Sex: Male
DOB: 06/02/1972
Age: 52 Years
Religion: Roman Catholic
SSN: XXX-XX-5302
Marital Status: Married

Contact Information

Emergency Contact
Contact Name: VERONICA GARCIA
Patient's Reltn: Spouse
Sex:
Home Phone: 3238304406

Next of Kin
Contact Name: VERONICA GARCIA
Patient's Reltn: Spouse
Sex:
Home Phone: 3238304406

Primary Insurance

Subscriber Name: GARCIA, ARMANDO
Patient's Reltn:
Sex: Male
DOB: 06/02/1972
Age: 52 Years
Employer Name:
Employer Phone:
Financial Class: Medicare Acute Care
Group Name: NEVADA

Insurance Name: MEDICARE PART A INVISION
Claim Address: ATTN NOVITAS SOLUTIO
MECHANISCBU, PA 17055
Insurance Phone: 8006334227
Policy Number: 2EC2PY1UK70
Group Number:
Authorization Number:
Authorization Phone:
Authorization Contact:

Secondary Insurance

Subscriber Name:
Patient's Reltn:
Sex:
DOB:
Age:
Employer Name:
Employer Phone:
Financial Class: Medicare Managed Care
Group Name:

Insurance Name:
Claim Address:

Insurance Phone:
Policy Number:
Group Number:
Authorization Number:
Authorization Phone:
Authorization Contact:

Encounter Information

Reg Dt/Tm: 08/28/2024 01:14
Est Dt of Arrival: 08/30/2024 18:12
Inpt Adm Dt/Tm: 08/28/2024 01:14
Disch Dt/Tm:
Observation Dt/Tm:
VIP Indicator:
Admit Reason: RESPIRATORY FAILURE

Patient Type: Inpatient
Medical Service: IMC-Intermediate Ca
Location: HHN IMC
Room/Bed: 2027 / 01
Isolation:
Disease Alert:

Admit Type: Emergency
Admit Source: Emergency Room
Advance Directive:
Reg Clerk: Contributor_system, HHN_I
Admit Physician: Kabsoun MD, Sara H
Attend Physician: Kabsoun MD, Sara H
PCP: Pcp No

GARCIA, ARMANDO

MRN: HHN8210503

Male / 52 Years

FIN: HHN0005007238115

Printed By: USER, XR DEFAULT on 09/01/2024 14:00
Registration last updated by: Contributor_system, HHN_INVISION on 09/01/2024 00:40

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male

Admit: 8/28/2024
Disch: Disch Time:
FIN: HHN0005007238115
Readmit Risk: 63 - HIGH 08/30/24 09:45
Attending: Kabsoun MD,Sara H

Facesheets

DOCUMENT NAME:	Facesheets
SERVICE DATE/TIME:	8/27/2024 00:00 PDT
RESULT STATUS:	Unauth
PERFORM INFORMATION:	
SIGN INFORMATION:	

FACE SHEET REGISTRATION FORM_20240827.pdf
Please click on link to see image.

Attachment(s): 8/27/2024 00:00 PDT FACE SHEET REGISTRATION FORM_20240827.pdf

Henderson-Garcia, Armando-Enc #5007238115-IPT-ICU- FACE SHEET REGISTRATION FORM - 8/27/2024 - 1 pg

MED REC# **8210503**

PT#	PATIENT NAME
5007238115	**GARCIA, ARMANDO**

ADDRESS	COUNTY
6347 RUBYLYN AVE	001

CITY	STATE	ZIP	PHONE	PATIENT LANGUAGE
LAS VEGAS	NV	891221111	(323)690-6321	ENGLISH

AGE	D.O.B.	SEX	RACE	MS	ETHNICITY	PREVIOUS NAME	RELIGION	INFANT AGE
52Y	06/02/1972	M	3	M		,	CAT	

PT EMPLOYER	TELEPHONE	EXT	OCCUPATION
DISABLED,			DISABLED

EMPLOYER ADDRESS				
CITY	STATE	ZIP	RETIREMENT DATE	DISASTER TAG ID
			02/24/00	

Contact Information

NEAREST RELATIVE NAME				
GARCIA, VERONICA				
RLTN	ADDRESS			
U	6347 RUBYLYN AVE			
CITY	STATE	ZIP	PHONE	EXT.
LAS VEGAS	NV	891221111	(323)830-4406	

EMERGENCY CONTACT NAME				
GARCIA, VERONICA				
RLTN	ADDRESS			
U	6347 RUBYLYN AVE			
CITY	STATE	ZIP	PHONE	EXT.
LAS VEGAS	NV	891221111	(323)501-5123	

Guarantor Information

GUARANTOR NAME	RLTN
GARCIA, ARMANDO	S

ADDRESS	PHONE
6347 RUBYLYN AVE	(323)690-6321

CITY	STATE	ZIP	OCCUPATION
LAS VEGAS	NV	891221111	DISABLED

GUARANTOR EMPLOYER	PHONE
DISABLED	

ADDRESS		
CITY	STATE	ZIP

Insurance Information

PC: M

INSURANCE NAME 1	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY
MEDICARE PART A	A00	(800)633-4227	2EC2PY1UK70			

MAILING ADDRESS	CITY	STATE	ZIP
ATTN NOVITAS SOLUTIO P O BOX 3385	MECHANISCBU	PA	17055

SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM
GARCIA, ARMANDO	MEDICARE,	M	19720602	NEVADA

COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY
1	A00	(877)235-8073		Y	S		

INSURANCE NAME 2	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY

MAILING ADDRESS	CITY	STATE	ZIP

SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM

COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY

INSURANCE NAME 3	PLAN	PHONE	POLICY#	GROUP#	AUTH#	AUTHORIZED BY

MAILING ADDRESS	CITY	STATE	ZIP

SUBSCRIBER NAME	MAIL TO NAME	INS SEX	D.O.B.	GNM

COB	PAYOR ID	UR PHONE	EXT	INS VERIFIED	SUBSCRIBER RLTN	NOC REF NUMBER	NOC REF BY

Admitting Information

ADM DATE	TIME	SRC	PT TYPE	PRI	ACCIDENT INFO.	VOLUNTARY ADM	ORGAN DNR	ISOL	PT STS
08/28/2024	0114	EO	D	X	H 08/27/2024				IA

ATTENDING PHYSICIAN	ATTENDING #	REF SRC	ADM BY	PREV ADM DATE	PREVIOUS FACILITY
DANIELIAN MICHELLE	148007		REIDTR		

ADM DIAGNOSIS	HOSP SVC	DIAGNOSTIC CODE	SURGERY DATE	BLDLESS	N/S	RM/BED	Ethnicity
RESPIRATORY FAILURE	ICU		//	N	ICU	2059 01	

ADMITTING PHYSICIAN	ADMITTING #	DISCHARGE DATE	TIME	DISCHARGE DISP	PREVIOUS DISCHARGE DATE
DANIELIAN MICHELLE	148007				

PROCEDURE	CLINICAL COMMENT	MODE OF ARRIVAL / ACCOMPANIED BY
		AMB AMBULANCE

REFERRING PHYSICIAN	REFERRING #	EMAIL ADDRESS
	999870	

REL OF INFO	ADV DIR	LIVING WILL	LOCATION OF WILL / DIRECTIVE	FAMILY DOCTOR	FAMILY DOCTOR #	DIV
Y				NO PCP NO FAMILY DR	999987	H

Henderson Hospital
1050 W. Galleria Drive
Henderson, NV 89011

EL0012

MEDICAL RECORD

UHS-9002
Rev. 10/2021

Patient Identification

5007238115-8210503

GARCIA, ARMANDO
DOB: 06/02/1972 52Y SX: M ICU
MRN: 8210503 ADM/REG DT:08/28/2024
Henderson Hospital

Page 1 of 1

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Progress Note - Physician

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	8/31/2024 17:09 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Parekh DO,Parag P (8/31/2024 17:12 PDT)
SIGN INFORMATION:	Parekh DO,Parag P (8/31/2024 17:12 PDT)

Subjective

Consult received
Add fluid restriction and furosemide x1
Monitor corrected Na
Withdrawals, high BP noted. Add coreg
Full eval to follow

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**L** 36.4	08/31/2024 04:17	36.8	08/30/2024 18:15	36.8	08/30/2024 18:15
Heart Rate Monitored	90	08/31/2024 13:36	79	08/31/2024 01:53	94	08/30/2024 17:30
Resp Rate	20	08/31/2024 13:36	16	08/30/2024 17:30	20	08/31/2024 07:11
SBP	**C** 188	08/31/2024 16:12	**H** 173	08/31/2024 14:00	**C** 208	08/30/2024 17:30
DBP	**H** 86	08/31/2024 16:12	79	08/31/2024 04:17	**C** 93	08/30/2024 17:30
MAP	123	08/31/2024 01:53	123	08/31/2024 01:53	138	08/30/2024 20:04
SpO2	96	08/31/2024 13:36	96	08/31/2024 00:17	98	08/30/2024 18:15
O2 Flow Rate	on 4 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Intake and Output (current encounter)

	08/28 01:14 08/31 17:09	08/31 07:00 08/31 17:09	08/30 07:00 08/31 06:59	08/29 07:00 08/30 06:59	08/28 07:00 08/29 06:59
Intake	1319.6	0	0	582.8	716.5
Output	20930	1000	4270	4860	5750
Net Total	-19610.4	-1000	-4270	-4277.2	-5033.5
Stool Count	3	0	2	1	0

Physical Exam

Results

BMP, Mg, and Phos (Last Within 24hrs)

Sodium: 127 mmol/L Low (12:28)
Potassium: 4 mmol/L (12:28)
Chloride: 96 mmol/L Low (12:28)
CO2: 25 mmol/L (12:28)
BUN: 48 mg/dL High (12:28)
Creatinine: 1.49 mg/dL High (12:28)
Glucose Level: 302 mg/dL High (12:28)
Calcium: 9.8 mg/dL (12:28)

Progress Note - Physician

Mg Lvl: 2.7 mg/dL High (12:28)

Assessment/Plan
Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Home with family care

Electronically Signed By: Parekh, Parag DO
On: 08.31.2024 17:12 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	8/31/2024 15:12 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Moreno MD,Luis A (8/31/2024 15:14 PDT)
SIGN INFORMATION:	Moreno MD,Luis A (8/31/2024 15:14 PDT)

Subjective
No acute events.
labs and vital sign reviewed
Discussed with nurse

Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Ears and nose without masses, lesions or deformities. Tympanic membranes clear bilaterally. Trachea midline.
Neck: Supple, Full range of motion.
Respiratory: Clear to auscultation and percussion. No wheezing, rales or rhonchi.
Cardiovascular: Heart sounds normal. No thrills. Regular rate and rhythm, no murmurs, rubs or gallops.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds. No pulsatile mass. No hepatosplenomegaly.
Genitourinary: No costovertebral angle tenderness.
Neurologic: Cranial nerves II-XII grossly intact. No focal neurological deficits. Deep tendon reflexes +2 bilaterally. Flexor plantar response. Moves all extremities spontaneously. Sensation intact bilaterally.
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Assessment:
Hyponatremia: Sodium 127
AKI (acute kidney injury) (N17.9)
Acute respiratory failure
Sepsis
COPD exacerbation
Congestive heart failure exacerbation
Hyperglycemia
Hypomagnesemia
Hyponatremia

Progress Note - Physician

Hypophosphatemia
Acute on chronic kidney disease
History congestive heart failure
History of COPD on 2 L nasal cannula
History hypertension
History of diabetes mellitus type 2
S/P L carotid endarterectomy with patch angioplasty 7/2024

Plan:
extubated
Nephrologist consulted
Monitor sodium closely
Images reviewed. 08/31
SVN treatments as needed
on rocephina and doxy
HTN urgency added labetalol, started on PO meds with metoprolol and amlodipine bp is still high
added doxazosin
awaiting FEES
Latest echocardiogram 7/2024 EF 60 to 65%
Continue to monitor and trend renal indicis
HR is high per pt going through withdrawals will restart subaxone

Chief Complaint/Reason for Consultation
Chief Complaint: SOB difficulty breathing at home. tympanic temp of 104

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)
No Known Medication Allergies (Severity: Unknown severity, Onset: Unknown)

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**L** 36.4	08/31/2024 04:17	36.8	08/30/2024 18:15	**H** 37.4	08/30/2024 16:00
Heart Rate Monitored	89	08/31/2024 13:26	79	08/31/2024 01:53	94	08/30/2024 17:00
Resp Rate	20	08/31/2024 13:26	16	08/30/2024 17:30	20	08/31/2024 07:11
SBP	**H** 173	08/31/2024 14:00	**H** 173	08/31/2024 14:00	**C** 208	08/30/2024 17:30
DBP	**H** 81	08/31/2024 14:00	79	08/31/2024 04:17	**C** 93	08/30/2024 17:00
MAP	123	08/31/2024 01:53	117	08/30/2024 16:00	138	08/30/2024 20:04
SpO2	96	08/31/2024 13:26	96	08/31/2024 00:17	98	08/30/2024 16:00
O2 Flow Rate	on 4 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Intake and Output (current encounter)

	08/28 01:14	08/31 07:00	08/30 07:00	08/29 07:00	08/28 07:00
	08/31 15:12	08/31 15:12	08/31 06:59	08/30 06:59	08/29 06:59
Intake	1319.6	0	0	582.8	716.5
Output	20930	1000	4270	4860	5750

Progress Note - Physician

Net Total	-19610.4	-1000	-4270	-4277.2	-5033.5
Stool Count	3	0	2	1	0

Physical Exam

Home Medications
albuterol (Albuterol (Eqv-ProAir HFA) 90 mcg/inh inhalation aerosol) 2 Puffs Inhalation Every 4 hours as needed as needed for wheezing
amLODIPine (amLODIPine 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
aspirin (Aspirin Low Dose 81 mg oral delayed release tablet) 81 Milligram 1 Tabs By Mouth Daily
atorvastatin (atorvastatin 40 mg oral tablet) 40 Milligram 1 Tabs By Mouth Daily for 30 Days
buprenorphine-naloxone (buprenorphine-naloxone 4 mg-1 mg sublingual film) 1 Film Sublingual 3 Times a Day dissolve under the tongue
cholecalciferol (Vitamin D3 125 mcg (5000 intl units) oral tablet) 125 Microgram 1 Tabs By Mouth Daily
dapagliflozin (Farxiga 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
furosemide (furosemide 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
glimepiride (glimepiride 2 mg oral tablet) 2 Milligram 1 Tabs By Mouth 2 Times a Day
hydrALAZINE (hydrALAZINE 100 mg oral tablet) 100 Milligram 1 Tabs By Mouth 3 Times a Day
metoprolol (Metoprolol Tartrate 100 mg oral tablet) 200 Milligram 2 Tabs By Mouth 2 Times a Day
minoxidil (minoxidil 2.5 mg oral tablet) 5 Milligram 2 Tabs By Mouth 2 Times a Day
mirabegron (Myrbetriq 50 mg oral tablet, extended release) 50 Milligram 1 Tabs By Mouth Daily do not crush or chew
nortriptyline (nortriptyline 25 mg oral capsule) 50 Milligram 2 Capsules By Mouth Every PM
olodaterol (Striverdi Respimat 2.5 mcg/inh inhalation aerosol) 2 Puffs Inhalation Daily
traZODone (traZODone 100 mg oral tablet) 200 Milligram 2 Tabs By Mouth at Bedtime

Inpatient Medications
Medications (27) Active
SCHEDULED: (24)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 5 mg Tab (amLODIPine) 10 mg 2 Tabs, OG-Tube, Daily
aspirin 81 mg Chew Tab (aspirin 81 mg oral tablet, chewable) 81 mg 1 Tabs, OG-Tube, Daily
atorvastatin 40 mg Tab (Lipitor 40 mg oral tablet) 40 mg 1 Tabs, OG-Tube, qHS
buprenorphine 2 mg sublingual Dis Tab (buprenorphine) 4 mg 2 Tabs, SubLingual, TID
cefTRIAXone 2 gm vial (Rocephin 2 g injection) 2 gm 20 mL, IV Push, qHS
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
doxazosin 2 mg Tab (doxazosin) 2 mg 1 Tabs, OG-Tube, Daily
doxycycline + Dextrose 5% in Water 100 mL (doxycycline + Dextrose 5% in Water intravenous solution 100 mL) 100 mg 10 mL, IV Piggyback, BID
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin glargine 1 unit/0.01 mL syringe (Lantus) 5 units 0.05 mL, SubCutaneous, qAM
insulin lispro HumaLOG 1 unit/0.01mL syringe (insulin lispro HumaLOG correctional scale High) 4-18 Units, SubCutaneous, Before Meals and HS

Progress Note - Physician

methylPREDNISolone sod succinate 40 mg Vial PF (methylPREDNISolone sodium succinate (Solu-Medrol)) 20 mg 0.5 mL, IV Push, q12H Interval
metoprolol tartrate 50 mg Tab (Lopressor 50 mg oral tablet) 100 mg 2 Tabs, Oral, BID
nortriptyline 25 mg Cap (nortriptyline) 25 mg 1 Caps, Oral, qPM
pantoprazole 40 mg vial (Protonix) 40 mg, IV Push, Daily
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
traZODone 50 mg Tab (traZODone) 150 mg 3 Tabs, Oral, qHS
CONTINUOUS: (0)
PRN: (3)
acetaminophen 650 mg/20.3 mL Oral Liq (Tylenol Liquid) 650 mg 20.3 mL, OG-Tube, q6H
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H Interval

Results
Recent Labs
General Chemistry

Glucose Level	302 mg/dL (High)	08/31/2024 12:28
POC Glucose	273 mg/dL (High)	08/31/2024 10:49
Sodium	127 mmol/L (Low)	08/31/2024 12:28
Potassium	4.0 mmol/L (Normal)	08/31/2024 12:28
Chloride	96 mmol/L (Low)	08/31/2024 12:28
CO2	25 mmol/L (Normal)	08/31/2024 12:28
Anion Gap	6 mmol/L (Normal)	08/31/2024 12:28
BUN	48 mg/dL (High)	08/31/2024 12:28
Creatinine	1.490 mg/dL (High)	08/31/2024 12:28
BUN/Creat Ratio	32 (High)	08/31/2024 12:28
Calcium	9.8 mg/dL (Normal)	08/31/2024 12:28
Mg Lvl	2.7 mg/dL (High)	08/31/2024 12:28
Estimated Creatinine Clearance	56.11 mL/min ()	08/31/2024 13:43
eGFR Cr	56 mL/min/1.73m2 (N/A)	08/31/2024 12:28
eGFR Pediatric	Not Reported (N/A)	08/31/2024 12:28
Calc Osmo	279 mOsmol/kg (Normal)	08/31/2024 12:28
MAR Glucose Result	273 ()	08/31/2024 11:51

General Hematology

WBC	11.79 x10e3/mcL (Normal)	08/31/2024 12:28
RBC	4.93 x10e6/mcL (Normal)	08/31/2024 12:28

Hgb	15.1 gm/dL (Normal)	08/31/2024 12:28
Hct	47.3 % (Normal)	08/31/2024 12:28
MCV	95.9 Femtoliters (Normal)	08/31/2024 12:28
MCH	30.6 pg (Normal)	08/31/2024 12:28
MCHC	31.9 gm/dL (Normal)	08/31/2024 12:28
RDW-SD	44.8 Femtoliters (Normal)	08/31/2024 12:28
RDW-CV	12.6 % (Normal)	08/31/2024 12:28
Plt	141 x10e3/mcL (Normal)	08/31/2024 12:28
MPV	10.0 Femtoliters (Normal)	08/31/2024 12:28
Neut % Auto	90.2 % (N/A)	08/30/2024 02:33
Lymph % Auto	4.8 % (N/A)	08/30/2024 02:33
Mono % Auto	3.1 % (N/A)	08/30/2024 02:33
Eos % Auto	0.0 % (N/A)	08/30/2024 02:33
Baso % Auto	0.2 % (N/A)	08/30/2024 02:33
Immature Grans %	1.7 % (High)	08/30/2024 02:33
Neut # Auto	10.77 x10e3/mcL (High)	08/30/2024 02:33
Lymph # Auto	0.57 x10e3/mcL (Low)	08/30/2024 02:33
Mono # Auto	0.37 x10e3/mcL (Normal)	08/30/2024 02:33
Eos # Auto	0.00 x10e3/mcL (Normal)	08/30/2024 02:33
Baso # Auto	0.02 x10e3/mcL (Normal)	08/30/2024 02:33
Immature Grans # Auto	0.20 x10e3/mcL (High)	08/30/2024 02:33

Abnormal Labs

General Chemistry

BUN 48 mg/dL (High) 08/31/2024 12:28
BUN/Creat Ratio 32 (High) 08/31/2024 12:28
Chloride 96 mmol/L (Low) 08/31/2024 12:28
Creatinine 1.490 mg/dL (High) 08/31/2024 12:28
Glucose Level 302 mg/dL (High) 08/31/2024 12:28
Mg Lvl 2.7 mg/dL (High) 08/31/2024 12:28
POC Glucose 273 mg/dL (High) 08/31/2024 10:49
Sodium 127 mmol/L (Low) 08/31/2024 12:28

Note: Critical results are displayed in red.

Progress Note - Physician

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**11.79** 08/31/2024 12:28	**11.93** 08/30/2024 02:33	**14.06 H** 08/29/2024 01:32	**12.75 H** 08/28/2024 03:50
Hgb	**15.1** 08/31/2024 12:28	**14.6** 08/30/2024 02:33	**13.8** 08/29/2024 01:32	**13.6** 08/28/2024 03:50
Hct	**47.3** 08/31/2024 12:28	**47.4** 08/30/2024 02:33	**41.9** 08/29/2024 01:32	**41.2** 08/28/2024 03:50
Baso # Auto	**0.02** 08/30/2024 02:33	**0.01** 08/29/2024 01:32	**0.04** 08/27/2024 20:57	
Baso % Auto	**0.2** 08/30/2024 02:33	**0.1** 08/29/2024 01:32	**0.3** 08/27/2024 20:57	
Eos # Auto	**0.00** 08/30/2024 02:33	**0.00** 08/29/2024 01:32	**0.06** 08/27/2024 20:57	
Eos % Auto	**0.0** 08/30/2024 02:33	**0.0** 08/29/2024 01:32	**0.5** 08/27/2024 20:57	
Immature Grans %	**1.7 H** 08/30/2024 02:33	**0.9 H** 08/29/2024 01:32	**1.2 H** 08/27/2024 20:57	
Immature Grans # Auto	**0.20 H** 08/30/2024 02:33	**0.13 H** 08/29/2024 01:32	**0.16 H** 08/27/2024 20:57	
Lymph # Auto	**0.57 L** 08/30/2024 02:33	**0.64 L** 08/29/2024 01:32	**1.13** 08/27/2024 20:57	
Lymph % Auto	**4.8** 08/30/2024 02:33	**4.6** 08/29/2024 01:32	**8.5** 08/27/2024 20:57	
Mono # Auto	**0.37** 08/30/2024 02:33	**0.46** 08/29/2024 01:32	**0.93** 08/27/2024 20:57	
Mono % Auto	**3.1** 08/30/2024 02:33	**3.3** 08/29/2024 01:32	**7.0** 08/27/2024 20:57	
Neut # Auto	**10.77 H** 08/30/2024 02:33	**12.82 H** 08/29/2024 01:32	**10.90 H** 08/27/2024 20:57	
Neut % Auto	**90.2** 08/30/2024 02:33	**91.1** 08/29/2024 01:32	**82.5** 08/27/2024 20:57	
Plt	**141** 08/31/2024 12:28	**148** 08/30/2024 02:33	**123 L** 08/29/2024 01:32	**109 L** 08/28/2024 03:50
Glucose Level	**302 H** 08/31/2024 12:28	**286 H** 08/30/2024 02:33	**264 H** 08/29/2024 01:32	**383 H** 08/28/2024 03:50
Sodium	**127 L** 08/31/2024 12:28	**141** 08/30/2024 02:33	**138** 08/29/2024 01:32	**130 L** 08/28/2024 03:50
Potassium	**4.0** 08/31/2024 12:28	**3.9** 08/30/2024 02:33	**3.7** 08/29/2024 01:32	**4.2** 08/28/2024 03:50
Chloride	**96 L** 08/31/2024 12:28	**110 H** 08/30/2024 02:33	**106** 08/29/2024 01:32	**98** 08/28/2024 03:50
CO2	**25** 08/31/2024 12:28	**26** 08/30/2024 02:33	**29** 08/29/2024 01:32	**27** 08/28/2024 03:50
Creatinine	**1.490 H**	**1.840 H**	**1.620 H**	**1.790 H**

Progress Note - Physician

	08/31/2024 12:28	08/30/2024 02:33	08/29/2024 01:32	08/28/2024 03:50
BUN	**48 H** 08/31/2024 12:28	**48 H** 08/30/2024 02:33	**35 H** 08/29/2024 01:32	**22 H** 08/28/2024 03:50
BUN/Creat Ratio	**32 H** 08/31/2024 12:28	**26 H** 08/30/2024 02:33	**22** 08/29/2024 01:32	**12** 08/28/2024 03:50
Mg Lvl	**2.7 H** 08/31/2024 12:28	**2.5 H** 08/30/2024 02:33	**2.6 H** 08/29/2024 01:32	**2.2** 08/28/2024 03:50
Phosphorus	**4.1** 08/28/2024 03:50	**0.7** 08/27/2024 20:57		
Calcium	**9.8** 08/31/2024 12:28	**10.2 H** 08/30/2024 02:33	**10.2 H** 08/29/2024 01:32	**9.8** 08/28/2024 03:50
Albumin. Level	**4.0** 08/27/2024 20:57			
Ur Protein	**3+** 08/27/2024 22:04			
T Bili	**1.0** 08/27/2024 20:57			
Alk Phos	**72** 08/27/2024 20:57			
AST	**20** 08/27/2024 20:57			
ALT	**24** 08/27/2024 20:57			
aPTT	**31** 08/28/2024 00:53			
PT	**11.9** 08/28/2024 00:53			
INR	**1.1** 08/28/2024 00:53			

Assessment/Plan
Diagnoses
AKI (acute kidney injury) (N17.9)
Bilateral lower extremity edema (R60.0)
CHF exacerbation (I50.9)
COPD exacerbation (J44.1)
Congestive heart failure (I50.9)
Elevated brain natriuretic peptide (BNP) level (R79.89)
Fever (R50.9)
History of pneumonia (Z87.01)
Hyperglycemia (R73.9)
Hyperglycemia due to diabetes mellitus (E11.65)
Hypertensive emergency (I16.1)
Hypomagnesemia (E83.42)
Hyponatremia (E87.1)

Hypophosphatemia (E83.39)
Leukocytosis (D72.829)
Morbid obesity (E66.01)
Oxygen dependent (Z99.81)
Respiratory distress (R06.03)
Respiratory failure (J96.90)
Sepsis (A41.9)
1. Respiratory failure (J96.90)

Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Home with family care

Electronically Signed By: Moreno, Luis MD
On: 08.31.2024 15:14 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	8/30/2024 14:06 PDT
RESULT STATUS:	Modified
PERFORM INFORMATION:	Kabsoun MD,Sara H (8/30/2024 14:10 PDT)
SIGN INFORMATION:	Kabsoun MD,Sara H (8/30/2024 19:48 PDT); Kabsoun MD, Sara H (8/30/2024 14:10 PDT)

Subjective
seen and examined no chest pain no sob discussed with RN

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)
No Known Medication Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(6)
Disc disorder
DM (diabetes mellitus)
ESBL Escherichia coli
Gout
HLD (hyperlipidemia)
HTN (hypertension)

Past Surgical History
Bronchoscopy w/ Procedures (Laterality NA): 08/29/24
Endarterectomy Carotid Artery (Left): 07/26/24
EGD w/ Biopsy (Laterality NA): 06/27/22
foot sx
shoulder sx

Progress Note - Physician

Social History

Alcohol
Risk Assessment: Denies Alcohol Use; Details: Denies

Substance Abuse
Risk Assessment: Denies Substance Abuse; Details: Denies

Tobacco
Risk Assessment: High Risk; Details: Current

Family History
No Family History documented.

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**H** 37.8	08/30/2024 12:00	**H** 37.6	08/29/2024 19:00	**H** 38	08/29/2024 20:00
Heart Rate Monitored	99	08/30/2024 12:00	73	08/29/2024 15:00	**H** 123	08/29/2024 20:00
Resp Rate	**H** 21	08/30/2024 12:00	16	08/30/2024 03:30	**C** 35	08/30/2024 02:30
SBP	**C** 207	08/30/2024 12:00	134	08/30/2024 03:30	**C** 207	08/30/2024 12:00
DBP	**C** 96	08/30/2024 12:00	63	08/30/2024 03:30	**C** 96	08/30/2024 12:00
MAP	138	08/30/2024 12:00	91	08/30/2024 03:30	138	08/30/2024 12:00
SpO2	97	08/30/2024 12:00	94	08/29/2024 20:00	99	08/29/2024 15:00
O2 Flow Rate	on 2 l/min		on 5 l/min		on 30 %	
O2 Therapy	Nasal cannula		Nasal cannula		Ventilator	

Intake and Output (current encounter)

	08/28 01:14	08/30 07:00	08/29 07:00	08/28 07:00	08/27 07:00
	08/30 14:06	08/30 14:06	08/30 06:59	08/29 06:59	08/28 06:59
Intake	1319.6	0	582.8	716.5	20.3
Output	18160	2500	4860	5750	5050
Net Total	-16840.4	-2500	-4277.2	-5033.5	-5029.7
Stool Count	2	1	1	0	0

Mobility & Ambulation Level
PT Bed Mobility
Bed Mobility Roll Left: Rehab Contact guard assistance (08/30/24)
Bed Mobility Roll Right: Rehab Contact guard assistance (08/30/24)
Bed Mobility Supine to Sit: Rehab Contact guard assistance (08/30/24)
Bed Mobility Sit to Supine: Rehab Contact guard assistance (08/30/24)
PT Transfer Mobility
Transfer Sit to Stand Rehab: Rehab Contact guard assistance (08/30/24)
Transfer Stand to Sit Rehab: Rehab Contact guard assistance (08/30/24)

Progress Note - Physician

Transfer Bed to and From Chair Rehab: Rehab Contact guard assistance (08/30/24)

Physical Exam

Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Ears and nose without masses, lesions or deformities. Tympanic membranes clear bilaterally. Trachea midline.
Neck: Supple, Full range of motion.
Respiratory: Clear to auscultation and percussion. No wheezing, rales or rhonchi.
Cardiovascular: Heart sounds normal. No thrills. Regular rate and rhythm, no murmurs, rubs or gallops.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds. No pulsatile mass. No hepatosplenomegaly.
Genitourinary: No costovertebral angle tenderness.
Neurologic: Cranial nerves II-XII grossly intact. No focal neurological deficits. Deep tendon reflexes +2 bilaterally. Flexor plantar response. Moves all extremities spontaneously. Sensation intact bilaterally.
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.
Lymphatics: Palpation of neck reveals no swelling or tenderness of neck nodes. Palpation of groin reveals no swelling or tenderness of groin nodes.

Home Medications
albuterol (Albuterol (Eqv-ProAir HFA) 90 mcg/inh inhalation aerosol) 2 Puffs Inhalation Every 4 hours as needed as needed for wheezing
amLODIPine (amLODIPine 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
aspirin (Aspirin Low Dose 81 mg oral delayed release tablet) 81 Milligram 1 Tabs By Mouth Daily
atorvastatin (atorvastatin 40 mg oral tablet) 40 Milligram 1 Tabs By Mouth Daily for 30 Days
buprenorphine-naloxone (buprenorphine-naloxone 4 mg-1 mg sublingual film) 1 Film Sublingual 3 Times a Day dissolve under the tongue
cholecalciferol (Vitamin D3 125 mcg (5000 intl units) oral tablet) 125 Microgram 1 Tabs By Mouth Daily
dapagliflozin (Farxiga 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
furosemide (furosemide 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
glimepiride (glimepiride 2 mg oral tablet) 2 Milligram 1 Tabs By Mouth 2 Times a Day
hydrALAZINE (hydrALAZINE 100 mg oral tablet) 100 Milligram 1 Tabs By Mouth 3 Times a Day
metoprolol (Metoprolol Tartrate 100 mg oral tablet) 200 Milligram 2 Tabs By Mouth 2 Times a Day
minoxidil (minoxidil 2.5 mg oral tablet) 5 Milligram 2 Tabs By Mouth 2 Times a Day
mirabegron (Myrbetriq 50 mg oral tablet, extended release) 50 Milligram 1 Tabs By Mouth Daily do not crush or chew
nortriptyline (nortriptyline 25 mg oral capsule) 50 Milligram 2 Capsules By Mouth Every PM
olodaterol (Striverdi Respimat 2.5 mcg/inh inhalation aerosol) 2 Puffs Inhalation Daily
traZODone (traZODone 100 mg oral tablet) 200 Milligram 2 Tabs By Mouth at Bedtime

Inpatient Medications
Medications (24) Active
SCHEDULED: (20)

Progress Note - Physician

albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 5 mg Tab (amLODIPine) 10 mg 2 Tabs, Oral, Daily
aspirin 81 mg Chew Tab (aspirin 81 mg oral tablet, chewable) 81 mg 1 Tabs, OG-Tube, Daily
atorvastatin 40 mg Tab (Lipitor 40 mg oral tablet) 40 mg 1 Tabs, OG-Tube, qHS
buprenorphine 2 mg sublingual Dis Tab (buprenorphine) 4 mg 2 Tabs, SubLingual, TID
cefTRIAXone 2 gm vial (Rocephin 2 g injection) 2 gm 20 mL, IV Push, qHS
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
doxycycline + Dextrose 5% in Water 100 mL (doxycycline + Dextrose 5% in Water intravenous solution 100 mL) 100 mg 10 mL, IV Piggyback, BID
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin lispro HumaLOG 1 unit/0.01mL syringe (insulin lispro HumaLOG correctional scale High) 4-18 Units, SubCutaneous, q4H Interval
methylPREDNISolone sod succinate 40 mg Vial PF (methylPREDNISolone sodium succinate (Solu-Medrol)) 20 mg 0.5 mL, IV Push, q12H Interval
metoprolol tartrate 50 mg Tab (Lopressor 50 mg oral tablet) 50 mg 1 Tabs, Oral, BID
pantoprazole 40 mg vial (Protonix) 40 mg, IV Push, Daily
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
CONTINUOUS: (0)
PRN: (4)
acetaminophen 650 mg/20.3 mL Oral Liq (Tylenol Liquid) 650 mg 20.3 mL, OG-Tube, q6H
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H Interval
morphine 4 mg/mL injection 1 mL (morphine) 4 mg 1 mL, IV Push, q4H

Results
Recent Labs
Blood Gases

pH Arterial	7.47 (High)	08/29/2024 15:09
Art pCO2	43.6 mmHg (Normal)	08/29/2024 15:09
Art pO2	84.9 mmHg (Normal)	08/29/2024 15:09
HCO3 Art	30.9 mmol/L (High)	08/29/2024 15:09
Arterial pCO2(T)	43.6 mmHg (N/A)	08/29/2024 15:09
Arterial pH(T)	7.47 (N/A)	08/29/2024 15:09
Arterial pO2(T)	84.9 mmHg (N/A)	08/29/2024 15:09
Art Base Excess	7.1 mmol/L (High)	08/29/2024 15:09
Art sO2	97 % (Normal)	08/29/2024 15:09
POC Pt Temp	37.0 DegC (N/A)	08/29/2024 15:09
POC FIO2	30 % (N/A)	08/29/2024 15:09
POC Sample	Arterial (N/A)	08/29/2024 15:09
POC Site	Radial Right (N/A)	08/29/2024 15:09
POC Allen Test	POS (N/A)	08/29/2024 15:09

Progress Note - Physician

POC Del Sys	VENTILATOR (N/A)	08/29/2024 15:09
POC Vent Mode	SPN-CPAP VS (N/A)	08/29/2024 15:09
POC Total RR	18 br/min (N/A)	08/29/2024 15:09
POC Set RR	18 br/min (N/A)	08/29/2024 04:00
POC VE	10.0 L/min (N/A)	08/29/2024 04:00
POC VT	550 mL (N/A)	08/29/2024 04:00
POC PEEP	5 cm H2O (N/A)	08/29/2024 15:09
POC PS	7 cm H2O (N/A)	08/29/2024 15:09
POC IT	0.900 Seconds (N/A)	08/29/2024 04:00

General Chemistry

Glucose Level	286 mg/dL (High)	08/30/2024 02:33
POC Glucose	247 mg/dL (High)	08/30/2024 13:44
Sodium	141 mmol/L (Normal)	08/30/2024 02:33
Potassium	3.9 mmol/L (Normal)	08/30/2024 02:33
Chloride	110 mmol/L (High)	08/30/2024 02:33
CO2	26 mmol/L (Normal)	08/30/2024 02:33
Anion Gap	5 mmol/L (Normal)	08/30/2024 02:33
BUN	48 mg/dL (High)	08/30/2024 02:33
Creatinine	1.840 mg/dL (High)	08/30/2024 02:33
BUN/Creat Ratio	26 (High)	08/30/2024 02:33
Calcium	10.2 mg/dL (High)	08/30/2024 02:33
Mg Lvl	2.5 mg/dL (High)	08/30/2024 02:33
Estimated Creatinine Clearance	45.43 mL/min ()	08/30/2024 03:15
eGFR Cr	44 mL/min/1.73m2 (N/A)	08/30/2024 02:33
eGFR Pediatric	Not Reported (N/A)	08/30/2024 02:33
Calc Osmo	304 mOsmol/kg (High)	08/30/2024 02:33
MAR Glucose Result	247 ()	08/30/2024 13:50

General Hematology

WBC	11.93 x10e3/mcL (Normal)	08/30/2024 02:33
RBC	4.84 x10e6/mcL (Normal)	08/30/2024 02:33
Hgb	14.6 gm/dL (Normal)	08/30/2024 02:33
Hct	47.4 % (Normal)	08/30/2024 02:33
MCV	97.9 Femtoliters (High)	08/30/2024 02:33
MCH	30.2 pg (Normal)	08/30/2024 02:33
MCHC	30.8 gm/dL (Low)	08/30/2024 02:33

Progress Note - Physician

RDW-SD	47.8 Femtoliters (Normal)	08/30/2024 02:33
RDW-CV	13.2 % (Normal)	08/30/2024 02:33
Plt	148 x10e3/mcL (Normal)	08/30/2024 02:33
MPV	10.4 Femtoliters (Normal)	08/30/2024 02:33
Neut % Auto	90.2 % (N/A)	08/30/2024 02:33
Lymph % Auto	4.8 % (N/A)	08/30/2024 02:33
Mono % Auto	3.1 % (N/A)	08/30/2024 02:33
Eos % Auto	0.0 % (N/A)	08/30/2024 02:33
Baso % Auto	0.2 % (N/A)	08/30/2024 02:33
Immature Grans %	1.7 % (High)	08/30/2024 02:33
Neut # Auto	10.77 x10e3/mcL (High)	08/30/2024 02:33
Lymph # Auto	0.57 x10e3/mcL (Low)	08/30/2024 02:33
Mono # Auto	0.37 x10e3/mcL (Normal)	08/30/2024 02:33
Eos # Auto	0.00 x10e3/mcL (Normal)	08/30/2024 02:33
Baso # Auto	0.02 x10e3/mcL (Normal)	08/30/2024 02:33
Immature Grans # Auto	0.20 x10e3/mcL (High)	08/30/2024 02:33

Abnormal Labs

Blood Gases

Art Base Excess 7.1 mmol/L (High) 08/29/2024 15:09
HCO3 Art 30.9 mmol/L (High) 08/29/2024 15:09
pH Arterial 7.47 (High) 08/29/2024 15:09

General Chemistry

BUN 48 mg/dL (High) 08/30/2024 02:33
BUN/Creat Ratio 26 (High) 08/30/2024 02:33
Calc Osmo 304 mOsmol/kg (High) 08/30/2024 02:33
Calcium 10.2 mg/dL (High) 08/30/2024 02:33
Chloride 110 mmol/L (High) 08/30/2024 02:33
Creatinine 1.840 mg/dL (High) 08/30/2024 02:33
Glucose Level 286 mg/dL (High) 08/30/2024 02:33
Mg Lvl 2.5 mg/dL (High) 08/30/2024 02:33
POC Glucose 247 mg/dL (High) 08/30/2024 13:44

General Hematology

Immature Grans % 1.7 % (High) 08/30/2024 02:33
Immature Grans # Auto 0.20 x10e3/mcL (High) 08/30/2024 02:33

Progress Note - Physician

```
Lymph # Auto          0.57 x10e3/mcL (Low)          08/30/2024 02:33
MCHC          30.8 gm/dL (Low)          08/30/2024 02:33
MCV          97.9 Femtoliters (High)          08/30/2024 02:33
Neut # Auto          10.77 x10e3/mcL (High)          08/30/2024 02:33
```

Note: Critical results are displayed in red.

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**11.93** 08/30/2024 02:33	**14.06 H** 08/29/2024 01:32	**12.75 H** 08/28/2024 03:50	**13.22 H** 08/27/2024 20:57
Hgb	**14.6** 08/30/2024 02:33	**13.8** 08/29/2024 01:32	**13.6** 08/28/2024 03:50	**14.1** 08/27/2024 20:57
Hct	**47.4** 08/30/2024 02:33	**41.9** 08/29/2024 01:32	**41.2** 08/28/2024 03:50	**41.2** 08/27/2024 20:57
Baso # Auto	**0.02** 08/30/2024 02:33	**0.01** 08/29/2024 01:32	**0.04** 08/27/2024 20:57	
Baso % Auto	**0.2** 08/30/2024 02:33	**0.1** 08/29/2024 01:32	**0.3** 08/27/2024 20:57	
Eos # Auto	**0.00** 08/30/2024 02:33	**0.00** 08/29/2024 01:32	**0.06** 08/27/2024 20:57	
Eos % Auto	**0.0** 08/30/2024 02:33	**0.0** 08/29/2024 01:32	**0.5** 08/27/2024 20:57	
Immature Grans %	**1.7 H** 08/30/2024 02:33	**0.9 H** 08/29/2024 01:32	**1.2 H** 08/27/2024 20:57	
Immature Grans # Auto	**0.20 H** 08/30/2024 02:33	**0.13 H** 08/29/2024 01:32	**0.16 H** 08/27/2024 20:57	
Lymph # Auto	**0.57 L** 08/30/2024 02:33	**0.64 L** 08/29/2024 01:32	**1.13** 08/27/2024 20:57	
Lymph % Auto	**4.8** 08/30/2024 02:33	**4.6** 08/29/2024 01:32	**8.5** 08/27/2024 20:57	
Mono # Auto	**0.37** 08/30/2024 02:33	**0.46** 08/29/2024 01:32	**0.93** 08/27/2024 20:57	
Mono % Auto	**3.1** 08/30/2024 02:33	**3.3** 08/29/2024 01:32	**7.0** 08/27/2024 20:57	
Neut # Auto	**10.77 H** 08/30/2024 02:33	**12.82 H** 08/29/2024 01:32	**10.90 H** 08/27/2024 20:57	
Neut % Auto	**90.2** 08/30/2024 02:33	**91.1** 08/29/2024 01:32	**82.5** 08/27/2024 20:57	
Plt	**148** 08/30/2024 02:33	**123 L** 08/29/2024 01:32	**109 L** 08/28/2024 03:50	**117 L** 08/27/2024 20:57
Glucose Level	**286 H** 08/30/2024 02:33	**264 H** 08/29/2024 01:32	**383 H** 08/28/2024 03:50	**316 H** 08/27/2024 20:57
Sodium	**141** 08/30/2024 02:33	**138** 08/29/2024 01:32	**130 L** 08/28/2024 03:50	**134 L** 08/27/2024 20:57
Potassium	**3.9**	**3.7**	**4.2**	**3.9**

Progress Note - Physician

	08/30/2024 02:33	08/29/2024 01:32	08/28/2024 03:50	08/27/2024 20:57
Chloride	**110 H** 08/30/2024 02:33	**106** 08/29/2024 01:32	**98** 08/28/2024 03:50	**100** 08/27/2024 20:57
CO2	**26** 08/30/2024 02:33	**29** 08/29/2024 01:32	**27** 08/28/2024 03:50	**28** 08/27/2024 20:57
Creatinine	**1.840 H** 08/30/2024 02:33	**1.620 H** 08/29/2024 01:32	**1.790 H** 08/28/2024 03:50	**2.020 H** 08/27/2024 20:57
BUN	**48 H** 08/30/2024 02:33	**35 H** 08/29/2024 01:32	**22 H** 08/28/2024 03:50	**17** 08/27/2024 20:57
BUN/Creat Ratio	**26 H** 08/30/2024 02:33	**22** 08/29/2024 01:32	**12** 08/28/2024 03:50	**8** 08/27/2024 20:57
Mg Lvl	**2.5 H** 08/30/2024 02:33	**2.6 H** 08/29/2024 01:32	**2.2** 08/28/2024 03:50	**1.7 L** 08/27/2024 20:57
Phosphorus	**4.1** 08/28/2024 03:50	**0.7** 08/27/2024 20:57		
Calcium	**10.2 H** 08/30/2024 02:33	**10.2 H** 08/29/2024 01:32	**9.8** 08/28/2024 03:50	**10.4 H** 08/27/2024 20:57
Albumin. Level	**4.0** 08/27/2024 20:57			
Ur Protein	**3+** 08/27/2024 22:04			
T Bili	**1.0** 08/27/2024 20:57			
Alk Phos	**72** 08/27/2024 20:57			
AST	**20** 08/27/2024 20:57			
ALT	**24** 08/27/2024 20:57			
aPTT	**31** 08/28/2024 00:53			
PT	**11.9** 08/28/2024 00:53			
INR	**1.1** 08/28/2024 00:53			

Blood Glucose Trend

Glucose Level: 286 mg/dL High (08/30/24 02:33:00)
POC Glucose: 247 mg/dL High (08/30/24 13:44:00)
POC Glucose: 276 mg/dL High (08/30/24 09:50:00)
POC Glucose: 329 mg/dL High (08/30/24 05:48:00)
POC Glucose: 246 mg/dL High (08/30/24 01:54:00)
POC Glucose: 214 mg/dL High (08/29/24 21:07:00)
POC Glucose: 230 mg/dL High (08/29/24 17:36:00)

Progress Note - Physician

CBC (Last Within 24hrs)
WBC: 11.93 x10e3/mcL (02:33)
Hgb: 14.6 gm/dL (02:33)
Hct: 47.4 % (02:33)
Plt: 148 x10e3/mcL (02:33)

Differential (Last Within 24hrs)
- Automated -

Neut % Auto: 90.2 % (02:33)
Lymph % Auto: 4.8 % (02:33)
Mono % Auto: 3.1 % (02:33)
Eos % Auto: 0 % (02:33)
Baso % Auto: 0.2 % (02:33)
Immature Grans %: 1.7 % High (02:33)
Neut # Auto: 10.77 x10e3/mcL High (02:33)
Lymph # Auto: 0.57 x10e3/mcL Low (02:33)
Mono # Auto: 0.37 x10e3/mcL (02:33)
Baso # Auto: 0.02 x10e3/mcL (02:33)
Immature Grans # Auto: 0.2 x10e3/mcL High (02:33)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 141 mmol/L (02:33)
Potassium: 3.9 mmol/L (02:33)
Chloride: 110 mmol/L High (02:33)
CO2: 26 mmol/L (02:33)
BUN: 48 mg/dL High (02:33)
Creatinine: 1.84 mg/dL High (02:33)
Glucose Level: 286 mg/dL High (02:33)
Calcium: 10.2 mg/dL High (02:33)
Mg Lvl: 2.5 mg/dL High (02:33)

Microbiology

Culture Respiratory **Stain**

Source:	Bronchial Lavage	Body Site:	
Collected Dt/Tm:	08/29/2024 13:08	Last Updated Dt/Tm:	08/29/2024 21:13

Progress Note - Physician

Rare epithelial cells
Many White Blood Cells
Many Gram Negative Rods
Rare Gram Positive Rods
Rare gram positive cocci in pairs
Rare Yeast

Culture Sputum		**Completed**	
Source:	Sputum, Suctioned	Body Site:	
Collected Dt/Tm:	08/28/2024 01:12	Last Updated Dt/Tm:	08/29/2024 16:52

Heavy Growth Mixed Upper Respiratory Flora

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	
Collected Dt/Tm:	08/27/2024 21:01	Last Updated Dt/Tm:	08/30/2024 02:01

No growth at 48 hours.

Cardiology Labs
NT-ProBNP: 1242 pg/mL High (08/27/24 20:57:00)
Troponin TNIH: 105.7 ng/L Critical (08/28/24 00:53:00)
Troponin TNIH: 63.6 ng/L (08/27/24 22:04:00)
Troponin TNIH: 44.1 ng/L (08/27/24 20:57:00)

Blood Gases (Last Within 24hrs)
pH Arterial: 7.47 High (15:09)
Art pO2: 84.9 mmHg (15:09)
Art pCO2: 43.6 mmHg (15:09)
HCO3 Art: 30.9 mmol/L High (15:09)
Art sO2: 97 % (15:09)

Assessment/Plan
Diagnoses
AKI (acute kidney injury) (N17.9)
Acute respiratory failure
Sepsis
COPD exacerbation
Congestive heart failure exacerbation
Hyperglycemia
Hypomagnesemia
Hyponatremia
Hypophosphatemia
Acute on chronic kidney disease
History congestive heart failure
History of COPD on 2 L nasal cannula

Progress Note - Physician

History hypertension
History of diabetes mellitus type 2
S/P L carotid endarterectomy with patch angioplasty 7/2024

Plan:
extubated
SVN treatments as needed
on rocephina and doxy
HTN urgency added labetalol, started on PO meds with metoprolol and amlodipine bp is still high
added doxazosin
awaiting FEES
Latest echocardiogram 7/2024 EF 60 to 65%
Continue to monitor and trend renal indicis
HR is high per pt going through withdrawals will restart subaxone
Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Home with family care

Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Home with family care

Electronically Signed By: Kabsoun, Sara MD
On: 08.30.2024 14:10 PDT

Electronically Signed On: 08.30.2024 19:48 PDT
Kabsoun, Sara MD

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	8/30/2024 10:22 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Al-owir MD,Bassam S (8/30/2024 10:25 PDT)
SIGN INFORMATION:	Al-owir MD,Bassam S (8/30/2024 10:25 PDT)

Garcia, A

Patient: **GARCIA, ARMANDO** **MRN: HHN8210503** **FIN: HHN0005007238115**
Age: **52 years** Sex: **Male** DOB: **6/2/1972**
Associated Diagnoses: **None**
Author: **Al-owir MD, Bassam S**

Plan
Impression and Plan: **Diagnosis**

Respiratory failure
COPD exacerbation
CHF exacerbation
Pneumonia

Progress Note - Physician

AKI
Hypophosphatemia
Hyperglycemia
Hypokalemia
HTN emergency
H/O DM , COPD, Carotid endarterectomy
CHF, and HTN

Plan:

extubated

O2

follow BAL

follow ABG`s

SVN

wean down steroids

ABX

IV Hydralazine

Amlodipine

Metoprolol

Close VS and rhythm monitoring

D/C Lasix

follow BUN/Cr

Strict I`s and O's

electrolytes per protocol

ACCU checks / ISS

DVT/GI prophylaxis

see orders
D/W RN, RT, and the pharmacist.
.

Objective
Intake and Output
Fluid Balance Primitives

8/30/2024 8:00 PDT	Stool Count	1
8/30/2024 6:00 PDT	Urine Catheter.	160 mL

Progress Note - Physician

8/30/2024 5:00 PDT	Urine Catheter.	130 mL
8/30/2024 4:00 PDT	Urine Catheter.	160 mL
8/30/2024 3:00 PDT	Urine Catheter.	140 mL
8/30/2024 2:00 PDT	Urine Catheter.	170 mL
8/30/2024 1:00 PDT	Urine Catheter.	110 mL
8/30/2024 0:00 PDT	Urine Catheter.	150 mL
8/29/2024 23:00 PDT	Urine Catheter.	150 mL
8/29/2024 22:00 PDT	Urine Catheter.	140 mL
8/29/2024 21:00 PDT	Urine Catheter.	120 mL
8/29/2024 20:00 PDT	Urine Catheter.	150 mL
8/29/2024 19:00 PDT	Urine Catheter.	140 mL
8/29/2024 18:00 PDT	Urine Catheter.	150 mL
8/29/2024 17:00 PDT	Urine Catheter.	200 mL

VS/Measurements
Inpt. Vital Signs (ST)

Vital Signs (24 hrs)	Last Charted	Minimum	Maximum
Temp	**H** 38.5	(AUG 30 04:00)	**H** 37.6 (AUG 29 19:00) **H** 38 (AUG 29 20:00)
Peripheral Pulse Rate	85 (AUG 30 07:53)	85 (AUG 30 07:53)	94 (AUG 30 01:37)
Resp Rate	20 (AUG 30 09:58)	14 (AUG 29 13:16)	**C** 35 (AUG 30 02:30)
SBP	**C** 184 (AUG 30 06:30)	134 (AUG 30 03:30)	**C** 204 (AUG 29 19:00)
DBP	**H** 86 (AUG 30 06:30)	63 (AUG 30 03:30)	**C** 94 (AUG 29 19:00)
SpO2	96 (AUG 30 07:51)	94 (AUG 29 11:00)	99 (AUG 29 13:00)
	on 5 L/min	on 30 %	
	Nasal cannula	Ventilator	

General: Alert and oriented.
HENT: Normocephalic, Oral mucosa is moist, Oropharynx Clear.
Neck: Supple, Non-tender, No carotid bruit, No jugular venous distention.
Respiratory: Breath sounds are equal, Symmetrical chest wall expansion, basilar ronchi.
Cardiovascular: Normal rate, Regular rhythm, No murmur.
Gastrointestinal: Soft, Non-tender, Non-distended, Normal bowel sounds.

Health Status
Allergies:

Allergies (2) Active	Reaction
No Known Allergies	None Documented
No Known Medication Allergies	None Documented

Current medications: Inpatient medications (ST)
Medications (22) Active
SCHEDULED: (18)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
aspirin 81 mg Chew Tab (aspirin 81 mg oral tablet, chewable) 81 mg 1 Tabs, OG-Tube, Daily
atorvastatin 40 mg Tab (Lipitor 40 mg oral tablet) 40 mg 1 Tabs, OG-Tube, qHS
cefTRIAXone 2 gm vial (Rocephin 2 g injection) 2 gm 20 mL, IV Push, qHS
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily

Progress Note - Physician

doxycycline + Dextrose 5% in Water 100 mL (doxycycline + Dextrose 5% in Water intravenous solution 100 mL) 100 mg 10 mL, IV Piggyback, BID
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
furosemide 40 mg/4 mL vial (furosemide 10 mg/mL injectable solution) 40 mg 4 mL, IV Push, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin lispro HumaLOG 1 unit/0.01mL syringe (insulin lispro HumaLOG correctional scale High) 4-18 Units, SubCutaneous, q4H Interval
methylPREDNISolone sod succinate 40 mg Vial PF (methylPREDNISolone sodium succinate (Solu-Medrol)) 40 mg 1 mL, IV Push, q12H Interval
pantoprazole 40 mg vial (Protonix) 40 mg, IV Push, Daily
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
CONTINUOUS: (1)
dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL (dexmedeTOMIDine IV additive 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL) 100 mL, IV, 12.93 mL/hr
PRN: (3)
acetaminophen 650 mg/20.3 mL Oral Liq (Tylenol Liquid) 650 mg 20.3 mL, OG-Tube, q6H
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
morphine 4 mg/mL injection 1 mL (morphine) 4 mg 1 mL, IV Push, q4H

Problem list:
Active Problems (12)
Acute pain
At risk for falls
At risk for injury due to fall
Bowel dysfunction
Disc disorder
DM (diabetes mellitus)
ESBL Escherichia coli
Gout
HLD (hyperlipidemia)
HTN (hypertension)
Ineffective airway clearance
Sleep apnea

Assessment
Interpretation of Results
Laboratory: Fishbone Labs ST
AUG 30 02:33

141 **| H 110 | H 48 /**
_____ **H 286**

3.9 | 26 **| H 1.840 **

AUG 30 02:33

**** 14.6 **/**
11.93 _____ 148

Progress Note - Physician

/ 47.4 **\.**

Electronically Signed By: Al-owir, Bassam MD
On: 08.30.2024 10:25 PDT

Orders

Order: **Consult to Case Management (Case Management Consult)**			
Order Date/Time: 9/1/2024 08:33 PDT			
Signed Date/Time: 9/1/2024 08:33 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 9/1/2024 08:33 PDT		End-state Reason:	
Ordering Physician: Moreno MD,Luis A		Consulting Physician:	
Entered By: Moreno MD,Luis A on 9/1/2024 08:33 PDT			
Order Details: 9/1/24 8:33:00 AM PDT, Routine, Reason for Consult: D/C to Home w Home Health, IV: Yes, O2: Yes, DM education and insulin management education, BP and renal function monitoring, RN for safety evaluation, medication reconciliation,, PT/OT for balance and strengthening			
Order Comment:			

Order: **Consult to Case Management**			
Order Date/Time: 8/31/2024 15:07 PDT			
Signed Date/Time: 8/31/2024 15:07 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 8/31/2024 15:07 PDT		End-state Reason:	
Ordering Physician: Moreno MD,Luis A		Consulting Physician:	
Entered By: Moreno MD,Luis A on 8/31/2024 15:07 PDT			
Order Details: 8/31/24 3:07:00 PM PDT, Routine, Reason for Consult: Other Please Specify, IV: Yes, O2: Yes, No Isolation, Patient will need continuous home oxygen 2 L NC, Please provide oxygen tank and concentrator. Dx: COPD, hypoxia NPI: 1649839259 LON: 99			
Order Comment:			

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Orders - Admission/Discharge (Curaspan)

Order: **Placement Change**			
Order Date/Time: 8/30/2024 10:26 PDT			
Signed Date/Time: 8/30/2024 10:26 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 8/30/2024 10:26 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 8/30/2024 10:26 PDT			
Order Details: 8/30/24 10:26:45 AM PDT, Suggested Placement: IMC			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			
Action Type: Order	Action Date/Time: 8/30/2024 10:26 PDT	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 08/30/24 10:26:45 PDT, Suggested Placement: IMC			
Review Information: Nurse Review: Electronically Signed, Novak RN,Shayla on 8/30/2024 11:09 PDT Doctor Cosign: Not Required			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Order: **Discharge Request Task Order**			
Order Date/Time: 9/1/2024 08:33 PDT			
Signed Date/Time: 9/1/2024 08:33 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 9/1/2024 08:33 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 9/1/2024 08:33 PDT			
Order Details: 9/1/24 8:33:00 AM PDT, Home with Home Health			
Order Comment:			
Action Type: Order	Action Date/Time: 9/1/2024 08:33 PDT	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 09/01/24 8:33:00 PDT, Home with Home Health			
Review Information: Doctor Cosign: Not Required			
Order Comment:			

Orders - Admission/Discharge (Curaspan)

Order: **Discharge Request**			
Order Date/Time: 9/1/2024 08:32 PDT			
Signed Date/Time: 9/1/2024 08:32 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 9/1/2024 08:32 PDT		End-state Reason:	
Ordering Physician: Moreno MD,Luis A		Consulting Physician:	
Entered By: Moreno MD,Luis A on 9/1/2024 08:32 PDT			
Order Details: 9/1/24 8:32:00 AM PDT, Home with Home Health			
Order Comment:			

Action Type: Order	Action Date/Time: 9/1/2024 08:32 PDT	Action Personnel: Moreno MD,Luis A
Responsible Provider/Electronically Signed by: Moreno MD,Luis A	Supervising Provider:	Communication Type: Written
Order Details: 09/01/24 8:32:00 PDT, Home with Home Health		
Review Information: Nurse Review: Electronically Signed, Ly RN,Patrick on 9/1/2024 08:54 PDT Doctor Cosign: Not Required		
Order Comment:		

Order: **Change Attending Physician**			
Order Date/Time: 8/30/2024 10:26 PDT			
Signed Date/Time: 8/30/2024 10:26 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 8/30/2024 10:39 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 8/30/2024 10:26 PDT			
Order Details: 8/30/24 10:26:45 AM PDT, Admitting: Kabsoun MD, Sara H, Attending: Kabsoun MD, Sara H			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 8/30/2024 10:39 PDT	Action Personnel: Rivera,Lydia
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 08/30/24 10:26:45 PDT, Admitting: Kabsoun MD, Sara H, Attending: Kabsoun MD, Sara H		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 8/30/2024 10:26 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 08/30/24 10:26:45 PDT, Admitting: Kabsoun MD, Sara H, Attending: Kabsoun MD, Sara H		
Review Information: Nurse Review: Not Reviewed Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Orders - Admission/Discharge (Curaspan)

Order: **Case Management Discharge Notification**			
Order Date/Time: 9/1/2024 08:33 PDT			
Signed Date/Time: 9/1/2024 08:33 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 9/1/2024 08:33 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 9/1/2024 08:33 PDT			
Order Details: 9/1/24 8:33:00 AM PDT			
Order Comment: Home with Home Health			

Action Type: Order	Action Date/Time: 9/1/2024 08:33 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 09/01/24 8:33:00 PDT		
Review Information: Doctor Cosign: Not Required		
Order Comment: Home with Home Health		

Physical Therapy Forms

DOCUMENT TYPE:	PT Daily Documentation - Text
SERVICE DATE/TIME:	9/1/2024 08:34 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Coulson PT,DPT,Jeremy (9/1/2024 09:14 PDT)
SIGNED INFORMATION:	Coulson PT,DPT,Jeremy (9/1/2024 09:14 PDT)

PT Daily Documentation Entered On: 9/1/2024 9:17 PDT
Performed On: 9/1/2024 8:34 PDT by Coulson PT, DPT, Jeremy

Review/Treatments Provided
PT Goals : PT Short Term Goals

08/30/2024
Other PT Goal #1: Independent in bed mobility; Initial
Other PT Goal #2: Close supervision to transfer bed to chair; Initial
Other PT Goal #3: Close supervision to ambulate 300' with a SPC; Initial
PT Plan : Treatment Frequency: Other: 3-7 x week Performed By: Coulson PT, DPT, Jeremy 08/30/2024
Treatment Duration: 5-6 weeks Performed By: Coulson PT, DPT, Jeremy 08/30/2024
Planned Treatments: Balance training, Bed mobility training, Gait training, Therapeutic exercises, Transfer training Performed By: Coulson PT, DPT, Jeremy 08/30/2024
Physical Therapy Orders : Miscellaneous Physical Therapy Task - 08/30/24 7:23:00 PDT, Routine, Stop date 08/30/24 7:23:00 PDT, Stretcher, Yes, Yes, 3-7 x week for 6 weeks
Pain Symptoms : No
Suspected Stroke Patient : No

Physical Therapy Forms

Post-Acute Discharge Therapy Recommendation PT : No further skilled therapy services warranted
IDR PT Recommend/Goal : Safe for home DC from PT standpoint

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Mobility

Mobility Grid
Roll Left : Rehab Complete independence
Roll Right : Rehab Complete independence
Supine to Sit : Rehab Complete independence
Sit to Supine : Rehab Complete independence
Transfer Sit to Stand : Close supervision
Transfer Stand to Sit : Close supervision
Transfer Bed to and From Chair : Close supervision
Transfer Toilet : Close supervision
Walk 10 ft : Close supervision
Walk 50 ft, 2 Turns : Close supervision
Walk 150 ft : Close supervision

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Balance

General Balance : Both
General Balance Assessment : Good

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Education

Teaching Method : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Physical Therapy Education Grid
Ambulation : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Additional Information

Additional Information PT : Patient lying in bed on entry, ready for PT. eager to use toilet. Independent in bed mobility, supervision to stand, ambulate 15' to toilet, transfer on and off of it, then ambulate 300' further in hallway with no device. Patient with slow, shuffle, kyphotic gait. Cued for step length, gait speed, upright posture. Saturating mid 90's on 6L oxygen. Patient returned to room and left in recliner with call light
Environmental Safety Implemented : Bed in low position, Call device within reach, Lock wheeled equipment, Personal items within reach

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

PT Non Timed Modalities 2024

PT Treatment Start Time : 9/1/2024 8:34 PDT
PT Treatment End Time : 9/1/2024 9:12 PDT

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

PT Timed Modalities 2024

PT Timed Modality Treatment Start Time : 9/1/2024 8:34 PDT
PT Timed Modality Treatment End Time : 9/1/2024 9:12 PDT
Rehab Patient : No
PT Timed Min Total : 38
PT Timed Unit Total : 3

Physical Therapy Forms

PT THERAPEUTIC ACT 15MIN : 8
Activity Therapeutic Num of Units : 1
PT GAIT TRAINING 15MIN : 30
Gait Training Num of Units : 2

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

DOCUMENT TYPE:	PT Examination - Text
SERVICE DATE/TIME:	8/30/2024 06:30 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Coulson PT,DPT,Jeremy (8/30/2024 07:15 PDT)
SIGNED INFORMATION:	Coulson PT,DPT,Jeremy (8/30/2024 07:15 PDT)

PT Evaluation/Reevaluation Entered On: 8/30/2024 7:23 PDT
Performed On: 8/30/2024 6:30 PDT by Coulson PT, DPT, Jeremy

General Info
PT Assessment Type : Initial Evaluation
Physical Therapy Orders : PT Evaluation and Treatment - 08/29/24 19:31:00 PDT, Routine, Stop date 08/29/24 19:31:00 PDT, Stretcher, Yes, Yes
Precautions RTF : Active Orders:
» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Ordered
» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered

Discontinued Orders:
» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued
» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled
» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed
» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued
» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued
» Isolation, Precautions: COVID-19, Discontinued

Orientation Assessment Rehab : Oriented x 4
Affect/Behavior Rehab : Appropriate
Basic Command Following : Intact
Safety/Judgment : Intact
Pain Symptoms : No

Physical Therapy Forms

Suspected Stroke Patient : No

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Home Environment
Living Environment : Home Environment

No qualifying data available
Living Situation : Home independently
Lives With : Spouse
Lives In : Single level home

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Location Bed
Primary Bedroom : 1st floor
Primary Bathroom : 1st floor
Kitchen : 1st floor
Laundry : 1st floor

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Patient's Responsibilities Rehab : Community mobility, Personal ADL, Shopping

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Home Environment II
Living Environment : Home Environment

No qualifying data available
Home Equipment Rehab : Cane

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Prior Functional Status Grid
ADL : Independent
Mobility : Independent
Instrumental ADL : Independent
Cognitive-Communication Skills : Independent

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

LE Range/Strength
LE Overall Range of Motion Grid
Left Lower Extremity Passive Range : Within functional limits
Left Lower Extremity Active Range : Within functional limits
Right Lower Extremity Passive Range : Within functional limits
Right Lower Extremity Active Range : Within functional limits

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Left Lower Extremity Strength Grid
Hip Flexion : 4
Hip Extension : 4
Hip Abduction : 4
Hip Adduction : 4
Hip External Rotation : 4
Hip Internal Rotation : 4
Knee Flexion : 4
Knee Extension : 4
Ankle Dorsiflexion : 4
Ankle Plantarflexion : 4
Ankle Eversion : 4

Physical Therapy Forms

Ankle Inversion : 4
Great Toe Extension : 4

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Right Lower Extremity Strength Grid
Hip Flexion : 4
Hip Extension : 4
Hip Abduction : 4
Hip Adduction : 4
Hip External Rotation : 4
Hip Internal Rotation : 4
Knee Flexion : 4
Knee Extension : 4
Ankle Dorsiflexion : 4
Ankle Plantarflexion : 4
Ankle Eversion : 4
Ankle Inversion : 4
Great Toe Extension : 4

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

LE/Trunk Tone
remainder (less than 1/2 of ROM)
Left Lower Extremity : Normal
Right Lower Extremity : Normal
Trunk Tone : Normal

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Balance
General Balance : Both
General Balance Assessment : Fair

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Mobility
Mobility Grid
Roll Left : Rehab Contact guard assistance
Roll Right : Rehab Contact guard assistance
Supine to Sit : Rehab Contact guard assistance
Sit to Supine : Rehab Contact guard assistance
Transfer Sit to Stand : Rehab Contact guard assistance
Transfer Stand to Sit : Rehab Contact guard assistance
Transfer Bed to and From Chair : Rehab Contact guard assistance
Walk 10 ft : Rehab Contact guard assistance
Walk 50 ft, 2 Turns : Rehab Contact guard assistance

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Assessment
PT Impairments or Limitations : Ambulation deficits, Balance deficits, Bed mobility deficits, Endurance deficits, Strength deficits, Transfer deficits

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Education
Teaching Method : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Physical Therapy Forms

Physical Therapy Education Grid

Ambulation : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Bed Mobility : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Exercise Program : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Physical Therapy Plan of Care : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Long Term Goals

PT Patient,Caregiver Goal : Go home when able

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

PT Long Term Goals Rehab

	Long Term Goal 1	Long Term Goal 2
Goal :	Independent to transfer bed to chair	Independent to ambulate 900'
Status :	Initial	Initial
Timeframe To Complete :	6 weeks	6 weeks
	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Short Term Goals

Other PT Goals Grid

	Goal #1	Goal #2	Goal #3
Goal :	Independent in bed mobility	Close supervision to transfer bed to chair	Close supervision to ambulate 300' with a SPC
Status :	Initial	Initial	Initial
Timeframe To Complete :	1 week	1 week	1 week
	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Plan

PT Anticipated Treatments, Needs : Balance training, Bed mobility training, Gait training, Therapeutic exercises, Transfer training
PT Frequency Rehab : Other: 3-7 x week
PT Duration Rehab : 5-6 weeks
PT Intensity Per Day : 0-30 minutes

Physical Therapy Forms

Rehabilitation Potential : Good
Post-Acute Discharge Therapy Recommendation PT : No further skilled therapy services warranted
IDR PT Recommend/Goal : Safe for home DC from PT standpoint, will progress as able
Other PT Treatment Provided : After PT eval patient transferred to a recliner and sat x 5 min to catch his breath, Noted to be saturating at 93% on 6L. Paitent CGA to stand and ambulate 50' back to room due to weakness, slow, shuffle, kyphotic gait with SOB and balance deficits. Cued for step length, gait speed, upright posture. Left in recliner with lap belt and call light. RN present with handoff given
PT Plan/Goals Established w Pt/Caregiver : Yes

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Additional Information
Additional Information PT : MD orders, chart reviewed. Pre/post handoff performed w/ RN and informed of patient participation in therapy. Patient ID verified via 2 identifiers (Name, DOB). On arrival patient semisupine in bed, alert, oriented x 4, coherent, and cooperative. Patient agreed to PT evaluation and POC. Patient CGA in all functional mobility and is able to sit, stand, transfer in and out of chair and ambulate 50' with no device and only slow, shuffle, kyphotic, unsteady gait with slight SOB. Patient will benefit from PT to address bed mobility, transfer and gait deficits. After PT Evaluation and POC discussion patient left semisupine in bed, bed alarm on, CLIr, all needs met.

Dx: Patient is a 52 year old male admitted with cardiac arrest

PMH: DM, CHF, COPD, HTN, carotid endartectomy

PLOF: Modified independent in community ambulation

DME: uses a SPC
DC recs: home with family
Environmental Safety Implemented : Bed in low position, Call device within reach, Lock wheeled equipment, Personal items within reach

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

PT Non Timed Modalities 2024
PT Treatment Start Time : 8/30/2024 6:30 PDT
PT Treatment End Time : 8/30/2024 6:44 PDT
PT Eval High Complexity Total Number Min : 1
PT Total Non Timed : 1

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

PT Timed Modalities 2024
PT Timed Modality Treatment Start Time : 8/30/2024 6:45 PDT
PT Timed Modality Treatment End Time : 8/30/2024 7:10 PDT
Rehab Patient : No
PT Timed Min Total : 25
PT Timed Unit Total : 2
PT GAIT TRAINING 15MIN : 25
Gait Training Num of Units : 2

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Speech Therapy

DOCUMENT TYPE:	Clinical Swallowing - Text
SERVICE DATE/TIME:	8/30/2024 10:49 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 11:28 PDT)
SIGNED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 11:28 PDT)

ST Bedside Swallow Eval Entered On: 8/30/2024 11:32 PDT
Performed On: 8/30/2024 10:49 PDT by Castilo CCC-SLP, Noreen ann

General Information
Speech Orders : ST Bedside Swallow Exam - 08/29/24 19:32:00 PDT, Bedside Swallow Exam, Routine, Stop date 08/29/24 19:32:00 PDT, Stretcher, IV Yes, O2 Yes, 17433508165.0
Speech Therapy Eval and Treat - 08/29/24 19:32:00 PDT, Bedside Swallow Exam, Routine, Stop date 08/29/24 19:32:00 PDT, Stretcher, IV Yes, O2 Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered

Discontinued Orders:
» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Deleted
» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued
» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled
» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed
» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued
» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued
» Isolation, Precautions: COVID-19, Discontinued

History of Swallow Studies : No qualifying data available
Level of Consciousness Rehab : Alert
Affect/Behavior Rehab : Cooperative
Basic Command Following : Intact
Orientation Assessment Rehab : Identifies self
Pain Interfering With SLP Treatment : No
Culture/Spiritual Beliefs to Incorporate : No

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Pre-Assessment
Oxygen Therapy : Nasal cannula

Speech Therapy

Tracheostomy Information : Oxygen Flow Rate: 5 L/min Performed by: Mower, Vanessa 08/30/24 07:51:00
FIO2: 30 Performed by: Stansfield RN, Erica 08/29/24 16:00:00

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Oral-Facial Exam
Facial Movement : Symmetric resting/crying
Dentition : Edentulous

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Oral Structure Grid
Labial Structure : Within functional limits
Mandible : Within functional limits
Lingual Structure : Within functional limits
Hard Palate : Within functional limits
Soft Palate : Within functional limits

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Oral Function Grid
Labial : Within functional limits
Mandible : Within functional limits
Lingual : Within functional limits
Velopharyngeal : Within functional limits

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Secretion Management : Reduced

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Upper Body Control Grid
Head Control : Upright centered position
Trunk Control : Upright centered position

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Swallowing Exam
Patient Position : Upright 90 degrees

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Liquids Swallowing Exam Adult Grid

	Ice Chips	Thin Liquid by Teaspoon	Thin Liquid by Straw
Bolus Size :		5 ml	
Total Intake :	Other: 5x	Other: 5x	3-4 ounces
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist	Speech-Language Pathologist
Oral Symptoms :	None observed	None observed	None observed
Pharyngeal Symptoms :	None observed	None observed	Cough/Throat clear after swallow
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Consistency Swallowing Exam Adult Grid

Speech Therapy

	Puree Consistency	Regular Consistency
Bolus Size :	5 ml	
Total Intake :	1-2 ounces	Other: one quarter graham cracker
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist
Oral Symptoms :	None observed	Prolonged mastication
Pharyngeal Symptoms :	Wet vocal quality after swallow	Wet vocal quality after swallow
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Assessment/Recommendations
Clinical Swallowing Impression Grid
Oral Dysphagia : Liquids, Solids
Pharyngeal Dysphagia : Liquids, Solids

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

NPO Status : Except for ice chips
Clinical Swallowing Recommendations : Dysphagia treatment
Assessment Recommendations ST Daily : Chart reviewed, consulted with Shayla RN- cleared pt for evaluation. Swallow evaluation indicated d/t recent extubation. Pt Identified via name and DOB. Pt is agreeable to evaluation. Pt is a 52 year old male to Henderson Hospital for SOB at home.
Dx: intubated 08/28-08/29; COPD exacerbation, SOB, sepsis, O2 dependent, resp distress, resp failure, elevate BNP, hypomagnesemia, hyponatremia, AKI, hypophosphatemia
Hyperglycemia d/t DM, leukocytosis, fever, HTN emergency, morbid obesity, CHF exacerbation, bil lower extremity edema, s/p left endarterectomy, elevated lactic acid, elevated CRP, PUI COVID; bronchoscopy 8/29 - mod to sev bil inflammation, mod bil white thick secretions, high position ET tube.
Significant hx includes but not limited to DM, HTN, diastolic CHF, COPD, hyperlipidemia, disc disorder, ESBL ecoli, gout, HLD, HTN, bil ICA stenosis s/p endarterectomy, EGD w/ biopsy 06/2022
Home Diet: Regular consistencies
Current Diet: NPO
WBC: WNL
CXR: 8/30 2. Low lung volumes with bronchovascular crowding. Bibasilar airspace disease and/or atelectasis.
CT Head: 8/28 no acute intracranial abnormality

IMPRESSION:
Severe oropharyngeal dysphagia characterized by s/s suspected aspiration, observed continued wet vocal quality t/o this evaluation, pt reported r/t neck surgery a month ago and was told there will be swelling. However noted reduced O2 sat during swallow completion with PO trials. See Swallowing Exam Section for details.

Speech Therapy

RECOMMENDATIONS:
NPO, except ice chips with aspiration precautions and oral care TID to reduce risk of aspiration PNA
Alternative means nutrition, hydration, and medication-Consider NGT.
FEES to fully evaluate swallow function

ST to treat for dysphagia
Results and recommendations discussed with: Shayla RN
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Education
Learner(s) Present for Session : Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Document learning evaluation for Responsible Learner(s)
Speech Language/Pathology Tx Plan : Verbalizes understanding
Swallowing Strategies : Verbalizes understanding

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Long Term Goals
SLP Patient, Caregiver Goals : "to eat food"

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will tolerate least restrictive consistencies with risk of aspiration reduced as much as reasonably possible.
Status :	Initial
Timeframe To Complete :	9/13/24
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Short Term Goals
Other SLP Short Term Goal Grid

	Goal #1	Goal #2
Other :	Pt will tolerate 90ml challenge	Pt will perform compensatory

Speech Therapy

	vs instrumental swallow without uncompensated aspiration or penetration on thin liquids	strategies to protect the airway and improve clearance of the bolus in 80% of trials independently
Status :	Initial	Initial
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Plan
Planned Treatments : Dysphagia therapy
Frequency : 3-5 times per week
Duration : 1-2 weeks
SLP Intensity Per Day : Other: as tolerated
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
Rehabilitation Potential : Good
Plan/Goals Established With Patient/Caregiver : Yes
Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Additional Information
Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach
Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

ST Non Timed Modalities 2024
ST Treatment Start Time : 8/30/2024 10:49 PDT
ST Treatment End Time : 8/30/2024 10:59 PDT
ST Eval Oral/Swallow Func Minutes : 10
ST Total Non Timed : 10
Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

DOCUMENT TYPE:	Inpatient SLP Daily Documentation - Text
SERVICE DATE/TIME:	8/31/2024 14:02 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/31/2024 15:27 PDT)
SIGNED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/31/2024 15:27 PDT)

ST Daily Documentation Entered On: 8/31/2024 15:28 PDT
Performed On: 8/31/2024 14:02 PDT by Castilo CCC-SLP, Noreen ann

Review/Treatments Provided

Speech Therapy

Speech Orders : Miscellaneous Speech Therapy Task - 08/30/24 11:32:00 PDT, Routine, Stop date 08/30/24 11:32:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes
Speech Language Plan : Treatment Frequency: 2-4 times per week (modified) Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
Treatment Duration: 1-2 weeks (modified) Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
Planned Treatments: Dysphagia therapy (modified) Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
Short Term Goals Reviewed : Yes
Long Term Goals Reviewed : Yes
Speech Language Goals RTF : SLP Short Term Goals

08/30/2024
Other Short Term Goals Goal #2: Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently (modified); Progressing, continue (modified)
Other Short Term Goals Goal #3: Pt will tolerate regular PO trials with no overt s/s of apsiration in 5 out o5 trials (modified); Initial (modified)
Pain Interfering With SLP Treatment : No

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Short Term Goals

Other SLP Short Term Goal Grid

	Goal #1	Goal #2	Goal #3
Other :	Pt will tolerate 90ml challenge vs instrumental swallow without uncompensated aspiration or penetration on thin liquids	Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently	Pt will tolerate regular PO trials with no overt s/s of apsiration in 5 out o5 trials
Status :	Goal met	Progressing, continue	Progressing, continue
Date Met :	8/30/2024 PDT		
	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Long Term Goals

SLP Patient, Caregiver Goals : "to eat food"

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will tolerate least restrictive consistencies

Speech Therapy

	with risk of aspiration reduced as much as reasonably possible.
Status :	Progressing, continue
Timeframe To Complete :	9/13/24
	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Education
Learner(s) Present for Session : Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Speech Education Grid
Speech Language/Pathology Tx Plan : Verbalizes understanding
Swallowing Strategies : Verbalizes understanding, Needs further teaching

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Assessment/Recommendations
Assessment Recommendations ST Daily : S: Chart reviewed, consulted with Alejandra RN- cleared pt for tx. Pt alert, agreeable treatment. Clinician discussed with Alejandra RN re: FEES findings with images in document.

O: Ordered textures: IDDSI Soft and bite size with thin liquids; WBC: WNL; CXR: 8/30 1. Interval removal of the endotracheal tube and enteric tube. 2. Low lung volumes with bronchovascular crowding. Bibasilar airspace disease and/or atelectasis.
PO trials:
Thin liquids via straw 1-3oz: no overt s/s of aspiration
Regular graham cracker one half piece: prolonged mastication, no overt s/s of aspiration, independent use of liquid wash
Compensatory/safe swallow strategies: upright in bed, small bites/sips, slow rate for PO intake, swallow two times
Education: s/s of aspiration, swallowing strategies, POC. Pt verbalized understanding and had no further questions at this time. See Also Education Grid

A: pt continues to exhibit oropharyngeal dysphagia r/t FEES on 08/30/24 noted continued wet gurgly voice prior to and after PO intake not r/t aspiration concerns at this time

P: Recommendations
Food Consistencies: IDDSI 6 Soft and Bite Sized
Liquid Consistencies: Thin Liquids
Meds: Whole with puree

Speech Therapy

Supervision/Assist: Distant supervision needed for feeding (provided initial cueing to initiate swallow strategies check on the patient frequently and assess the patient progress at least two to three times during the meal)

Aspiration precautions:
Sit upright as close to 90 degrees as possible,
remain upright 30 mins after meals
Small bites/sips
Swallow two times
Alternate solids/liquids

ST to f/u per POC
Consider further imaging of neck tissue
Consider f/u ENT outpatient when feasible
Discussed with: Alejandra RN

Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Additional Information

Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

ST Non Timed Modalities 2024

ST Treatment Start Time : 8/31/2024 14:02 PDT
ST Treatment End Time : 8/31/2024 14:11 PDT
ST TREATMENT FOR FEED Minutes : 9
ST Total Non Timed : 9

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

DOCUMENT TYPE:	FEES - Text
SERVICE DATE/TIME:	8/30/2024 12:58 PDT
RESULT STATUS:	Modified
PERFORMED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 14:29 PDT)
SIGNED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 14:39 PDT); Castilo CCC-SLP,Noreen ann (8/30/2024 14:29 PDT)

ST FEES Entered On: 8/30/2024 14:39 PDT
Performed On: 8/30/2024 12:58 PDT by Castilo CCC-SLP, Noreen ann

General Information

Speech Orders : FEES - 08/30/24 11:33:00 PDT, Routine, Stop date 08/30/24 11:33:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes
Miscellaneous Speech Therapy Task - 08/30/24 11:32:00 PDT, Routine, Stop date 08/30/24 11:32:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes

Speech Therapy

~~{ [FEES - 08/30/24 11:33:00 PDT, Routine, Stop date 08/30/24 11:33:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes~~
~~Miscellaneous Speech Therapy Task - 08/30/24 11:32:00 PDT, Routine, Stop date 08/30/24 11:32:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes~~
~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Precautions : Active Orders:
» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered

Discontinued Orders:
» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Deleted
» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued
» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled
» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed
» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued
» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued
» Isolation, Precautions: COVID-19, Discontinued

~~{ [Active Orders:~~
~~» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered~~

~~Discontinued Orders:~~
~~» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Deleted~~
~~» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued~~
~~» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled~~
~~» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed~~
~~» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued~~
~~» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued~~
~~» Isolation, Precautions: COVID-19, Discontinued~~

~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

History of Swallow Studies : Clinical Swallowing Recommendations: Dysphagia treatment Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024

~~{ [Clinical Swallowing Recommendations: Dysphagia treatment Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Level of Consciousness Rehab : Alert

~~{ [Alert] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Affect/Behavior Rehab : Cooperative

~~{ [Cooperative] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Basic Command Following : Intact

~~{ [Intact] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Orientation Assessment Rehab : Identifies self

~~{ [Identifies self] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Pain Interfering With Session : No

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~~{ [No] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Pre-Assessment

Oxygen Therapy : Nasal cannula

~~{ [Nasal cannula] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Tracheostomy Information : Oxygen Flow Rate: 2 L/min Performed by: Novak RN, Shayla 08/30/24 12:00:00
FIO2: 30 Performed by: Stansfield RN, Erica 08/29/24 16:00:00

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

~~{ [Oxygen Flow Rate: 2 L/min Performed by: Novak RN, Shayla 08/30/24 12:00:00
FIO2: 30 Performed by: Stansfield RN, Erica 08/29/24 16:00:00
] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Anatomic and Structural Assessment

Dentition : Edentulous

~~{ [Edentulous] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Labial : Within normal limits

~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Lingual : Within functional limits

~~{ [Within functional limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Mandible : Within normal limits

~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Velopharyngeal : Within functional limits

~~{ [Within functional limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Base of Tongue Retraction : Reduced

~~{ [Reduced] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Glottic Gap : Within normal limits

~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Aryepiglottice Rim : Lesion, Other: ecchymosis at left side

~~{ [Lesion, Other: ecchymosis at left side] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Arytenoids : Other: left arytenoid reducced adduction

~~{ [Other: left arytenoid reducced adduction] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

Aryepiglottic Folds Touch : Not tested
~~{ [Not tested] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
True Vocal Folds : Other: left fixed at paramedian position
~~{ [Other: left fixed at paramedian position] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
False Focal Folds : Other: left fixed at perimedian position
~~{ [Other: left fixed at perimedian position] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Airway Protection : Incomplete TVC Closure, Incomplete FVC Closure
~~{ [Incomplete TVC Closure, Incomplete FVC Closure] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vocal Fold Closure/Arytenoid Mobility : Reduced Adduction, Assymetry
~~{ [Reduced Adduction, Assymetry] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Secretions : Foamy, Clear
~~{ [Foamy, Clear] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pharyngeal Walls : Other: left ecchymosis ?lesion
~~{ [Other: left ecchymosis ?lesion] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Endoscopic Exam Comments : visualized left base of tongue, anterior epiglottic fold, and left aeryepiglottic fold ecchomysis ?lesion
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [visualized left base of tongue, anterior epiglottic fold, and left aeryepiglottic fold ecchomysis ?lesion] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Texture Trials

Patient Position Texture Trials : Upright at 90 degrees
~~{ [Upright at 90 degrees] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Factors Affecting Performance : No Difficulties Participating in Study
~~{ [No Difficulties Participating in Study] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Scope Passed : Right Nare
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Right Nare] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Ice Chips Texture Trials

	Ice Chips
Texture Trials :	Single ice chips Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Single ice chips] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

Oral Phase :	Within normal limits Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To pyriforms Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To pyriforms] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Onset of Swallow :	Normal (0-2 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Normal (0-2 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	III Mild: 5-25% epiglottic ligament visible Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [III Mild: 5-25% epiglottic~~

Speech Therapy

	~~ligament visible] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	Lateral channels Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Lateral channels] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP,~~

Speech Therapy

	~~Noreen ann at 8/30/2024 14:29 PDT};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Thin Liquid Texture Trials

	Thin Liquid
Texture Trials :	Tsp, Straw Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Tsp, Straw] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Oral Phase :	Within normal limits Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To vallecular Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To vallecular] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

Onset of Swallow :	Normal (0-2 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Normal (0-2 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	Lateral channels Castilo CCC-SLP,

Speech Therapy

	Noreen ann - 8/30/2024 14:39 PDT ~~{ [Lateral channels] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Pureed Texture Trials

	Pureed
Texture Trials :	Via tsp Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Via tsp] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Oral Phase :	Within normal limits Castilo CCC-SLP, Noreen ann -

Speech Therapy

	8/30/2024 14:39 PDT ~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To vallecular Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To vallecular] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Onset of Swallow :	Mild delay (3-5 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Mild delay (3-5 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	III Mild: 5-25% epiglottic ligament visible, IV Moderate: 25-50% epiglottic ligament covered Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [III Mild: 5-25%~~

Speech Therapy

	~~epiglottic ligament visible, IV Moderate: 25-50% epiglottic ligament covered] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	Lateral channels Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Lateral channels] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1.~~

Speech Therapy

	~~No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Compensatory Strategies Trialed :	Yes Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT (Comment: liquid wash moderately effective in clearing residue consecutive swallow moderately effective in clearing residue [Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT]) ~~{ [Yes] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT [Comment] liquid wash moderately effective in clearing residue consecutive swallow moderately effective in clearing residue - [Castilo CCC-SLP, Noreen ann at~~

Speech Therapy

	~~8/30/2024 14:29 PDT])};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Mixed Consistency Texture Trials

	Mixed Consistency
Texture Trials :	Total trials Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT (Comment: regular graham cracker [Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT]) ~~{ [Total trials] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT [Comment] regular graham cracker - [Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT])};~~
Oral Phase :	Reduced mastication Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Reduced mastication] - previously charted by~~

Speech Therapy

	~~Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To pyriforms Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To pyriforms] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Onset of Swallow :	Mild delay (3-5 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Mild delay (3-5 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	III Mild: 5-25% epiglottic ligament visible Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [III Mild: 5-25% epiglottic ligament visible] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

Pyriform Residue :	I None: 0% or no residue Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [I None: 0% or no residue] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	None Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [None] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Compensatory Strategies Trialed :	Yes Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT (Comment: liquid wash moderately effective in

Speech Therapy

	clearing residue consecutive swallow moderately effective in clearing residue [Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT]) ~~{ [Yes] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT [Comment] liquid wash moderately effective in clearing residue consecutive swallow moderately effective in clearing residue - [Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT])};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Assessment

swallowing impression

Oral Dysphagia : Liquids, Solids

~~{ [Liquids, Solids] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Pharyngeal Dysphagia : Liquids, Solids

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

~~{ [Liquids, Solids] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Contributing Factors to Dysphagia : Reduced mastication, Delayed initiation of swallow, Reduced base of tongue retraction

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

Speech Therapy

~~{ [Reduced mastication, Delayed initiation of swallow, Reduced base of tongue retraction] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Recommendations

Diet Consistency Recommendation : IDDSI 6 Soft and Bite Sized
~~{ [IDDSI 6 Soft and Bite Sized] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Liquid Viscosity Recommendation : Thin
~~{ [Thin] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Medications Swallow Recommendation : Whole in food
~~{ [Whole in food] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Supervision Swallow Recommendation : Intermittent
~~{ [Intermittent] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Positioning Swallow Recommendation : Upright 90 degrees
~~{ [Upright 90 degrees] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Endo Swallow Recommendations : Dysphagia treatment
~~{ [Dysphagia treatment] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Assessment Recommendations ST Daily : Subjective/Patient Report:
Chart reviewed, consulted with Shayla RN- cleared pt for evaluation. Swallow evaluation indicated d/t recent extubation. Pt Identified via name and DOB. Pt is agreeable to evaluation. Family (wife) present at bedside for education. Pt is a 52 year old male to Henderson Hospital for SOB at home.
Dx: intubated 08/28-08/29; COPD exacerbation, SOB, sepsis, O2 dependent, resp distress, resp failure, elevate BNP, hypomagnesemia, hyponatremia, AKI, hypophosphatemia
Hyperglycemia d/t DM, leukocytosis, fever, HTN emergency, morbid obesity, CHF exacerbation, bil lower extremity edema, s/p left endarterectomy, elevated lactic acid, elevated CRP, PUI COVID; bronchoscopy 8/29 - mod to sev bil inflammation, mod bil white thick secretions, high position ET tube.
Significant hx includes but not limited to DM, HTN, diastolic CHF, COPD, hyperlipidemia, disc disorder, ESBL ecoli, gout, HLD, HTN, bil ICA stenosis s/p endarterectomy, EGD w/ biopsy 06/2022
Home Diet: Regular consistencies
Current Diet: NPO

Dr. Kabsuon ordered FEES for swallow evaluation.
Annie SLP assisted with feeding during FEES

Informed consent: Procedure was explained including indications for FEES, risks, benefits, and alternatives to FEES including no evaluation as all. Consent obtained- patient signed

Assessment
Findings: Pt presents with mild oropharyngeal dysphagia r/t visualized left base of tongue ?lesion, characterized by prolonged bolus preparation, reduced base of tongue retraction; resulting to trace to moderate pharyngeal residue at vallecular region/pyriform sinus region; no penetration/aspiration noted t/o this trial. Pt observed with overcompensation of surrounding pharyngeal muscles during swallow and vocalization d/t left true vocal fold fixed at paramedian position. Pt is at increased risk due to reduced pharyngeal residue awareness and ?lesion at left base of tongue at this time.

Therapeutic Exercises, precautions/safe swallow strategies, and diet modification indicated to increase swallow safety

Recommendations:
Food Consistencies: IDDSI 6 Soft and Bite Sized

Speech Therapy

Liquid Consistencies: Thin Liquids
Meds: Whole with puree
Supervision/Assist: Distant supervision needed for feeding (provided initial cueing to initiate swallow strategies check on the patient frequently and assess the patient progress at least two to three times during the meal)

Aspiration precautions:
Sit upright as close to 90 degrees as possible,
remain upright 30 mins after meals
Small bites/sips
Swallow two times
Alternate solids/liquids

ST to treat for dysphagia
Consider further imaging of neck tissue
Consider f/u ENT outpatient when feasible
Pt would benefit to change of tx frequency to 2-4x/wk
Results and recommendations discussed with: Shayla RN

~~{ [Subjective/Patient Report:~~
~~Chart reviewed, consulted with Shayla RN- cleared pt for evaluation. Swallow evaluation indicated d/t recent extubation. Pt Identified via name and DOB. Pt is agreeable to evaluation. Family (wife) present at bedside for education. Pt is a 52 year old male to Henderson Hospital for SOB at home.~~
~~Dx: intubated 08/28-08/29; COPD exacerbation, SOB, sepsis, O2 dependent, resp distress, resp failure, elevate BNP, hypomagnesemia, hyponatremia, AKI, hypophosphatemia~~
~~Hyperglycemia d/t DM, leukocytosis, fever, HTN emergency, morbid obesity, CHF exacerbation, bil lower extremity edema, s/p left endarterectomy, elevated lactic acid, elevated CRP, PUI COVID; bronchoscopy 8/29 - mod to sev bil inflammation, mod bil white thick secretions, high position ET tube.~~
~~Significant hx includes but not limited to DM, HTN, diastolic CHF, COPD, hyperlipidemia, disc disorder, ESBL ecoli, gout, HLD, HTN, bil ICA stenosis s/p endarterectomy, EGD w/ biopsy 06/2022~~
~~Home Diet: Regular consistencies~~
~~Current Diet: NPO~~

~~Dr. Kabsuon ordered FEES for swallow evaluation.~~
~~Annie SLP assisted with feeding during FEES~~

~~Informed consent: Procedure was explained including indications for FEES, risks, benefits, and alternatives to FEES including no evaluation as all. Consent obtained- patient signed~~

~~Assessment~~
~~Findings: Pt presents with mild oropharyngeal dysphagia r/t visualized left base of tongue ?lesion, characterized by prolonged bolus preparation, reduced base of tongue retraction; resulting to trace to moderate pharyngeal residue at vallecular region/pyriform sinus region; no penetration/aspiration noted t/o this trial. Pt observed with overcompensation of surrounding pharyngeal muscles during swallow and vocalization d/t left true vocal fold fixed at paramedian position. Pt is at increased risk due to reduced pharyngeal residue awareness and ?lesion at left base of tongue at this time.~~

~~Therapeutic Exercises, precautions/safe swallow strategies, and diet modification indicated to increase swallow safety~~

Speech Therapy

~~Recommendations:~~
~~Food Consistencies: IDDSI 6 Soft and Bite Sized~~
~~Liquid Consistencies: Thin Liquids~~
~~Meds: Whole with puree~~
~~Supervision/Assist: Distant supervision needed for feeding (provided initial cueing to initiate swallow strategies check on the patient frequently and assess the patient progress at least two to three times during the meal)~~

~~Aspiration precautions:~~
~~Sit upright as close to 90 degrees as possible,~~
~~remain upright 30 mins after meals~~
~~Small bites/sips~~
~~Swallow two times~~
~~Alternate solids/liquids~~

~~ST to treat for dysphagia~~
~~Consider further imaging of neck tissue~~
~~Consider f/u ENT outpatient when feasible~~
~~Pt would benefit to change of tx frequency to 2-4x/wk~~
~~Results and recommendations discussed with: Shayla RN~~

~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Continued intermittent therapy to address functional deficits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Education

Procedure Explanation : Indications for FEES, Risks, Benefits, Alternatives to FEES including no evaluation
~~{ [Indications for FEES, Risks, Benefits, Alternatives to FEES including no evaluation] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Consent Given : Patient Gave Consent
~~{ [Patient Gave Consent] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Teaching Method : Explanation, Video/Educational TV
~~{ [Explanation, Video/Educational TV] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Learner(s) Present : Self, Spouse, Family member
~~{ [Self, Spouse, Family member] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Barriers to Learning : None evident
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [None evident] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Document learning evaluation for Responsible Learner(s)
Speech Language/Pathology Tx Plan : Verbalizes understanding
~~{ [Verbalizes understanding] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Swallowing Strategies : Verbalizes understanding, Needs further teaching
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

Speech Therapy

~~{ [Verbalizes understanding, Needs further teaching] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Plan

Planned Treatments : Dysphagia therapy
~~{ [Dysphagia therapy] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Frequency : 2-4 times per week
~~{ [2-4 times per week] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Duration : 1-2 weeks
~~{ [1-2 weeks] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
SLP Intensity Per Day : Other: as tolerated
~~{ [Other: as tolerated] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
~~{ [Continued intermittent therapy to address functional deficits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Rehabilitation Potential : Good
~~{ [Good] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Plan/Goals Established With Patient/Caregiver : Yes
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Yes] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Additional Information

Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Bed in low position, Call device within reach, Personal items within reach] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Long Term Goals

SLP Patient, Caregiver Goals : "to eat food"
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ ["to eat food"] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will tolerate least restrictive consistencies with risk of aspiration reduced as much as reasonably possible. Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will tolerate least restrictive consistencies~~

Speech Therapy

	~~with risk of aspiration reduced as much as reasonably possible.] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Status :	Progressing, continue Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Progressing, continue] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Timeframe To Complete :	9/13/24 Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [9/13/24] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Short Term Goals

Other SLP Short Term Goal Grid

	Goal #1	Goal #2	Goal #3
Other :	Pt will tolerate 90ml challenge	Pt will perform compensatory	Pt will tolerate regular PO trials

Speech Therapy

	vs instrumental swallow without uncompensated aspiration or penetration on thin liquids Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will tolerate 90ml challenge vs instrumental swallow without uncompensated aspiration or penetration on thin liquids] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	strategies to protect the airway and improve clearance of the bolus in 80% of trials independently Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	with no overt s/s of apsiration in 5 out o5 trials Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will tolerate regular PO trials with no overt s/s of apsiration in 5 out o5 trials] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Status :	Goal met Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Goal met] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	Progressing, continue Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Progressing, continue] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	Initial Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Initial] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Date Met :	8/30/2024 PDT Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [8/30/2024 PDT] - previously~~		

Speech Therapy

	~~charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~		
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

ST Non Timed Modalities 2024

ST Treatment Start Time : 8/30/2024 12:58 PDT
~~{ [8/30/2024 12:58 PDT] - previously charged by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
ST Treatment End Time : 8/30/2024 13:58 PDT
~~{ [8/30/2024 13:58 PDT] - previously charged by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
FEES BY CINE/VIDEO RECORDING Min : 60
~~{ [60] - previously charged by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
ST Total Non Timed : 60
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [60] - previously charged by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

History and Physical

DOCUMENT NAME:	History & Physical
SERVICE DATE/TIME:	8/28/2024 03:00 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Justice PAC,Tia M (8/28/2024 03:27 PDT)
SIGN INFORMATION:	Al-owir MD,Bassam S (8/28/2024 15:08 PDT); Justice PAC, Tia M (8/28/2024 08:49 PDT); Justice PAC,Tia M (8/28/2024 08:47 PDT)

Chief Complaint/Reason for Consultation
SOB difficulty breathing at home. tympanic temp of 104

History of Present Illness
The patient is a 52 year old male with a past medical hx of diabetes, hypertension, CHF, COPD, and hyperlipidemia who presented to Henderson Hospital ED for difficulty breathing with fever of 104 F. Patient was placed on BiPAP but was not tolerating, patient was intubated in the ED.
Patient was recently admitted for left carotid endarterectomy 7/26/24.

All labs and imaging reviewed.
Initial labs include WBC 13.22, platelets 117, glucose 316, sodium 134, creatinine 2.02, phosphorus 0.7, lactic acid 4.5, BNP 1242, CRP 4.29
vbg: pH 7.51, CO2 36.1
CT angio head showed Fluid within the left carotid space.

Histories
Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)
No Known Medication Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(6)
Disc disorder
DM (diabetes mellitus)
ESBL Escherichia coli
Gout
HLD (hyperlipidemia)

History and Physical

Tortuous left internal carotid artery. Patent vessels with no significant stenosis.
CT angio chest: Status post intubation and feeding tube placement. No central pulmonary embolus. Artifact. Normal appendix. Foley catheter. Hydroceles

The patient was seen and evaluated in the emergency department. The patient is intubated and sedated. The patient is not following commands but attempts to open his eyes when stimulated. Patient was spontaneously moving upper extremities. The patients wife was at bedside and stated the patient was breathing very heavy and altered at home. She states he was upstairs yelling and saying abnormal things. She states he has acted this way in the past when he had a UTI. UA workup was negative in ED for UTI. At home the patient is dependent on 2L NC of O2. The patient will be admitted to the ICU for vent management, monitoring, and treatment.

Review of Systems
Unable to obtain due to patient's condition

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**H** 38	08/28/2024 02:15	**H** 38	08/27/2024 22:36	**H** 39.2	08/27/2024 20:48
Heart Rate Monitored	74	08/28/2024 01:15	74	08/28/2024 01:15	86	08/27/2024 23:22
Resp Rate	16	08/28/2024 02:15	16	08/28/2024 00:36	**C** 30	08/27/2024 20:43
SBP	132	08/28/2024 02:15	118	08/28/2024 00:36	**C** 205	08/27/2024 20:48
DBP	63	08/28/2024 02:15	**L** 56	08/28/2024 00:36	**C** 108	08/27/2024 21:13
MAP	95	08/28/2024 01:15	95	08/28/2024 01:15	95	08/28/2024 01:15
SpO2	100	08/28/2024 02:15	99	08/27/2024 21:13	100	08/27/2024 23:22
O2 Flow Rate	on 75 %		on 90 %		on 90 %	
O2 Therapy	Ventilator		BiPAP		Ventilator	

Pain Scores (Last Within 24hrs)
Numeric Pain Scale: 0 = No pain (20:42)

Ventilator Settings (Last Within 24hrs)
Ventilator Mode: VC-AC (23:22) FIO2: 75 % (02:15)
Ventilator Frequency, Mandatory: 20 br/min (23:22)
Tidal Volume, Delivered: 50 mL (23:22)
Pressure Support: 9 cm H2O (21:02)
Positive End Expiratory Pressure: 5 cm H2O (23:22)

Intake and Output (current encounter)

No Data Available

HTN (hypertension)

Past Surgical History
Endarterectomy Carotid Artery (Left): 07/26/24
EGD w/ Biopsy (Laterality NA): 06/27/22
foot sx
shoulder sx

Social History
Alcohol
Risk Assessment: Denies Alcohol Use; Details: Denies
Substance Abuse
Risk Assessment: Denies Substance Abuse; Details: Denies
Tobacco
Risk Assessment: High Risk; Details: Current

Past Psychiatric History

Family History
No Family History documented.

Medications
Home Medications
albuterol (Albuterol (Eqv-ProAir HFA) 90 mcg/inh inhalation aerosol)
2 Puffs Inhalation Every 4 hours as needed as needed for wheezing
amLODIPine (amLODIPine 10 mg oral tablet)
10 Milligram 1 Tabs By Mouth Daily
aspirin (Aspirin Low Dose 81 mg oral delayed release tablet)
81 Milligram 1 Tabs By Mouth Daily
atorvastatin (atorvastatin 40 mg oral tablet)
40 Milligram 1 Tabs By

History and Physical

Glasgow Coma Scale
Eye Opening Response Glasgow: Spontaneously (08/27/24 20:48:00)
Best Verbal Response Glasgow: Oriented and converses (08/27/24 20:48:00)
Best Motor Response Glasgow: Obeys commands (08/27/24 20:48:00)
Glasgow Coma Score: 15 (08/27/24 20:48:00)

Physical Exam
General: Intubated sedated.
Head: Normocephalic.
Eyes: Pupils are pinpoint
Ear, Nose and Throat: ETT.
Neck: Supple
Respiratory: Diminished lung sounds bilaterally. No wheezing, rales or rhonchi.
Cardiovascular: Regular rate and rhythm, no murmurs, rubs or gallops appreciated.
Gastrointestinal: Abdomen soft, non-distended. Normal bowel sounds.
Genitourinary: Urinary catheter
Neurologic: Exam completed while patient on sedation. Cough reflex present. Gag reflex present. Patient moves all four extremities spontaneously and response to painful stimuli.
Skin: No rashes or lesions.
Musculoskeletal: No gross deformities.
Extremities: No edema noted. Pulses palpable x 4. Mild discoloration on lower extremities bilaterally.

Assessment/Plan

Assessment:
Acute hypoxic respiratory failure
COPD exacerbation
CHF exacerbation
Leukocytosis reactive versus infectious
Acute kidney injury
Hypophosphatemia
Hyperglycemia
Lactic acidosis
Hypertensive emergency-resolved
Fever

H/O: Diabetes, COPD dependent with 2 L NC, CHF EF 60-65% 7/24 , hypertension, left carotid endarterectomy with patch angioplasty 7/2024

Plan:
Admit patient to ICU
Close vital sign and hemodynamic monitoring
Intubated and sedated

Mouth Daily for 30 Days
buprenorphine-naloxone (buprenorphine-naloxone 4 mg-1 mg sublingual film) 1 Film Sublingual 3 Times a Day dissolve under the tongue
cholecalciferol (Vitamin D3 125 mcg (5000 intl units) oral tablet)
125 Microgram 1 Tabs By Mouth Daily
dapagliflozin (Farxiga 10 mg oral tablet)
10 Milligram 1 Tabs By Mouth Daily
furosemide (furosemide 20 mg oral tablet)
20 Milligram 1 Tabs By Mouth Daily
glimepiride (glimepiride 2 mg oral tablet)
2 Milligram 1 Tabs By Mouth 2 Times a Day
hydrALAZINE (hydrALAZINE 100 mg oral tablet)
100 Milligram 1 Tabs By Mouth 3 Times a Day
metoprolol (Metoprolol Tartrate 100 mg oral tablet)
200 Milligram 2 Tabs By Mouth 2 Times a Day
minoxidil (minoxidil 2.5 mg oral tablet)
5 Milligram 2 Tabs By Mouth 2 Times a Day
mirabegron (Myrbetriq 50 mg oral tablet, extended release)
50 Milligram 1 Tabs By Mouth Daily do not crush or chew
nortriptyline (nortriptyline 25 mg oral capsule)
50 Milligram 2 Capsules By Mouth Every PM
olodaterol (Striverdi Respimat 2.5 mcg/inh inhalation aerosol)
2 Puffs Inhalation Daily

History and Physical

Continue sedation; RASS -2
Vent management per ABG; spo2 88-92%
Start methylprednisolone
Monitor WBC
Procal and lactic acid pending
Empiric ceftriaxone and azithromycin; de-escalate or change as appropriate
No obvious source of infection at this time
Monitor renal function; I and O's
Consider nephrology consult if renal function does not improve or patient is oligoric
Replace electrolytes per protocol
Repeat Phos level
Cardiology consult in AM
Q4 blood glucose checks
Intermediate sliding scale
One time dose hydralazine, repeat if effective and SBP >170
PRN DuoNeb
PRN Tylenol for fever
VTE/GI prophylaxis

Discussed with RN and patient's wife at bedside
Total time spent in direct and indirect patient care activities including, but not limited to, complex medical decision making, review of the medical record, electronic order entry, discussion with patient and other healthcare providers: 72 mins

Portions of this note were generated using speech-to-text voice dictation software. Although this document has
been reviewed completely, inadvertent spelling and grammatical errors may persist due to the nature of software generated text. Please contact me directly with any specific questions.

traZODone (traZODone 100 mg oral tablet)
200 Milligram 2 Tabs By Mouth at Bedtime

Inpatient Medications
Medications (11) Active
SCHEDULED: (9)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT Once
azithromycin + Dextrose 5% in Water 250 mL (azithromycin + Dextrose 5% in Water intravenous solution 250 mL) 500 mg 10 mL, IV Piggyback, q24H Interval
cefTRIAXone 1 gm vial (cefTRIAXone) 1 gm 10 mL, IV Push, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
methylPREDNISolone sod succinate 125 mg vial (methylPREDNISolone sodium succinate (Solu-Medrol)) 80 mg 1.28 mL, IV Push, q6H
Nitro Ointment Removal 250 inch, Topical, Once
CONTINUOUS: (2)
dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL (dexmedeTOMIDine IV additive 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL) 100 mL, IV, 12.93 mL/hr
nitroglycerin/D5W 50 mg [5 mcg/min] + Premix D5W 250 mL (nitroglycerin IV additive 50 mg [5 mcg/min] + Premix D5W 250 mL) 250 mL, IV, 1.5 mL/hr
PRN: (0)

72 hour Antibiotic History

Completed/Discontinued Antibiotics	Stop Date/Time	Last Administered	First Administered

History and Physical

vancomycin 2 gm, 500 mL, 250 mL/hr, IV Piggyback, Once	08/28/2024 02:08	08/28/2024 02:08	08/28/2024 02:08
cefTRIAXone 1 gm, 10 mL, IV Push, Once	08/27/2024 21:13	08/27/2024 21:13	08/27/2024 21:13

Results

Recent Labs

Antibody Screen

ABSC SP 3 I	Negative ABSC (Normal)	08/28/2024 00:53

Blood Gases

Art Na	138 mmol/L (Normal)	08/28/2024 01:05
Art K	4.1 mmol/L (Normal)	08/28/2024 01:05
Art iCa	1.22 mmol/L (Normal)	08/28/2024 01:05
pH Arterial	7.41 (Normal)	08/28/2024 01:05
Art pCO2	48.0 mmHg (High)	08/28/2024 01:05
Art pO2	208.0 mmHg (High)	08/28/2024 01:05
HCO3 Art	28.6 mmol/L (High)	08/28/2024 01:05
Arterial pCO2(T)	48.0 mmHg (N/A)	08/28/2024 01:05
Arterial pH(T)	7.41 (N/A)	08/28/2024 01:05
Arterial pO2(T)	208.0 mmHg (N/A)	08/28/2024 01:05
Art Base Excess	4.7 mmol/L (High)	08/28/2024 01:05
Art sO2	100 % (High)	08/28/2024 01:05
Art tHB	13.4 gm/dL (Normal)	08/28/2024 01:05
Art Hct	41 % (Normal)	08/28/2024 01:05
Ven pH	7.51 (High)	08/27/2024 20:47
Ven pCO2	36.1 mmHg (Low)	08/27/2024 20:47
Ven pO2	40.6 mmHg (Normal)	08/27/2024 20:47
Venous pCO2(T)	36.1 mmHg (N/A)	08/27/2024 20:47
Venous pH(T)	7.51 (N/A)	08/27/2024 20:47
Venous pO2(T)	40.6 mmHg (N/A)	08/27/2024 20:47

History and Physical

Venous HCO3	29.3 mmol/L (High)	08/27/2024 20:47
Ven Base Excess	5.9 mmol/L (High)	08/27/2024 20:47
Ven sO2	82 % (High)	08/27/2024 20:47
Ven tHb	14.7 gm/dL (Normal)	08/27/2024 20:47
Ven Hct	45 % (Normal)	08/27/2024 20:47
POC Pt Temp	37.0 DegC (N/A)	08/28/2024 01:05
POC FIO2	100 % (N/A)	08/28/2024 01:05
POC Sample	Arterial (N/A)	08/28/2024 01:05
POC Site	Radial Right (N/A)	08/28/2024 01:05
POC Allen Test	N/A (N/A)	08/28/2024 01:05
POC Del Sys	VENTILATOR (N/A)	08/28/2024 01:05
POC Vent Mode	AC (N/A)	08/28/2024 01:05
POC Set RR	20 br/min (N/A)	08/28/2024 01:05
POC VT	550 mL (N/A)	08/28/2024 01:05
POC PEEP	5 cm H2O (N/A)	08/28/2024 01:05
POC IT	1.000 Seconds (N/A)	08/28/2024 01:05

Blood Group and Type

ABO+Rh I	B POS (Unknown)	08/28/2024 00:53

Cardiac

NT-ProBNP	1242 pg/mL (High)	08/27/2024 20:57
Troponin TNIH	105.7 ng/L (Critical)	08/28/2024 00:53

General Chemistry

Glucose Level	316 mg/dL (High)	08/27/2024 20:57
POC Glucose	263 mg/dL (High)	08/27/2024 20:49
Sodium	134 mmol/L (Low)	08/27/2024 20:57
Ven Na	137 mmol/L (Normal)	08/27/2024 20:47
Potassium	3.9 mmol/L (Normal)	08/27/2024 20:57
Ven K	4.2 mmol/L (Normal)	08/27/2024 20:47

Chloride	100 mmol/L (Normal)	08/27/2024 20:57
CO2	28 mmol/L (Normal)	08/27/2024 20:57
Anion Gap	6 mmol/L (Normal)	08/27/2024 20:57
BUN	17 mg/dL (Normal)	08/27/2024 20:57
Creatinine	2.020 mg/dL (High)	08/27/2024 20:57
BUN/Creat Ratio	8 (Normal)	08/27/2024 20:57
Calcium	10.4 mg/dL (High)	08/27/2024 20:57
Ven iCa	1.13 mmol/L (Normal)	08/27/2024 20:47
Albumin. Level	4.0 gm/dL (Normal)	08/27/2024 20:57
TP	8.4 gm/dL (High)	08/27/2024 20:57
A/G Ratio	0.9 (Normal)	08/27/2024 20:57
T Bili	1.0 mg/dL (Normal)	08/27/2024 20:57
Alk Phos	72 units/L (Normal)	08/27/2024 20:57
AST	20 units/L (Normal)	08/27/2024 20:57
ALT	24 units/L (Normal)	08/27/2024 20:57
Mg Lvl	1.7 mg/dL (Low)	08/27/2024 20:57
Phosphorus	0.7 mg/dL (Critical)	08/27/2024 20:57
Estimated Creatinine Clearance	41.39 mL/min ()	08/27/2024 22:59
eGFR Cr	39 mL/min/1.73m2 (N/A)	08/27/2024 20:57
eGFR Pediatric	Not Reported (N/A)	08/27/2024 20:57
Calc Osmo	282 mOsmol/kg (Normal)	08/27/2024 20:57
Lactic Acid Lvl	2.0 mmol/L (Normal)	08/28/2024 01:06

General Coagulation

PT	11.9 Seconds (Normal)	08/28/2024 00:53
INR	1.1 (Normal)	08/28/2024 00:53
aPTT	31 Seconds (Normal)	08/28/2024 00:53

General Hematology

History and Physical

WBC	13.22 x10e3/mcL (High)	08/27/2024 20:57
RBC	4.56 x10e6/mcL (Normal)	08/27/2024 20:57
Hgb	14.1 gm/dL (Normal)	08/27/2024 20:57
Hct	41.2 % (Normal)	08/27/2024 20:57
MCV	90.4 Femtoliters (Normal)	08/27/2024 20:57
MCH	30.9 pg (Normal)	08/27/2024 20:57
MCHC	34.2 gm/dL (Normal)	08/27/2024 20:57
RDW-SD	41.1 Femtoliters (Normal)	08/27/2024 20:57
RDW-CV	12.6 % (Normal)	08/27/2024 20:57
Plt	117 x10e3/mcL (Low)	08/27/2024 20:57
MPV	10.4 Femtoliters (Normal)	08/27/2024 20:57
NRBC %	0.2 /100 WBC (Normal)	08/27/2024 20:57
NRBC #	0.02 x10e3/mcL (High)	08/27/2024 20:57
Neut % Auto	82.5 % (N/A)	08/27/2024 20:57
Lymph % Auto	8.5 % (N/A)	08/27/2024 20:57
Mono % Auto	7.0 % (N/A)	08/27/2024 20:57
Eos % Auto	0.5 % (N/A)	08/27/2024 20:57
Baso % Auto	0.3 % (N/A)	08/27/2024 20:57
Immature Grans %	1.2 % (High)	08/27/2024 20:57
Neut # Auto	10.90 x10e3/mcL (High)	08/27/2024 20:57
Lymph # Auto	1.13 x10e3/mcL (Normal)	08/27/2024 20:57
Mono # Auto	0.93 x10e3/mcL (Normal)	08/27/2024 20:57
Eos # Auto	0.06 x10e3/mcL (Normal)	08/27/2024 20:57
Baso # Auto	0.04 x10e3/mcL (Normal)	08/27/2024 20:57
Immature Grans # Auto	0.16 x10e3/mcL (High)	08/27/2024 20:57

Other Chemistry

Beta Hydroxybutyrate Lvl	1.7 mg/dL (Normal)	08/27/2024 20:57
Ammonia Level	25 mcmol/L (Normal)	08/28/2024 00:53

History and Physical

CRP	4.29 mg/dL (High)	08/27/2024 20:57

Toxicology

Ur Amp/Methamp	Not Detected (Normal)	08/27/2024 22:12
Ur Barbiturates	Not Detected (Normal)	08/27/2024 22:12
Ur Benzodiazepine Screen	Not Detected (Normal)	08/27/2024 22:12
Ur Cannabinoids	Not Detected (Normal)	08/27/2024 22:12
Ur Cocaine	Not Detected (Normal)	08/27/2024 22:12
Ur Fentanyl	Not Detected (Normal)	08/27/2024 22:12
Ur Methadone	Not Detected (Normal)	08/27/2024 22:12
Ur Opiates	Not Detected (Normal)	08/27/2024 22:12
Ur PCP	Not Detected (Normal)	08/27/2024 22:12
See Comment	Comment Charted (N/A)	08/27/2024 22:12
Ethanol	<9 mg/dL (Normal)	08/27/2024 21:22

UA Macroscopic

Ur Color	Yellow (Normal)	08/27/2024 22:04
Ur Clarity	Cloudy (Abnormal)	08/27/2024 22:04
UR Glucose	3+ (Abnormal)	08/27/2024 22:04
Ur Bili	Negative (Normal)	08/27/2024 22:04
Ur Ketones	Negative (Normal)	08/27/2024 22:04
Ur Spec Grav	1.029 (Normal)	08/27/2024 22:04
Ur Blood	1+ (Abnormal)	08/27/2024 22:04
Ur pH	7.0 (Normal)	08/27/2024 22:04
Ur Protein	3+ (Abnormal)	08/27/2024 22:04
Ur Urobilinogen	<2.0 mg/dL (Normal)	08/27/2024 22:04
Ur Nitrite	Negative (Normal)	08/27/2024 22:04
Ur Leuk Est	Negative (Normal)	08/27/2024 22:04
UA Culture?	No (N/A)	08/27/2024 22:04

History and Physical

Signs/Symptoms of UTI?	Fever (Normal)	08/27/2024 22:04

UA Microscopic

UA WBC Inst	4 /HPF (Normal)	08/27/2024 22:04
UA RBC Inst	2 /HPF (Normal)	08/27/2024 22:04
Ur Bacteria	None Seen /HPF (Normal)	08/27/2024 22:04
UA Squam Epi Inst	<1 /HPF (Normal)	08/27/2024 22:04
Ur Mucous	Rare /LPF (Abnormal)	08/27/2024 22:04

Abnormal Labs

Blood Gases

Art Base Excess 4.7 mmol/L (High) 08/28/2024 01:05
Art pCO2 48.0 mmHg (High) 08/28/2024 01:05
Art pO2 208.0 mmHg (High) 08/28/2024 01:05
Art sO2 100 % (High) 08/28/2024 01:05
HCO3 Art 28.6 mmol/L (High) 08/28/2024 01:05
Ven Base Excess 5.9 mmol/L (High) 08/27/2024 20:47
Ven pCO2 36.1 mmHg (Low) 08/27/2024 20:47
Ven pH 7.51 (High) 08/27/2024 20:47
Ven sO2 82 % (High) 08/27/2024 20:47
Venous HCO3 29.3 mmol/L (High) 08/27/2024 20:47

Blood Group and Type

ABO+Rh I B POS (Unknown) 08/28/2024 00:53

Cardiac

NT-ProBNP 1242 pg/mL (High) 08/27/2024 20:57
Troponin TNIH 105.7 ng/L (Critical) 08/28/2024 00:53

History and Physical

General Chemistry

Calcium 10.4 mg/dL
(High) 08/27/2024 20:57
Creatinine 2.020 mg/dL
(High) 08/27/2024 20:57
Glucose Level 316 mg/dL
(High) 08/27/2024 20:57
Mg Lvl 1.7 mg/dL
(Low) 08/27/2024 20:57
POC Glucose 263 mg/dL
(High) 08/27/2024 20:49
Phosphorus 0.7 mg/dL
(Critical) 08/27/2024
20:57
Sodium 134 mmol/L
(Low) 08/27/2024 20:57
TP 8.4 gm/dL
(High) 08/27/2024 20:57

General Hematology

Immature Grans % 1.2 %
(High) 08/27/2024 20:57
Immature Grans #
Auto 0.16 x10e3/mcL
(High) 08/27/2024 20:57
NRBC # 0.02 x10e3/mcL
(High) 08/27/2024 20:57
Neut # Auto 10.90
x10e3/mcL
(High) 08/27/2024 20:57
Plt 117 x10e3/mcL
(Low) 08/27/2024 20:57
WBC 13.22 x10e3/mcL
(High) 08/27/2024 20:57

Other Chemistry

CRP 4.29 mg/dL
(High) 08/27/2024 20:57

UA Macroscopic

UR Glucose 3+
(Abnormal) 08/27/2024
22:04
Ur Blood 1+
(Abnormal) 08/27/2024
22:04
Ur Clarity Cloudy
(Abnormal) 08/27/2024
22:04
Ur Protein 3+
(Abnormal) 08/27/2024
22:04

History and Physical

UA Microscopic
Ur Mucous Rare /LPF
(Abnormal) 08/27/2024
22:04

Note: Critical results are displayed in red.

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**13.22 H** 08/27/2024 20:57			
Hgb	**14.1** 08/27/2024 20:57			
Hct	**41.2** 08/27/2024 20:57			
Baso # Auto	**0.04** 08/27/2024 20:57			
Baso % Auto	**0.3** 08/27/2024 20:57			
Eos # Auto	**0.06** 08/27/2024 20:57			
Eos % Auto	**0.5** 08/27/2024 20:57			
Immature Grans %	**1.2 H** 08/27/2024 20:57			
Immature Grans # Auto	**0.16 H** 08/27/2024 20:57			
Lymph # Auto	**1.13** 08/27/2024 20:57			
Lymph % Auto	**8.5** 08/27/2024 20:57			
Mono # Auto	**0.93** 08/27/2			

History and Physical

	024 20:57			
Mono % Auto	**7.0** 08/27/2024 20:57			
Neut # Auto	**10.90 H** 08/27/2024 20:57			
Neut % Auto	**82.5** 08/27/2024 20:57			
Plt	**117 L** 08/27/2024 20:57			
Glucose Level	**316 H** 08/27/2024 20:57			
Sodium	**134 L** 08/27/2024 20:57			
Potassium	**3.9** 08/27/2024 20:57			
Chloride	**100** 08/27/2024 20:57			
CO2	**28** 08/27/2024 20:57			
Creatinine	**2.020 H** 08/27/2024 20:57			
BUN	**17** 08/27/2024 20:57			
BUN/Creat Ratio	**8** 08/27/2024 20:57			
Mg Lvl	**1.7 L** 08/27/2024 20:57			
Phosphorus	**0.7** 08/27/2024 20:57			
Calcium	**10.4 H**			

History and Physical

	08/27/2024 20:57			
Albumin. Level	**4.0** 08/27/2024 20:57			
Ur Protein	**3+** 08/27/2024 22:04			
T Bili	**1.0** 08/27/2024 20:57			
Alk Phos	**72** 08/27/2024 20:57			
AST	**20** 08/27/2024 20:57			
ALT	**24** 08/27/2024 20:57			
aPTT	**31** 08/28/2024 00:53			
PT	**11.9** 08/28/2024 00:53			
INR	**1.1** 08/28/2024 00:53			

LFT (Last Within 24hrs)
AST: 20 units/L (20:57)
ALT: 24 units/L (20:57)
Alk Phos: 72 units/L (20:57)
T Bili: 1 mg/dL (20:57)
TP: 8.4 gm/dL High (20:57)

Urinalysis (Last Within 24hrs)
Ur Color: Yellow (22:04)
Ur Clarity: Cloudy UA Abnormal (22:04)
UR Glucose: 3+ Abnormal (22:04)
Ur Bili: NegativeIRIS (22:04)
Ur Ketones: NegativeIRIS (22:04)
Ur Spec Grav: 1.029 (22:04)
Ur Blood: 1+ Abnormal (22:04)
Ur pH: 7 (22:04)

History and Physical

Ur Protein: 3+ Abnormal (22:04)
Ur Urobilinogen: <2.0 (22:04)
Ur Nitrite: NegativeIRIS (22:04)
Ur Leuk Est: NegativeIRIS (22:04)
UA Culture?: No (22:04)
Signs/Symptoms of UTI?: Fever (22:04)
UA WBC Inst: 4 /HPF (22:04)
UA RBC Inst: 2 /HPF (22:04)
Ur Bacteria: None Seen Iris (22:04)
UA Squam Epi Inst: <1 (22:04)
Ur Mucous: Rare Iris Alpha Abnormal (22:04)

Microbiology

Culture Sputum		**In-Lab**	
Source:	Sputum, Suctioned	Body Site:	
Collected Dt/Tm:	08/28/2024 01:12	Last Updated Dt/Tm:	08/28/2024 01:12

Culture Blood ARD		**In-Lab**	
Source:	Blood	Body Site:	
Collected Dt/Tm:	08/27/2024 21:01	Last Updated Dt/Tm:	08/27/2024 20:54

Cardiology Labs
NT-ProBNP: 1242 pg/mL High (08/27/24 20:57:00)
Troponin TNIH: 105.7 ng/L Critical (08/28/24 00:53:00)
Troponin TNIH: 63.6 ng/L (08/27/24 22:04:00)
Troponin TNIH: 44.1 ng/L (08/27/24 20:57:00)

Blood Gases (Last Within 24hrs)
pH Arterial: 7.41 (01:05)
Art pO2: 208 mmHg High (01:05)
Art pCO2: 48 mmHg High (01:05)
HCO3 Art: 28.6 mmol/L High (01:05)
Art sO2: 100 % High (01:05)
Ven pH: 7.51 High (20:47)
Ven pO2: 40.6 mmHg (20:47)
Ven pCO2: 36.1 mmHg Low (20:47)
Ven sO2: 82 % High (20:47)

Image

History and Physical

XR Chest 1 View Frontal 08/27/2024 21:00 by Ozburn, Asyiah

IMPRESSION: No evidence of an acute process. These imaging findings do not exclude additional clinically significant abnormalities. This report should be interpreted in the context of clinical information including patient symptoms and available laboratory findings. Follow-up imaging or other interventions may be appropriate, if indicated by your clinical impression. Electronically signed by: Douglas Larson MD 08/27/2024 09:31 PM PDT RP Workstation: DRTELRS20417

Signature Line

***** Final *****

CT Angio Chest w/wo + CT Abd/Pel w/Cont 08/28/2024 00:25 by Peinado, Brooke

IMPRESSION: Status post intubation and feeding tube placement. No central pulmonary embolus. Artifact. Normal appendix. Foley catheter. Hydroceles. Electronically signed by: Terence Scipione MD 08/28/2024 12:49 AM PDT RP Workstation: DRHMERS31715

Signature Line

***** Final *****

XR Chest 1 View Frontal 08/27/2024 23:54 by Henderson R.T. (R), Shannon

IMPRESSION: Status post intubation and feeding tube placement. No pneumothorax. Electronically signed by: Terence Scipione MD 08/28/2024 12:04 AM PDT RP Workstation: DRHMERS31715

Signature Line

***** Final *****

CT Head or Brain w/o Contrast 08/28/2024 00:25 by

History and Physical

Peinado, Brooke

IMPRESSION: No evidence of an acute intracranial process. These imaging findings do not exclude additional clinically significant abnormalities. This report should be interpreted in the context of clinical information including patient symptoms and available laboratory findings. Follow-up imaging or other interventions may be appropriate, if indicated by your clinical impression. Electronically signed by: Douglas Larson MD 08/28/2024 12:47 AM PDT RP Workstation: DRTELRS20417

CT Angio Head/Neck w/Contr Incl w/o Imag 08/28/2024 00:25 by Peinado, Brooke

IMPRESSION: Fluid within the left carotid space. Tortuous left internal carotid artery. Patent vessels with no significant stenosis. Electronically signed by: Terence Scipione MD 08/28/2024 12:53 AM PDT RP Workstation: DRHMERS31715

Signature Line

***** Final *****

Electronically Signed By: Justice, Tia PAC
On: 08.28.2024 08:47 PDT

Electronically Signed On: 08.28.2024 08:49 PDT
Justice, Tia PAC

Electronically Signed On: 08.28.2024 15:08 PDT
Al-owir, Bassam MD

Consultation

DOCUMENT NAME:	Consult - Nephrology
SERVICE DATE/TIME:	9/1/2024 08:33 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Parekh DO,Parag P (9/1/2024 08:33 PDT)
SIGN INFORMATION:	Parekh DO,Parag P (9/1/2024 08:33 PDT)

Consultation

Chief Complaint/Reason for Consultation
SOB difficulty breathing at home. tympanic temp of 104

History of Present Illness
HPI:
52 year old male with history of CKD3, DM, HTN, CHF, COPD, HLD, Obesity who presented to HH 8/27/24 with shortness of breath and fevers. He was unable to tolerate BiPAP, requiring intubation in the ER. He was treated for PNA and CHF/COPD exacerbation. Now asymptomatic. Denies urinary symptoms. Nephrology consulted for hyponatremia (sodium dipped to 127) and uncontrolled HTN.
10 point review of systems otherwise negative

A/P
AKI on CKD3, baseline appears around 1.3-1.6.
Hyponatremia
CHF exacerbation, COPD exacerbation, PNA, Acute respiratory failure, HLD, HTN, Obesity
Recent carotid endarterectomy

- Ok to DC from renal standpoint
- Monitor Cr
- Monitor Corrected Na
- BP still elevated, but missing home meds and remains on steroids. BP logs from home well controlled. OK to DC on home regimen.
- OK to resume farxiga and lasix
- Encouraged follow up in clinic for further CKD mgmt

Physical Exam:
VSS reviewed
Gen: NAD, appears stated age
HEENT: NC/AT, MMM
Neck: no mass/swelling
CV: nl S1 S2, no rub
Lungs: Unlabored respirations, symmetric chest rise
Abd: Soft, no suprapubic tenderness
Extremities: no cyanosis, clubbing, edema

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Heart Rate Monitored	84	09/01/2024 07:38	84	09/01/2024 07:38	90	08/31/2024 13:36
Resp Rate	18	09/01/2024 07:38	16	09/01/2024 07:28	20	08/31/2024 13:26
SBP	**H** 174	08/31/2024 17:16	**H** 173	08/31/2024 14:00	**C** 188	08/31/2024 16:12
DBP	**H** 81	08/31/2024 17:16	**H** 81	08/31/2024 14:00	**H** 86	08/31/2024 16:12
SpO2	99	09/01/2024 07:38	96	08/31/2024 13:26	99	09/01/2024 07:38
O2 Flow Rate	on 3	l/min	on 4	l/min	on 3	l/min

Histories
Past Medical History
Active Problems(6)
Disc disorder
DM (diabetes mellitus)
ESBL Escherichia coli
Gout
HLD (hyperlipidemia)
HTN (hypertension)

Past Surgical History
Bronchoscopy w/ Procedures (Laterality NA): 08/29/24
Endarterectomy Carotid Artery (Left): 07/26/24
EGD w/ Biopsy (Laterality NA): 06/27/22
foot sx
shoulder sx

Social History
Alcohol
Risk Assessment: Denies Alcohol Use; Details: Denies
Substance Abuse
Risk Assessment: Denies Substance Abuse; Details: Denies
Tobacco
Risk Assessment: High Risk; Details: Current

Family History
No Family History documented.

Medications
Inpatient Medications
Medications (23) Active
SCHEDULED: (20)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 5 mg Tab (amLODIPine) 10 mg 2 Tabs, OG-Tube, Daily
aspirin 81 mg Chew Tab (aspirin 81 mg oral tablet, chewable) 81 mg 1 Tabs, OG-Tube, Daily
atorvastatin 40 mg Tab (Lipitor 40 mg oral tablet) 40 mg 1 Tabs, OG-Tube, qHS

Consultation

O2 Therapy	Nasal cannula	Nasal cannula	Nasal cannula

Intake and Output (current encounter)

	08/28 01:14 09/01 08:33	09/01 07:00 09/01 08:33	08/31 07:00 09/01 06:59	08/30 07:00 08/31 06:59	08/29 07:00 08/30 06:59
Intake	1419.6	0	100	0	582.8
Output	20930	0	1000	4270	4860
Net Total	-19510.4	0	-900	-4270	-4277.2
Stool Count	3	0	0	2	1

Physical Exam

Assessment/Plan

Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Home with family care

buprenorphine 2 mg sublingual Dis Tab (buprenorphine) 4 mg 2 Tabs, SubLingual, TID
carvedilol 12.5 mg Tab (carvedilol) 12.5 mg 1 Tabs, Oral, BID With Meals
cefTRIAXone 2 gm vial (Rocephin 2 g injection) 2 gm 20 mL, IV Push, qHS
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
doxazosin 2 mg Tab (doxazosin) 2 mg 1 Tabs, OG-Tube, Daily
doxycycline + Dextrose 5% in Water 100 mL (doxycycline + Dextrose 5% in Water intravenous solution 100 mL) 100 mg 10 mL, IV Piggyback, BID
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin glargine 1 unit/0.01 mL syringe (Lantus) 5 units 0.05 mL, SubCutaneous, qAM
insulin lispro HumaLOG 1 unit/0.01mL syringe (insulin lispro HumaLOG correctional scale High) 4-18 Units, SubCutaneous, Before Meals and HS
methylPREDNISolone sod succinate 40 mg Vial PF (methylPREDNISolone sodium succinate (Solu-Medrol)) 20 mg 0.5 mL, IV Push, q12H Interval
nortriptyline 25 mg Cap (nortriptyline) 25 mg 1 Caps, Oral, qPM
pantoprazole 40 mg vial (Protonix) 40 mg, IV Push, Daily
traZODone 50 mg Tab (traZODone) 150 mg 3 Tabs, Oral, qHS
CONTINUOUS: (0)
PRN: (3)
acetaminophen 650 mg/20.3 mL Oral Liq (Tylenol Liquid) 650 mg 20.3 mL, OG-Tube, q6H
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV

Consultation

Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H Interval

Results
BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 132 mmol/L Low (03:38)
Potassium: 4.2 mmol/L (03:38)
Chloride: 98 mmol/L (03:38)
CO2: 28 mmol/L (03:38)
BUN: 51 mg/dL High (03:38)
Creatinine: 1.46 mg/dL High (03:38)
Glucose Level: 250 mg/dL High (03:38)
Calcium: 9.5 mg/dL (03:38)
Mg Lvl: 2.5 mg/dL High (03:38)

Electronically Signed By: Parekh, Parag DO
On: 09.01.2024 08:33 PDT

Operative Reports

DOCUMENT NAME:	Operative Reports
SERVICE DATE/TIME:	8/29/2024 12:20 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Al-owir MD,Bassam S (8/29/2024 12:22 PDT)
SIGN INFORMATION:	Al-owir MD,Bassam S (8/29/2024 12:22 PDT)

Procedure Name
Bronchoscopy

Consent
Informed consent obtained from the family

Indication
Abnormal chest CT

Sedation
Moderate sedation protocol with propofol

Complications
None

Operative Reports

Technique
Videoscope was brought to bedside, videoscope advanced through the ET tube without complications, carina was reached. Multiple photos taken, videoscope advanced into the right bronchial tree, multiple photos taken again, bronchoalveolar lavage was done from each tube separately, with the scope retracted to the carina. Videoscope advanced into the left bronchial tree, multiple photos were taken, bronchoalveolar lavage was done from each lobe separately, videoscope retracted the carina, and out of the ET tube

Estimated Blood Loss
None

Findings
Moderate to severe bilateral inflammation, moderate bilateral white thick secretions. High position ET tube

Recommendations
Bronchoalveolar lavage was sent for aerobic culture, anaerobic culture, and fungal culture. Respiratory therapist was instructed to advance the ET tube by 1 to 2 cm and follow-up chest x-ray. We will hold sedation after bronchoscopy and start CPAP trial.

Electronically Signed By: Al-owir, Bassam MD
On: 08.29.2024 12:22 PDT

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male

Admit: 8/28/2024
Disch: Disch Time:
FIN: HHN0005007238115
Readmit Risk: 63 - HIGH 08/30/24 09:45
Attending: Kabsoun MD,Sara H

Orders

Order: **Consult to Case Management (Case Management Consult)**			
Order Date/Time: 9/1/2024 08:33 PDT			
Signed Date/Time: 9/1/2024 08:33 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 9/1/2024 08:33 PDT		End-state Reason:	
Ordering Physician: Moreno MD,Luis A		Consulting Physician:	
Entered By: Moreno MD,Luis A on 9/1/2024 08:33 PDT			
Order Details: 9/1/24 8:33:00 AM PDT, Routine, Reason for Consult: D/C to Home w Home Health, IV: Yes, O2: Yes, DM education and insulin management education, BP and renal function monitoring, RN for safety evaluation, medication reconciliation,, PT/OT for balance and strengthening			
Order Comment:			

Order: **Consult to Case Management**			
Order Date/Time: 8/31/2024 15:07 PDT			
Signed Date/Time: 8/31/2024 15:07 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Consults	Activity Type: Case Management Consults
End-state Date/Time: 8/31/2024 15:07 PDT		End-state Reason:	
Ordering Physician: Moreno MD,Luis A		Consulting Physician:	
Entered By: Moreno MD,Luis A on 8/31/2024 15:07 PDT			
Order Details: 8/31/24 3:07:00 PM PDT, Routine, Reason for Consult: Other Please Specify, IV: Yes, O2: Yes, No Isolation, Patient will need continuous home oxygen 2 L NC, Please provide oxygen tank and concentrator. Dx: COPD, hypoxia NPI: 1649839259 LON: 99			
Order Comment:			

Medications

Documented medications

Order: **atorvastatin (Lipitor 40 mg oral tablet)**
Order Date/Time: 9/1/2024 08:15 PDT
Order Details: 40 mg = 1 Tabs, OG-Tube, qHS, 0 Refill(s), Maintenance, 129.27

Prescription

Order: **doxycycline (doxycycline hyclate 100 mg oral capsule)**
Order Date/Time: 9/1/2024 08:23 PDT
Order Details: 100 mg = 1 Caps, Oral, BID, X 7 Days, # 14 Caps, 0 Refill(s), Acute, 9/8/24 8:23:00 AM PDT, Pharmacy: Walmart Pharmacy 3350, 1 Caps Oral BID,x7 Days, 129.27, kg, 08/28/24 13:31:00 PDT, Weight

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medications

Prescription

Order: **albuterol (ProAir HFA 90 mcg/inh inhalation aerosol)**
Order Date/Time: 9/1/2024 08:18 PDT
Order Details: 2 Puffs, Inhalation, q4H, PRN for wheezing, # 8.5 gm, 0 Refill(s), Maintenance, Pharmacy: Walmart Pharmacy 3350, 2 Puffs Inhalation q4H,PRN:for wheezing, 129.27, kg, 08/28/24 13:31:00 PDT, Weight

Order: **methylPREDNISolone (Medrol Dosepak 4 mg oral tablet)**
Order Date/Time: 9/1/2024 08:18 PDT
Order Details: 1 Packets, Oral, Daily, as directed on package labeling, X 6 Days, # 21 Tabs, 0 Refill(s), Acute, 9/7/24 8:18:00 AM PDT, Pharmacy: Walmart Pharmacy 3350, 1 Packets Oral Daily,x6 Days,Instr:as directed on package labeling, 129.27, kg, 08/28/24 13:31:00 PDT, Weight

Order: **cefdinir (cefdinir 300 mg oral capsule)**
Order Date/Time: 9/1/2024 08:17 PDT
Order Details: 300 mg = 1 Caps, Oral, q12H, X 7 Days, # 14 Caps, 0 Refill(s), Acute, 9/8/24 8:17:00 AM PDT, Pharmacy: Walmart Pharmacy 3350, 1 Caps Oral q12H,x7 Days, 129.27, kg, 08/28/24 13:31:00 PDT, Weight

Order: **insulin aspart (insulin aspart 100 units/mL injectable solution)**
Order Date/Time: 9/1/2024 08:17 PDT
Order Details: Sliding Scale (Intermediate), SubCutaneous, Before Meals and HS, If 180-200: 3 units, 201-230: 4 units, 231-260: 5 units, 261-290: 7 units, 291-320: 9 units, 321-350: 11 units, greater than 350: 13 units., # 1 Each, 0 Refill(s), Maintenance, Pharmacy: Walmart Pharmacy 3350, provide insulin supplies, Sliding Scale (Intermediate) SubCutaneous Before Meals and HS,Instr:If 180-200: 3 units, 201-230: 4 units, 231-260: 5 units, 261-290: 7 units, 291-320: 9 units, 321-350: 11 units, greater than 350: 13 units.,The Maximum Daily Dose is 52 units, 129.27, kg, 08/28/24 13:31:00 PDT, Weight, 52, units

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Orders - Admission/Discharge (Curaspan)

Order: **Placement Change**			
Order Date/Time: 8/30/2024 10:26 PDT			
Signed Date/Time: 8/30/2024 10:26 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 8/30/2024 10:26 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 8/30/2024 10:26 PDT			
Order Details: 8/30/24 10:26:45 AM PDT, Suggested Placement: IMC			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			
Action Type: Order	Action Date/Time: 8/30/2024 10:26 PDT	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 08/30/24 10:26:45 PDT, Suggested Placement: IMC			
Review Information: Nurse Review: Electronically Signed, Novak RN,Shayla on 8/30/2024 11:09 PDT Doctor Cosign: Not Required			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Order: **Discharge Request Task Order**			
Order Date/Time: 9/1/2024 08:33 PDT			
Signed Date/Time: 9/1/2024 08:33 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 9/1/2024 08:33 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 9/1/2024 08:33 PDT			
Order Details: 9/1/24 8:33:00 AM PDT, Home with Home Health			
Order Comment:			
Action Type: Order	Action Date/Time: 9/1/2024 08:33 PDT	Action Personnel: SYSTEM	
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert	
Order Details: 09/01/24 8:33:00 PDT, Home with Home Health			
Review Information: Doctor Cosign: Not Required			
Order Comment:			

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Orders - Admission/Discharge (Curaspan)

Order: **Discharge Request**			
Order Date/Time: 9/1/2024 08:32 PDT			
Signed Date/Time: 9/1/2024 08:32 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 9/1/2024 08:32 PDT		End-state Reason:	
Ordering Physician: Moreno MD,Luis A		Consulting Physician:	
Entered By: Moreno MD,Luis A on 9/1/2024 08:32 PDT			
Order Details: 9/1/24 8:32:00 AM PDT, Home with Home Health			
Order Comment:			

Action Type: Order	Action Date/Time: 9/1/2024 08:32 PDT	Action Personnel: Moreno MD,Luis A
Responsible Provider/Electronically Signed by: Moreno MD,Luis A	Supervising Provider:	Communication Type: Written
Order Details: 09/01/24 8:32:00 PDT, Home with Home Health		
Review Information: Nurse Review: Electronically Signed, Ly RN,Patrick on 9/1/2024 08:54 PDT Doctor Cosign: Not Required		
Order Comment:		

Order: **Change Attending Physician**			
Order Date/Time: 8/30/2024 10:26 PDT			
Signed Date/Time: 8/30/2024 10:26 PDT			
Order Status: Completed	Department Status: Completed	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 8/30/2024 10:39 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 8/30/2024 10:26 PDT			
Order Details: 8/30/24 10:26:45 AM PDT, Admitting: Kabsoun MD, Sara H, Attending: Kabsoun MD, Sara H			
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00			

Action Type: Complete	Action Date/Time: 8/30/2024 10:39 PDT	Action Personnel: Rivera,Lydia
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type:
Order Details: 08/30/24 10:26:45 PDT, Admitting: Kabsoun MD, Sara H, Attending: Kabsoun MD, Sara H		
Review Information: Doctor Cosign: Not Required		
Order Comment:		

Action Type: Order	Action Date/Time: 8/30/2024 10:26 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 08/30/24 10:26:45 PDT, Admitting: Kabsoun MD, Sara H, Attending: Kabsoun MD, Sara H		
Review Information: Nurse Review: Not Reviewed Doctor Cosign: Not Required		
Order Comment: Ordered secondary to modification of admission/observation/transfer order. 1.00		

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Orders - Admission/Discharge (Curaspan)

Order: **Case Management Discharge Notification**			
Order Date/Time: 9/1/2024 08:33 PDT			
Signed Date/Time: 9/1/2024 08:33 PDT			
Order Status: Ordered	Department Status: Ordered	Catalog Type: Admit/Transfer/Discharge	Activity Type: Admit/Transfer/Discharge
End-state Date/Time: 9/1/2024 08:33 PDT		End-state Reason:	
Ordering Physician: SYSTEM		Consulting Physician:	
Entered By: SYSTEM on 9/1/2024 08:33 PDT			
Order Details: 9/1/24 8:33:00 AM PDT			
Order Comment: Home with Home Health			

Action Type: Order	Action Date/Time: 9/1/2024 08:33 PDT	Action Personnel: SYSTEM
Responsible Provider/Electronically Signed by: SYSTEM	Supervising Provider:	Communication Type: Discern Expert
Order Details: 09/01/24 8:33:00 PDT		
Review Information: Doctor Cosign: Not Required		
Order Comment: Home with Home Health		

Medication Administration Summary

Medications

Administration Date/Time: 9/1/2024 13:00 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Admin Details: (Not Done) Patient Refused
Peripheral Pulse Rate, Respiratory Rate, albuterol-ipratropium

Administration Date/Time: 9/1/2024 11:58 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 9/1/2024 11:58 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 4 units
Admin Details: (Auth) SubCutaneous, Right Arm
MAR Glucose Result: 195

Administration Date/Time: 9/1/2024 11:56 PDT
Medication Name: **pantoprazole (Protonix)**
Admin Details: (Not Given) Not Appropriate at this time

Administration Date/Time: 9/1/2024 11:55 PDT
Medication Name: **ethanol topical (Nozin POPswab 62% topical swab)**
Admin Details: (Not Given) Other -see comment

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 9/1/2024 11:55 PDT
Medication Name: **doxycycline + Dextrose 5% in Water intravenous solution 100 mL**
Admin Details: (Not Given) Other -see comment

Administration Date/Time: 9/1/2024 09:24 PDT
Medication Name: **insulin glargine (Lantus)**
Ingredients: insu1000Sol 5 units 0.01 mL
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 206

Administration Date/Time: 9/1/2024 09:21 PDT
Medication Name: **doxazosin**
Ingredients: doxa2Tab 2 mg 1 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 9/1/2024 09:21 PDT
Medication Name: **aspirin (aspirin 81 mg oral tablet, chewable)**
Ingredients: aspi81chewTab 81 mg 1 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 9/1/2024 09:21 PDT
Medication Name: **amLODIPine**
Ingredients: amlo5Tab 10 mg 2 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 9/1/2024 07:28 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 97 bpm; Respiratory Rate: 16 br/min

Administration Date/Time: 9/1/2024 06:58 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 5 units
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 205

Administration Date/Time: 9/1/2024 06:58 PDT
Medication Name: **methylPREDNISolone (methylPREDNISolone sodium succinate (Solu-Medrol))**
Ingredients: methPF40Inj 20 mg 1 mL
Admin Details: (Auth) IV Push, Left Arm

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 9/1/2024 06:58 PDT
Medication Name: **buprenorphine**
Ingredients: bupr2Tab 4 mg 2 Tabs
Admin Details: (Auth) SubLingual

Administration Date/Time: 9/1/2024 03:58 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 1 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 9/1/2024 01:00 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Admin Details: (Not Done) Patient Refused
Peripheral Pulse Rate, Respiratory Rate, albuterol-ipratropium

Administration Date/Time: 8/31/2024 22:11 PDT
Medication Name: **traZODone**
Ingredients: traz50Tab 150 mg 3 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 8/31/2024 22:11 PDT
Medication Name: **nortriptyline**
Ingredients: nort25Cap 25 mg 1 Caps
Admin Details: (Auth) Oral

Administration Date/Time: 8/31/2024 22:11 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 6 units
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 258

Administration Date/Time: 8/31/2024 22:11 PDT
Medication Name: **ethanol topical (Nozin POPswab 62% topical swab)**
Ingredients: Nozin POPswab 62% topical swab 1 amp
Admin Details: (Auth) Topical

Administration Date/Time: 8/31/2024 22:11 PDT
Medication Name: **cefTRIAXone (Rocephin 2 g injection)**
Ingredients: ceft2REC 2 gm 20 mL
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/31/2024 22:11 PDT
Medication Name: **buprenorphine**
Ingredients: bupr2Tab 4 mg 2 Tabs
Admin Details: (Auth) SubLingual

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/31/2024 22:11 PDT
Medication Name: **atorvastatin (Lipitor 40 mg oral tablet)**
Ingredients: ator40Tab 40 mg 1 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 8/31/2024 22:08 PDT
Medication Name: **doxycycline + Dextrose 5% in Water intravenous solution 100 mL**
Ingredients: D5%100ml 100 mL; doxy100REC 100 mg 10 mL
Admin Details: (Auth) IV Piggyback, Right Arm

Administration Date/Time: 8/31/2024 20:59 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 4 mL (Waste: 10 mg)
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/31/2024 19:44 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 90 bpm; Respiratory Rate: 18 br/min

Administration Date/Time: 8/31/2024 17:49 PDT
Medication Name: **furosemide**
Ingredients: furosemide 40 mg/4 mL vial 40 mg 4 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/31/2024 17:49 PDT
Medication Name: **carvedilol**
Ingredients: carv12Tab 12.5 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 8/31/2024 17:12 PDT
Medication Name: **methylPREDNISolone (methylPREDNISolone sodium succinate (Solu-Medrol))**
Ingredients: methPF40Inj 20 mg 0.5 mL
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/31/2024 16:30 PDT
Medication Name: **labetalol**
Admin Details: Auth (Verified)
Medication Administration Reason: Hypertension; Medication Effective: Yes

Administration Date/Time: 8/31/2024 16:15 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL (Waste: 10 mg)
Admin Details: (Auth) IV Push, Left Arm

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/31/2024 16:15 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 6 units
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 246

Administration Date/Time: 8/31/2024 13:58 PDT
Medication Name: **hydrALAZINE**
Admin Details: Auth (Verified)
Medication Effective: Yes; Medication Administration Reason: Hypertension

Administration Date/Time: 8/31/2024 13:28 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/31/2024 13:28 PDT
Medication Name: **buprenorphine**
Ingredients: bupr2Tab 4 mg 2 Tabs
Admin Details: (Auth) SubLingual

Administration Date/Time: 8/31/2024 13:26 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 89 bpm; Respiratory Rate: 20 br/min

Administration Date/Time: 8/31/2024 11:51 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 9 units
Admin Details: (Auth) SubCutaneous, Right Arm
MAR Glucose Result: 273

Administration Date/Time: 8/31/2024 10:11 PDT
Medication Name: **insulin glargine (Lantus)**
Ingredients: insu1000Sol 5 units 0.01 mL
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 301

Administration Date/Time: 8/31/2024 09:37 PDT
Medication Name: **pantoprazole (Protonix)**
Ingredients: pant40vial 40 mg
Admin Details: (Auth) IV Push, Left Upper Forearm

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/31/2024 09:37 PDT
Medication Name: **metoprolol (Lopressor 50 mg oral tablet)**
Ingredients: meto50Tab 50 mg 1 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 8/31/2024 09:37 PDT
Medication Name: **ethanol topical (Nozin POPswab 62% topical swab)**
Ingredients: Nozin POPswab 62% topical swab 1 amp
Admin Details: (Auth) Topical

Administration Date/Time: 8/31/2024 09:37 PDT
Medication Name: **doxazosin**
Ingredients: doxa2Tab 2 mg 1 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 8/31/2024 09:37 PDT
Medication Name: **aspirin (aspirin 81 mg oral tablet, chewable)**
Ingredients: aspi81chewTab 81 mg 1 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 8/31/2024 09:37 PDT
Medication Name: **amLODIPine**
Ingredients: amlo5Tab 10 mg 2 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 8/31/2024 09:36 PDT
Medication Name: **doxycycline + Dextrose 5% in Water intravenous solution 100 mL**
Ingredients: D5%100ml 100 mL; doxy100REC 100 mg 10 mL
Admin Details: (Auth) IV Piggyback, Left Arm

Administration Date/Time: 8/31/2024 08:09 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL (Waste: 10 mg)
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/31/2024 07:11 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Respiratory Rate: 20 br/min

Administration Date/Time: 8/31/2024 05:56 PDT
Medication Name: **methylPREDNISolone (methylPREDNISolone sodium succinate (Solu-Medrol))**
Ingredients: methPF40Inj 20 mg 0.5 mL
Admin Details: (Auth) IV Push, Left Arm

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/31/2024 05:56 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 5 units 0.05 mL
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 208

Administration Date/Time: 8/31/2024 05:56 PDT
Medication Name: **buprenorphine**
Ingredients: bupr2Tab 4 mg 2 Tabs
Admin Details: (Auth) SubLingual

Administration Date/Time: 8/31/2024 05:47 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/31/2024 02:23 PDT
Medication Name: **hydrALAZINE**
Admin Details: Auth (Verified)
Medication Effective: Yes; Medication Administration Reason: Hypertension

Administration Date/Time: 8/31/2024 02:05 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 89 bpm; Respiratory Rate: 17 br/min

Administration Date/Time: 8/31/2024 01:53 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/31/2024 01:53 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 5 units 0.05 mL
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 225

Administration Date/Time: 8/30/2024 22:22 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 15 units 0.15 mL
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 330

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/30/2024 22:22 PDT
Medication Name: **buprenorphine**
Ingredients: bupr2Tab 4 mg 2 Tabs
Admin Details: (Auth) SubLingual

Administration Date/Time: 8/30/2024 21:33 PDT
Medication Name: **metoprolol (Lopressor 50 mg oral tablet)**
Ingredients: meto50Tab 50 mg 1 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 8/30/2024 21:33 PDT
Medication Name: **ethanol topical (Nozin POPswab 62% topical swab)**
Ingredients: Nozin POPswab 62% topical swab 1 amp
Admin Details: (Auth) Topical

Administration Date/Time: 8/30/2024 21:33 PDT
Medication Name: **cefTRIAXone (Rocephin 2 g injection)**
Ingredients: ceft2REC 2 gm 20 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/30/2024 21:33 PDT
Medication Name: **atorvastatin (Lipitor 40 mg oral tablet)**
Ingredients: ator40Tab 40 mg 1 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 8/30/2024 21:28 PDT
Medication Name: **doxycycline + Dextrose 5% in Water intravenous solution 100 mL**
Ingredients: doxy100REC 100 mg 10 mL; D5%100ml 100 mL
Admin Details: (Auth) IV Piggyback, Left Arm

Administration Date/Time: 8/30/2024 20:05 PDT
Medication Name: **doxazosin**
Ingredients: doxa2Tab 2 mg
Admin Details: (Auth) Oral

Administration Date/Time: 8/30/2024 19:15 PDT
Medication Name: **hydrALAZINE**
Admin Details: Auth (Verified)
Medication Effective: No; Medication Administration Reason: Hypertension

Administration Date/Time: 8/30/2024 19:00 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Admin Details: (Not Done) Not Appropriate at this Time
albuterol-ipratropium, Peripheral Pulse Rate, Respiratory Rate

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/30/2024 18:45 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 1 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/30/2024 18:04 PDT
Medication Name: **labetalol**
Admin Details: Auth (Verified)
Medication Administration Reason: Hypertension; Medication Effective: Yes

Administration Date/Time: 8/30/2024 17:49 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL (Waste: 10 mg)
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/30/2024 17:49 PDT
Medication Name: **methylPREDNISolone (methylPREDNISolone sodium succinate (Solu-Medrol))**
Ingredients: methPF40Inj 20 mg 0.5 mL
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/30/2024 17:49 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 6 units
Admin Details: (Auth) SubCutaneous, Left Upper Arm
MAR Glucose Result: 260

Administration Date/Time: 8/30/2024 15:30 PDT
Medication Name: **hydrALAZINE**
Admin Details: Auth (Verified)
Medication Effective: Yes; Medication Administration Reason: Hypertension

Administration Date/Time: 8/30/2024 15:00 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/30/2024 15:00 PDT
Medication Name: **buprenorphine**
Ingredients: bupr2Tab 4 mg 2 Tabs
Admin Details: (Auth) SubLingual

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/30/2024 13:50 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 6 units
Admin Details: (Auth) SubCutaneous, Left Lowr Abdom Quad
MAR Glucose Result: 247

Administration Date/Time: 8/30/2024 13:50 PDT
Medication Name: **metoprolol (Lopressor 50 mg oral tablet)**
Ingredients: meto50Tab 50 mg 1 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 8/30/2024 13:50 PDT
Medication Name: **amLODIPine**
Ingredients: amlo5Tab 10 mg 2 Tabs
Admin Details: (Auth) Oral

Administration Date/Time: 8/30/2024 13:50 PDT
Medication Name: **aspirin (aspirin 81 mg oral tablet, chewable)**
Ingredients: aspi81chewTab 81 mg 1 Tabs
Admin Details: (Auth) OG-Tube

Administration Date/Time: 8/30/2024 13:00 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Admin Details: (Not Done) Patient Out on Pass
Peripheral Pulse Rate, albuterol-ipratropium, Respiratory Rate

Administration Date/Time: 8/30/2024 11:40 PDT
Medication Name: **labetalol**
Admin Details: Auth (Verified)
Medication Administration Reason: Hypertension; Medication Effective: Yes

Administration Date/Time: 8/30/2024 11:25 PDT
Medication Name: **labetalol**
Ingredients: labetalol 20 mg/4 mL inj 10 mg 2 mL (Waste: 10 mg)
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/30/2024 10:28 PDT
Medication Name: **morphine**
Admin Details: Auth (Verified)
Medication Administration Reason: Pain; Pain Assessment Tool Utilized: Numeric; Numeric Pain Scale: 3

Administration Date/Time: 8/30/2024 10:28 PDT
Medication Name: **hydrALAZINE**
Admin Details: Auth (Verified)
Medication Administration Reason: Hypertension; Medication Effective: Yes

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/30/2024 10:03 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 9 units
Admin Details: (Auth) SubCutaneous, Left Lowr Abdom Quad
MAR Glucose Result: 276

Administration Date/Time: 8/30/2024 09:58 PDT
Medication Name: **morphine**
Ingredients: morp4mg/mLinj 4 mg 1 mL
Admin Details: (Auth) IV Push, Right Arm
Respiratory Rate: 20 br/min

Administration Date/Time: 8/30/2024 09:58 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/30/2024 09:58 PDT
Medication Name: **pantoprazole (Protonix)**
Ingredients: pant40vial 40 mg
Admin Details: (Auth) IV Push, Right Arm

Administration Date/Time: 8/30/2024 09:58 PDT
Medication Name: **ethanol topical (Nozin POPswab 62% topical swab)**
Ingredients: Nozin POPswab 62% topical swab 1 amp
Admin Details: (Auth) Topical

Administration Date/Time: 8/30/2024 09:58 PDT
Medication Name: **doxycycline + Dextrose 5% in Water intravenous solution 100 mL**
Ingredients: D5%100ml 100 mL; doxy100REC 100 mg 10 mL
Admin Details: (Auth) IV Piggyback, Right Arm

Administration Date/Time: 8/30/2024 07:53 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 85 bpm; Respiratory Rate: 18 br/min

Administration Date/Time: 8/30/2024 07:03 PDT
Medication Name: **morphine**
Admin Details: Auth (Verified)
Medication Administration Reason: Pain; Pain Assessment Tool Utilized: Numeric; Numeric Pain Scale: 5 = Moderate pain

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/30/2024 07:03 PDT
Medication Name: **hydrALAZINE**
Admin Details: Auth (Verified)
Medication Effective: No; Medication Administration Reason: Hypertension

Administration Date/Time: 8/30/2024 06:33 PDT
Medication Name: **morphine**
Ingredients: morp4mg/mLinj 4 mg 1 mL
Admin Details: (Auth) IV Push, Left Arm
Respiratory Rate: 21 br/min

Administration Date/Time: 8/30/2024 06:33 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/30/2024 06:05 PDT
Medication Name: **methylPREDNISolone (methylPREDNISolone sodium succinate (Solu-Medrol))**
Ingredients: methPF40Inj 40 mg 1 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/30/2024 06:05 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 15 units
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 329

Administration Date/Time: 8/30/2024 06:05 PDT
Medication Name: **furosemide (furosemide 10 mg/mL injectable solution)**
Ingredients: furosemide 40 mg/4 mL vial 40 mg 4 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/30/2024 05:47 PDT
Medication Name: **potassium chloride (potassium chloride 10 mEq/100 mL intravenous solution)**
Ingredients: pota/swfi 10/100 10 mEq 100 mL
Admin Details: (Auth) IV Piggyback, Right Arm

Administration Date/Time: 8/30/2024 04:50 PDT
Medication Name: **potassium chloride (potassium chloride 10 mEq/100 mL intravenous solution)**
Ingredients: pota/swfi 10/100 10 mEq 100 mL
Admin Details: (Auth) IV Piggyback, Right Arm

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Medications

Administration Date/Time: 8/30/2024 03:05 PDT
Medication Name: **hydrALAZINE**
Admin Details: Auth (Verified)
Medication Effective: Yes; Medication Administration Reason: Hypertension

Administration Date/Time: 8/30/2024 02:48 PDT
Medication Name: **morphine**
Admin Details: Auth (Verified)
Medication Administration Reason: Pain; Numeric Pain Scale: Unable to obtain score; sleeping (eyes closed), no distress; Pain Assessment Tool Utilized: Numeric

Administration Date/Time: 8/30/2024 02:35 PDT
Medication Name: **hydrALAZINE**
Ingredients: hydrALAZINE 20 mg/mL vial 10 mg 0.5 mL
Admin Details: (Auth) IV Push, Left Arm

Administration Date/Time: 8/30/2024 02:18 PDT
Medication Name: **morphine**
Ingredients: morp4mg/mLinj 4 mg 1 mL
Admin Details: (Auth) IV Push, Left Arm
Respiratory Rate: 19 br/min

Administration Date/Time: 8/30/2024 02:18 PDT
Medication Name: **insulin lispro (insulin lispro HumaLOG correctional scale High)**
Ingredients: insu1000Sol 6 units
Admin Details: (Auth) SubCutaneous, Left Arm
MAR Glucose Result: 246

Administration Date/Time: 8/30/2024 01:37 PDT
Medication Name: **ipratropium-albuterol (DuoNeb)**
Ingredients: albuterol-ipratropium Inh Sol 3 mL 3 mL
Admin Details: (Auth) NEB
Peripheral Pulse Rate: 94 bpm; Respiratory Rate: 18 br/min

Administration Date/Time: 8/30/2024 00:44 PDT
Medication Name: **acetaminophen (acetaminophen 10 mg/mL intravenous solution)**
Admin Details: Auth (Verified)
Medication Administration Reason: Fever; Medication Effective: No

Administration Date/Time: 8/30/2024 00:14 PDT
Medication Name: **acetaminophen (acetaminophen 10 mg/mL intravenous solution)**
Ingredients: acet1000mg/100mL 650 mg 65 mL
Admin Details: (Auth) IV Piggyback, Right Antecubital Foss

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Medication Administration Summary

Continuous Infusions

Administration Date/Time: 8/30/2024 05:00 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 0.4 mcg/kg/hr
Admin Details: (Rate Change) (Auth) 12.93 mL/hr, Left Lower Forearm, 129.27 kg

Administration Date/Time: 8/30/2024 04:00 PDT to 8/30/2024 04:59 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 51.6 mcg; Premix NS Sed 12.9 mL
Admin Details: (Infuse) (Auth) 12.9 mL, 12.93 mL/hr, Left Lower Forearm

Administration Date/Time: 8/30/2024 04:00 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 0.4 mcg/kg/hr
Admin Details: (Rate Change) (Auth) 12.93 mL/hr, Left Lower Forearm, 129.27 kg

Administration Date/Time: 8/30/2024 03:00 PDT to 8/30/2024 03:59 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 51.68 mcg; Premix NS Sed 12.92 mL
Admin Details: (Infuse) (Auth) 12.92 mL, 12.93 mL/hr, Left Lower Forearm

Administration Date/Time: 8/30/2024 03:00 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 0.4 mcg/kg/hr
Admin Details: (Rate Change) (Auth) 12.93 mL/hr, Left Lower Forearm, 129.27 kg

Administration Date/Time: 8/30/2024 02:45 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 400 mcg 100 mL 0.4 mcg/kg/hr; Premix NS Sed 100 mL
Admin Details: (Begin Bag) (Auth) 100 mL, 12.93 mL/hr, Left Lower Forearm, 129.27 kg

Administration Date/Time: 8/30/2024 02:00 PDT to 8/30/2024 02:59 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 67.6 mcg; Premix NS Sed 16.9 mL
Admin Details: (Infuse) (Auth) 16.9 mL, 12.93 mL/hr, Left Lower Forearm

Administration Date/Time: 8/30/2024 02:00 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 0.6 mcg/kg/hr
Admin Details: (Rate Change) (Auth) 19.39 mL/hr, Left Lower Forearm, 129.27 kg

Administration Date/Time: 8/30/2024 01:00 PDT to 8/30/2024 01:59 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 77.6 mcg; Premix NS Sed 19.4 mL
Admin Details: (Infuse) (Auth) 19.4 mL, 19.39 mL/hr, Left Lower Forearm

Medication Administration Summary

Continuous Infusions

Administration Date/Time: 8/30/2024 01:00 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 0.6 mcg/kg/hr
Admin Details: (Rate Change) (Auth) 19.39 mL/hr, Left Lower Forearm, 129.27 kg

Administration Date/Time: 8/30/2024 00:00 PDT to 8/30/2024 00:59 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 77.6 mcg; Premix NS Sed 19.4 mL
Admin Details: (Infuse) (Auth) 19.4 mL, 19.39 mL/hr, Left Lower Forearm

Administration Date/Time: 8/30/2024 00:00 PDT
Medication Name: **dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL**
Ingredients: CHARGE ONLY (dexmed400/100NS) 0.6 mcg/kg/hr
Admin Details: (Rate Change) (Auth) 19.39 mL/hr, Left Lower Forearm, 129.27 kg

Physical Therapy Forms

DOCUMENT TYPE:	PT Daily Documentation - Text
SERVICE DATE/TIME:	9/1/2024 08:34 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Coulson PT,DPT,Jeremy (9/1/2024 09:14 PDT)
SIGNED INFORMATION:	Coulson PT,DPT,Jeremy (9/1/2024 09:14 PDT)

PT Daily Documentation Entered On: 9/1/2024 9:17 PDT
Performed On: 9/1/2024 8:34 PDT by Coulson PT, DPT, Jeremy

Review/Treatments Provided
PT Goals : PT Short Term Goals

08/30/2024
Other PT Goal #1: Independent in bed mobility; Initial
Other PT Goal #2: Close supervision to transfer bed to chair; Initial
Other PT Goal #3: Close supervision to ambulate 300' with a SPC; Initial
PT Plan : Treatment Frequency: Other: 3-7 x week Performed By: Coulson PT, DPT, Jeremy 08/30/2024
Treatment Duration: 5-6 weeks Performed By: Coulson PT, DPT, Jeremy 08/30/2024
Planned Treatments: Balance training, Bed mobility training, Gait training, Therapeutic exercises, Transfer training Performed By: Coulson PT, DPT, Jeremy 08/30/2024
Physical Therapy Orders : Miscellaneous Physical Therapy Task - 08/30/24 7:23:00 PDT, Routine, Stop date 08/30/24 7:23:00 PDT, Stretcher, Yes, Yes, 3-7 x week for 6 weeks
Pain Symptoms : No
Suspected Stroke Patient : No
Post-Acute Discharge Therapy Recommendation PT : No further skilled therapy services warranted

Physical Therapy Forms

IDR PT Recommend/Goal : Safe for home DC from PT standpoint

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Mobility

Mobility Grid

Roll Left : Rehab Complete independence
Roll Right : Rehab Complete independence
Supine to Sit : Rehab Complete independence
Sit to Supine : Rehab Complete independence
Transfer Sit to Stand : Close supervision
Transfer Stand to Sit : Close supervision
Transfer Bed to and From Chair : Close supervision
Transfer Toilet : Close supervision
Walk 10 ft : Close supervision
Walk 50 ft, 2 Turns : Close supervision
Walk 150 ft : Close supervision

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Balance

General Balance : Both
General Balance Assessment : Good

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Education

Teaching Method : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Physical Therapy Education Grid

Ambulation : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

Additional Information

Additional Information PT : Patient lying in bed on entry, ready for PT. eager to use toilet. Independent in bed mobility, supervision to stand, ambulate 15' to toilet, transfer on and off of it, then ambulate 300' further in hallway with no device. Patient with slow, shuffle, kyphotic gait. Cued for step length, gait speed, upright posture. Saturating mid 90's on 6L oxygen. Patient returned to room and left in recliner with call light
Environmental Safety Implemented : Bed in low position, Call device within reach, Lock wheeled equipment, Personal items within reach

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

PT Non Timed Modalities 2024

PT Treatment Start Time : 9/1/2024 8:34 PDT
PT Treatment End Time : 9/1/2024 9:12 PDT

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

PT Timed Modalities 2024

PT Timed Modality Treatment Start Time : 9/1/2024 8:34 PDT
PT Timed Modality Treatment End Time : 9/1/2024 9:12 PDT
Rehab Patient : No
PT Timed Min Total : 38
PT Timed Unit Total : 3
PT THERAPEUTIC ACT 15MIN : 8

Physical Therapy Forms

Activity Therapeutic Num of Units : 1
PT GAIT TRAINING 15MIN : 30
Gait Training Num of Units : 2

Coulson PT, DPT, Jeremy - 9/1/2024 9:14 PDT

DOCUMENT TYPE:	PT Examination - Text
SERVICE DATE/TIME:	8/30/2024 06:30 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Coulson PT,DPT,Jeremy (8/30/2024 07:15 PDT)
SIGNED INFORMATION:	Coulson PT,DPT,Jeremy (8/30/2024 07:15 PDT)

PT Evaluation/Reevaluation Entered On: 8/30/2024 7:23 PDT
Performed On: 8/30/2024 6:30 PDT by Coulson PT, DPT, Jeremy

General Info
PT Assessment Type : Initial Evaluation
Physical Therapy Orders : PT Evaluation and Treatment - 08/29/24 19:31:00 PDT, Routine, Stop date 08/29/24 19:31:00 PDT, Stretcher, Yes, Yes
Precautions RTF : Active Orders:
» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Ordered
» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered

Discontinued Orders:
» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued
» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled
» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed
» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued
» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued
» Isolation, Precautions: COVID-19, Discontinued

Orientation Assessment Rehab : Oriented x 4
Affect/Behavior Rehab : Appropriate
Basic Command Following : Intact
Safety/Judgment : Intact
Pain Symptoms : No
Suspected Stroke Patient : No

Physical Therapy Forms

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Home Environment
Living Environment : Home Environment

No qualifying data available
Living Situation : Home independently
Lives With : Spouse
Lives In : Single level home

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Location Bed
Primary Bedroom : 1st floor
Primary Bathroom : 1st floor
Kitchen : 1st floor
Laundry : 1st floor

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Patient's Responsibilities Rehab : Community mobility, Personal ADL, Shopping

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Home Environment II
Living Environment : Home Environment

No qualifying data available
Home Equipment Rehab : Cane

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Prior Functional Status Grid
ADL : Independent
Mobility : Independent
Instrumental ADL : Independent
Cognitive-Communication Skills : Independent

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

LE Range/Strength
LE Overall Range of Motion Grid
Left Lower Extremity Passive Range : Within functional limits
Left Lower Extremity Active Range : Within functional limits
Right Lower Extremity Passive Range : Within functional limits
Right Lower Extremity Active Range : Within functional limits

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Left Lower Extremity Strength Grid
Hip Flexion : 4
Hip Extension : 4
Hip Abduction : 4
Hip Adduction : 4
Hip External Rotation : 4
Hip Internal Rotation : 4
Knee Flexion : 4
Knee Extension : 4
Ankle Dorsiflexion : 4
Ankle Plantarflexion : 4
Ankle Eversion : 4
Ankle Inversion : 4

Physical Therapy Forms

Great Toe Extension : 4

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Right Lower Extremity Strength Grid
Hip Flexion : 4
Hip Extension : 4
Hip Abduction : 4
Hip Adduction : 4
Hip External Rotation : 4
Hip Internal Rotation : 4
Knee Flexion : 4
Knee Extension : 4
Ankle Dorsiflexion : 4
Ankle Plantarflexion : 4
Ankle Eversion : 4
Ankle Inversion : 4
Great Toe Extension : 4

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

LE/Trunk Tone
remainder (less than 1/2 of ROM)
Left Lower Extremity : Normal
Right Lower Extremity : Normal
Trunk Tone : Normal

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Balance
General Balance : Both
General Balance Assessment : Fair

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Mobility
Mobility Grid
Roll Left : Rehab Contact guard assistance
Roll Right : Rehab Contact guard assistance
Supine to Sit : Rehab Contact guard assistance
Sit to Supine : Rehab Contact guard assistance
Transfer Sit to Stand : Rehab Contact guard assistance
Transfer Stand to Sit : Rehab Contact guard assistance
Transfer Bed to and From Chair : Rehab Contact guard assistance
Walk 10 ft : Rehab Contact guard assistance
Walk 50 ft, 2 Turns : Rehab Contact guard assistance

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Assessment
PT Impairments or Limitations : Ambulation deficits, Balance deficits, Bed mobility deficits, Endurance deficits, Strength deficits, Transfer deficits

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Education
Teaching Method : Demonstration, Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Physical Therapy Education Grid

Physical Therapy Forms

Ambulation : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Bed Mobility : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Exercise Program : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision
Physical Therapy Plan of Care : Verbalizes understanding, Demonstrates, Needs further teaching, Needs practice/supervision

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Long Term Goals
PT Patient,Caregiver Goal : Go home when able

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

PT Long Term Goals Rehab

	Long Term Goal 1	Long Term Goal 2
Goal :	Independent to transfer bed to chair	Independent to ambulate 900'
Status :	Initial	Initial
Timeframe To Complete :	6 weeks	6 weeks
	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Short Term Goals
Other PT Goals Grid

	Goal #1	Goal #2	Goal #3
Goal :	Independent in bed mobility	Close supervision to transfer bed to chair	Close supervision to ambulate 300' with a SPC
Status :	Initial	Initial	Initial
Timeframe To Complete :	1 week	1 week	1 week
	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT	Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Plan
PT Anticipated Treatments, Needs : Balance training, Bed mobility training, Gait training, Therapeutic exercises, Transfer training
PT Frequency Rehab : Other: 3-7 x week
PT Duration Rehab : 5-6 weeks
PT Intensity Per Day : 0-30 minutes
Rehabilitation Potential : Good

Physical Therapy Forms

Post-Acute Discharge Therapy Recommendation PT : No further skilled therapy services warranted
IDR PT Recommend/Goal : Safe for home DC from PT standpoint, will progress as able
Other PT Treatment Provided : After PT eval patient transferred to a recliner and sat x 5 min to catch his breath, Noted to be saturating at 93% on 6L. Paitent CGA to stand and ambulate 50' back to room due to weakness, slow, shuffle, kyphotic gait with SOB and balance deficits. Cued for step length, gait speed, upright posture. Left in recliner with lap belt and call light. RN present with handoff given
PT Plan/Goals Established w Pt/Caregiver : Yes

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Additional Information
Additional Information PT : MD orders, chart reviewed. Pre/post handoff performed w/ RN and informed of patient participation in therapy. Patient ID verified via 2 identifiers (Name, DOB). On arrival patient semisupine in bed, alert, oriented x 4, coherent, and cooperative. Patient agreed to PT evaluation and POC. Patient CGA in all functional mobility and is able to sit, stand, transfer in and out of chair and ambulate 50' with no device and only slow, shuffle, kyphotic, unsteady gait with slight SOB. Patient will benefit from PT to address bed mobility, transfer and gait deficits. After PT Evaluation and POC discussion patient left semisupine in bed, bed alarm on, CLIr, all needs met.

Dx: Patient is a 52 year old male admitted with cardiac arrest

PMH: DM, CHF, COPD, HTN, carotid endartectomy

PLOF: Modified independent in community ambulation

DME: uses a SPC
DC recs: home with family
Environmental Safety Implemented : Bed in low position, Call device within reach, Lock wheeled equipment, Personal items within reach

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

PT Non Timed Modalities 2024
PT Treatment Start Time : 8/30/2024 6:30 PDT
PT Treatment End Time : 8/30/2024 6:44 PDT
PT Eval High Complexity Total Number Min : 1
PT Total Non Timed : 1

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

PT Timed Modalities 2024
PT Timed Modality Treatment Start Time : 8/30/2024 6:45 PDT
PT Timed Modality Treatment End Time : 8/30/2024 7:10 PDT
Rehab Patient : No
PT Timed Min Total : 25
PT Timed Unit Total : 2
PT GAIT TRAINING 15MIN : 25
Gait Training Num of Units : 2

Coulson PT, DPT, Jeremy - 8/30/2024 7:15 PDT

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Speech Therapy Forms

DOCUMENT TYPE:	Clinical Swallowing - Text
SERVICE DATE/TIME:	8/30/2024 10:49 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 11:28 PDT)
SIGNED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 11:28 PDT)

ST Bedside Swallow Eval Entered On: 8/30/2024 11:32 PDT
Performed On: 8/30/2024 10:49 PDT by Castilo CCC-SLP, Noreen ann

General Information
Speech Orders : ST Bedside Swallow Exam - 08/29/24 19:32:00 PDT, Bedside Swallow Exam, Routine, Stop date 08/29/24 19:32:00 PDT, Stretcher, IV Yes, O2 Yes, 17433508165.0
Speech Therapy Eval and Treat - 08/29/24 19:32:00 PDT, Bedside Swallow Exam, Routine, Stop date 08/29/24 19:32:00 PDT, Stretcher, IV Yes, O2 Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered

Discontinued Orders:
» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Deleted
» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued
» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled
» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed
» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued
» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued
» Isolation, Precautions: COVID-19, Discontinued

History of Swallow Studies : No qualifying data available
Level of Consciousness Rehab : Alert
Affect/Behavior Rehab : Cooperative
Basic Command Following : Intact
Orientation Assessment Rehab : Identifies self
Pain Interfering With SLP Treatment : No
Culture/Spiritual Beliefs to Incorporate : No

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Pre-Assessment
Oxygen Therapy : Nasal cannula

Speech Therapy Forms

Tracheostomy Information : Oxygen Flow Rate: 5 L/min Performed by: Mower, Vanessa 08/30/24 07:51:00 FIO2: 30 Performed by: Stansfield RN, Erica 08/29/24 16:00:00

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Oral-Facial Exam

Facial Movement : Symmetric resting/crying
Dentition : Edentulous

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Oral Structure Grid
Labial Structure : Within functional limits
Mandible : Within functional limits
Lingual Structure : Within functional limits
Hard Palate : Within functional limits
Soft Palate : Within functional limits

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Oral Function Grid
Labial : Within functional limits
Mandible : Within functional limits
Lingual : Within functional limits
Velopharyngeal : Within functional limits

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Secretion Management : Reduced

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Upper Body Control Grid
Head Control : Upright centered position
Trunk Control : Upright centered position

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Swallowing Exam

Patient Position : Upright 90 degrees

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Liquids Swallowing Exam Adult Grid

	Ice Chips	Thin Liquid by Teaspoon	Thin Liquid by Straw
Bolus Size :		5 ml	
Total Intake :	Other: 5x	Other: 5x	3-4 ounces
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist	Speech-Language Pathologist
Oral Symptoms :	None observed	None observed	None observed
Pharyngeal Symptoms :	None observed	None observed	Cough/Throat clear after swallow
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Consistency Swallowing Exam Adult Grid

Speech Therapy Forms

	Puree Consistency	Regular Consistency
Bolus Size :	5 ml	
Total Intake :	1-2 ounces	Other: one quarter graham cracker
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist
Oral Symptoms :	None observed	Prolonged mastication
Pharyngeal Symptoms :	Wet vocal quality after swallow	Wet vocal quality after swallow
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Assessment/Recommendations
Clinical Swallowing Impression Grid
Oral Dysphagia : Liquids, Solids
Pharyngeal Dysphagia : Liquids, Solids

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

NPO Status : Except for ice chips
Clinical Swallowing Recommendations : Dysphagia treatment
Assessment Recommendations ST Daily : Chart reviewed, consulted with Shayla RN- cleared pt for evaluation. Swallow evaluation indicated d/t recent extubation. Pt Identified via name and DOB. Pt is agreeable to evaluation. Pt is a 52 year old male to Henderson Hospital for SOB at home.
Dx: intubated 08/28-08/29; COPD exacerbation, SOB, sepsis, O2 dependent, resp distress, resp failure, elevate BNP, hypomagnesemia, hyponatremia, AKI, hypophosphatemia
Hyperglycemia d/t DM, leukocytosis, fever, HTN emergency, morbid obesity, CHF exacerbation, bil lower extremity edema, s/p left endarterectomy, elevated lactic acid, elevated CRP, PUI COVID; bronchoscopy 8/29 - mod to sev bil inflammation, mod bil white thick secretions, high position ET tube.
Significant hx includes but not limited to DM, HTN, diastolic CHF, COPD, hyperlipidemia, disc disorder, ESBL ecoli, gout, HLD, HTN, bil ICA stenosis s/p endarterectomy, EGD w/ biopsy 06/2022
Home Diet: Regular consistencies
Current Diet: NPO
WBC: WNL
CXR: 8/30 2. Low lung volumes with bronchovascular crowding. Bibasilar airspace disease and/or atelectasis.
CT Head: 8/28 no acute intracranial abnormality

IMPRESSION:
Severe oropharyngeal dysphagia characterized by s/s suspected aspiration, observed continued wet vocal quality t/o this evaluation, pt reported r/t neck surgery a month ago and was told there will be swelling. However noted reduced O2 sat during swallow completion with PO trials. See Swallowing Exam Section for details.

Speech Therapy Forms

RECOMMENDATIONS:
NPO, except ice chips with aspiration precautions and oral care TID to reduce risk of aspiration PNA
Alternative means nutrition, hydration, and medication-Consider NGT.
FEES to fully evaluate swallow function

ST to treat for dysphagia
Results and recommendations discussed with: Shayla RN
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Education
Learner(s) Present for Session : Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Document learning evaluation for Responsible Learner(s)
Speech Language/Pathology Tx Plan : Verbalizes understanding
Swallowing Strategies : Verbalizes understanding

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Long Term Goals
SLP Patient, Caregiver Goals : "to eat food"

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will tolerate least restrictive consistencies with risk of aspiration reduced as much as reasonably possible.
Status :	Initial
Timeframe To Complete :	9/13/24
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Short Term Goals
Other SLP Short Term Goal Grid

	Goal #1	Goal #2
Other :	Pt will tolerate 90ml challenge	Pt will perform compensatory

Speech Therapy Forms

	vs instrumental swallow without uncompensated aspiration or penetration on thin liquids	strategies to protect the airway and improve clearance of the bolus in 80% of trials independently
Status :	Initial	Initial
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Plan
Planned Treatments : Dysphagia therapy
Frequency : 3-5 times per week
Duration : 1-2 weeks
SLP Intensity Per Day : Other: as tolerated
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
Rehabilitation Potential : Good
Plan/Goals Established With Patient/Caregiver : Yes
Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Additional Information
Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach
Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

ST Non Timed Modalities 2024
ST Treatment Start Time : 8/30/2024 10:49 PDT
ST Treatment End Time : 8/30/2024 10:59 PDT
ST Eval Oral/Swallow Func Minutes : 10
ST Total Non Timed : 10
Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

DOCUMENT TYPE: Inpatient SLP Daily Documentation - Text
SERVICE DATE/TIME: 8/31/2024 14:02 PDT
RESULT STATUS: Auth (Verified)
PERFORMED INFORMATION: Castilo CCC-SLP,Noreen ann (8/31/2024 15:27 PDT)
SIGNED INFORMATION: Castilo CCC-SLP,Noreen ann (8/31/2024 15:27 PDT)

ST Daily Documentation Entered On: 8/31/2024 15:28 PDT
Performed On: 8/31/2024 14:02 PDT by Castilo CCC-SLP, Noreen ann

Review/Treatments Provided

Speech Therapy Forms

Speech Orders : Miscellaneous Speech Therapy Task - 08/30/24 11:32:00 PDT, Routine, Stop date 08/30/24 11:32:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes
Speech Language Plan : Treatment Frequency: 2-4 times per week (modified) Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
Treatment Duration: 1-2 weeks (modified) Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
Planned Treatments: Dysphagia therapy (modified) Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
Short Term Goals Reviewed : Yes
Long Term Goals Reviewed : Yes
Speech Language Goals RTF : SLP Short Term Goals

08/30/2024
Other Short Term Goals Goal #2: Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently (modified); Progressing, continue (modified)
Other Short Term Goals Goal #3: Pt will tolerate regular PO trials with no overt s/s of apsiration in 5 out o5 trials (modified); Initial (modified)
Pain Interfering With SLP Treatment : No

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Short Term Goals

Other SLP Short Term Goal Grid

	Goal #1	Goal #2	Goal #3
Other :	Pt will tolerate 90ml challenge vs instrumental swallow without uncompensated aspiration or penetration on thin liquids	Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently	Pt will tolerate regular PO trials with no overt s/s of apsiration in 5 out o5 trials
Status :	Goal met	Progressing, continue	Progressing, continue
Date Met :	8/30/2024 PDT		
	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Long Term Goals

SLP Patient, Caregiver Goals : "to eat food"

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will tolerate least restrictive consistencies

Speech Therapy Forms

	with risk of aspiration reduced as much as reasonably possible.
Status :	Progressing, continue
Timeframe To Complete :	9/13/24
	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Education
Learner(s) Present for Session : Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Speech Education Grid
Speech Language/Pathology Tx Plan : Verbalizes understanding
Swallowing Strategies : Verbalizes understanding, Needs further teaching

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Assessment/Recommendations
Assessment Recommendations ST Daily : S: Chart reviewed, consulted with Alejandra RN- cleared pt for tx. Pt alert, agreeable treatment. Clinician discussed with Alejandra RN re: FEES findings with images in document.

O: Ordered textures: IDDSI Soft and bite size with thin liquids; WBC: WNL; CXR: 8/30 1. Interval removal of the endotracheal tube and enteric tube. 2. Low lung volumes with bronchovascular crowding. Bibasilar airspace disease and/or atelectasis.
PO trials:
Thin liquids via straw 1-3oz: no overt s/s of aspiration
Regular graham cracker one half piece: prolonged mastication, no overt s/s of aspiration, independent use of liquid wash
Compensatory/safe swallow strategies: upright in bed, small bites/sips, slow rate for PO intake, swallow two times
Education: s/s of aspiration, swallowing strategies, POC. Pt verbalized understanding and had no further questions at this time. See Also Education Grid

A: pt continues to exhibit oropharyngeal dysphagia r/t FEES on 08/30/24 noted continued wet gurgly voice prior to and after PO intake not r/t aspiration concerns at this time

P: Recommendations
Food Consistencies: IDDSI 6 Soft and Bite Sized
Liquid Consistencies: Thin Liquids
Meds: Whole with puree

Speech Therapy Forms

Supervision/Assist: Distant supervision needed for feeding (provided initial cueing to initiate swallow strategies check on the patient frequently and assess the patient progress at least two to three times during the meal)

Aspiration precautions:
Sit upright as close to 90 degrees as possible,
remain upright 30 mins after meals
Small bites/sips
Swallow two times
Alternate solids/liquids

ST to f/u per POC
Consider further imaging of neck tissue
Consider f/u ENT outpatient when feasible
Discussed with: Alejandra RN

Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Additional Information

Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

ST Non Timed Modalities 2024

ST Treatment Start Time : 8/31/2024 14:02 PDT
ST Treatment End Time : 8/31/2024 14:11 PDT
ST TREATMENT FOR FEED Minutes : 9
ST Total Non Timed : 9

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

DOCUMENT TYPE:	FEES - Text
SERVICE DATE/TIME:	8/30/2024 12:58 PDT
RESULT STATUS:	Modified
PERFORMED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 14:29 PDT)
SIGNED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 14:39 PDT); Castilo CCC-SLP,Noreen ann (8/30/2024 14:29 PDT)

ST FEES Entered On: 8/30/2024 14:39 PDT
Performed On: 8/30/2024 12:58 PDT by Castilo CCC-SLP, Noreen ann

General Information

Speech Orders : FEES - 08/30/24 11:33:00 PDT, Routine, Stop date 08/30/24 11:33:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes
Miscellaneous Speech Therapy Task - 08/30/24 11:32:00 PDT, Routine, Stop date 08/30/24 11:32:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes

Speech Therapy Forms

~~{ [FEES - 08/30/24 11:33:00 PDT, Routine, Stop date 08/30/24 11:33:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes~~
~~Miscellaneous Speech Therapy Task - 08/30/24 11:32:00 PDT, Routine, Stop date 08/30/24 11:32:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes~~
~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Precautions : Active Orders:
» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered

Discontinued Orders:
» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Deleted
» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued
» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled
» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed
» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued
» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued
» Isolation, Precautions: COVID-19, Discontinued

~~{ [Active Orders:~~
~~» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered~~

~~Discontinued Orders:~~
~~» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Deleted~~
~~» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued~~
~~» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled~~
~~» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed~~
~~» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued~~
~~» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued~~
~~» Isolation, Precautions: COVID-19, Discontinued~~

~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy Forms

History of Swallow Studies : Clinical Swallowing Recommendations: Dysphagia treatment Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024

~~{ [Clinical Swallowing Recommendations: Dysphagia treatment Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Level of Consciousness Rehab : Alert

~~{ [Alert] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Affect/Behavior Rehab : Cooperative

~~{ [Cooperative] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Basic Command Following : Intact

~~{ [Intact] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Orientation Assessment Rehab : Identifies self

~~{ [Identifies self] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Pain Interfering With Session : No

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

~~{ [No] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Pre-Assessment

Oxygen Therapy : Nasal cannula

~~{ [Nasal cannula] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Tracheostomy Information : Oxygen Flow Rate: 2 L/min Performed by: Novak RN, Shayla 08/30/24 12:00:00
FIO2: 30 Performed by: Stansfield RN, Erica 08/29/24 16:00:00

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

~~{ [Oxygen Flow Rate: 2 L/min Performed by: Novak RN, Shayla 08/30/24 12:00:00
FIO2: 30 Performed by: Stansfield RN, Erica 08/29/24 16:00:00
] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Anatomic and Structural Assessment

Dentition : Edentulous

~~{ [Edentulous] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Labial : Within normal limits

~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Lingual : Within functional limits

~~{ [Within functional limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Mandible : Within normal limits

~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Velopharyngeal : Within functional limits

~~{ [Within functional limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Base of Tongue Retraction : Reduced

~~{ [Reduced] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Glottic Gap : Within normal limits

~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Aryepiglottice Rim : Lesion, Other: ecchymosis at left side

~~{ [Lesion, Other: ecchymosis at left side] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Arytenoids : Other: left arytenoid reducced adduction

~~{ [Other: left arytenoid reducced adduction] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Patient: GARCIA, ARMANDO Admit: 8/28/2024
MRN: HHN8210503; DSH5787309; CHH7351195 Disch:
DOB/Sex: 6/2/1972 / Male FIN: HHN0005007238115
Attending: Kabsoun MD,Sara H

Speech Therapy Forms

Aryepiglottic Folds Touch : Not tested
~~{ [Not tested] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
True Vocal Folds : Other: left fixed at paramedian position
~~{ [Other: left fixed at paramedian position] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
False Focal Folds : Other: left fixed at perimedian position
~~{ [Other: left fixed at perimedian position] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Airway Protection : Incomplete TVC Closure, Incomplete FVC Closure
~~{ [Incomplete TVC Closure, Incomplete FVC Closure] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vocal Fold Closure/Arytenoid Mobility : Reduced Adduction, Assymetry
~~{ [Reduced Adduction, Assymetry] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Secretions : Foamy, Clear
~~{ [Foamy, Clear] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pharyngeal Walls : Other: left ecchymosis ?lesion
~~{ [Other: left ecchymosis ?lesion] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Endoscopic Exam Comments : visualized left base of tongue, anterior epiglottic fold, and left aeryepiglottic fold ecchomysis ?lesion
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [visualized left base of tongue, anterior epiglottic fold, and left aeryepiglottic fold ecchomysis ?lesion] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Texture Trials

Patient Position Texture Trials : Upright at 90 degrees
~~{ [Upright at 90 degrees] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Factors Affecting Performance : No Difficulties Participating in Study
~~{ [No Difficulties Participating in Study] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Scope Passed : Right Nare
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Right Nare] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Ice Chips Texture Trials

	Ice Chips
Texture Trials :	Single ice chips Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Single ice chips] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy Forms

Oral Phase :	Within normal limits Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To pyriforms Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To pyriforms] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Onset of Swallow :	Normal (0-2 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Normal (0-2 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	III Mild: 5-25% epiglottic ligament visible Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [III Mild: 5-25% epiglottic~~

Speech Therapy Forms

	~~ligament visible] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	Lateral channels Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Lateral channels] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP,~~

Speech Therapy Forms

	~~Noreen ann at 8/30/2024 14:29 PDT};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Thin Liquid Texture Trials

	Thin Liquid
Texture Trials :	Tsp, Straw Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Tsp, Straw] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Oral Phase :	Within normal limits Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To vallecular Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To vallecular] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy Forms

Onset of Swallow :	Normal (0-2 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Normal (0-2 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	Lateral channels Castilo CCC-SLP,

Speech Therapy Forms

	Noreen ann - 8/30/2024 14:39 PDT ~~{ [Lateral channels] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Pureed Texture Trials

	Pureed
Texture Trials :	Via tsp Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Via tsp] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Oral Phase :	Within normal limits Castilo CCC-SLP, Noreen ann -

Speech Therapy Forms

	8/30/2024 14:39 PDT ~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To vallecular Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To vallecular] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Onset of Swallow :	Mild delay (3-5 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Mild delay (3-5 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	III Mild: 5-25% epiglottic ligament visible, IV Moderate: 25-50% epiglottic ligament covered Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [III Mild: 5-25%~~

Speech Therapy Forms

	~~epiglottic ligament visible, IV Moderate: 25-50% epiglottic ligament covered] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	Lateral channels Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Lateral channels] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1.~~

Speech Therapy Forms

	~~No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Compensatory Strategies Trialed :	Yes Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT (Comment: liquid wash moderately effective in clearing residue consecutive swallow moderately effective in clearing residue [Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT]) ~~{ [Yes] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT [Comment] liquid wash moderately effective in clearing residue consecutive swallow moderately effective in clearing residue - [Castilo CCC-SLP, Noreen ann at~~

Speech Therapy Forms

	~~8/30/2024 14:29 PDT])};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Mixed Consistency Texture Trials

	Mixed Consistency
Texture Trials :	Total trials Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT (Comment: regular graham cracker [Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT]) ~~{ [Total trials] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT [Comment] regular graham cracker - [Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT])};~~
Oral Phase :	Reduced mastication Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Reduced mastication] - previously charted by~~

Speech Therapy Forms

	~~Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To pyriforms Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To pyriforms] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Onset of Swallow :	Mild delay (3-5 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Mild delay (3-5 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	III Mild: 5-25% epiglottic ligament visible Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [III Mild: 5-25% epiglottic ligament visible] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy Forms

Pyriform Residue :	I None: 0% or no residue Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [I None: 0% or no residue] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	None Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [None] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Compensatory Strategies Trialed :	Yes Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT (Comment: liquid wash moderately effective in

Speech Therapy Forms

	clearing residue consecutive swallow moderately effective in clearing residue [Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT]) ~~{ [Yes] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT [Comment] liquid wash moderately effective in clearing residue consecutive swallow moderately effective in clearing residue - [Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT])};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Assessment

swallowing impression

Oral Dysphagia : Liquids, Solids

~~{ [Liquids, Solids] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Pharyngeal Dysphagia : Liquids, Solids

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

~~{ [Liquids, Solids] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Contributing Factors to Dysphagia : Reduced mastication, Delayed initiation of swallow, Reduced base of tongue retraction

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

Speech Therapy Forms

~~{ [Reduced mastication, Delayed initiation of swallow, Reduced base of tongue retraction] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Recommendations

Diet Consistency Recommendation : IDDSI 6 Soft and Bite Sized
~~{ [IDDSI 6 Soft and Bite Sized] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Liquid Viscosity Recommendation : Thin
~~{ [Thin] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Medications Swallow Recommendation : Whole in food
~~{ [Whole in food] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Supervision Swallow Recommendation : Intermittent
~~{ [Intermittent] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Positioning Swallow Recommendation : Upright 90 degrees
~~{ [Upright 90 degrees] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Endo Swallow Recommendations : Dysphagia treatment
~~{ [Dysphagia treatment] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Assessment Recommendations ST Daily : Subjective/Patient Report:
Chart reviewed, consulted with Shayla RN- cleared pt for evaluation. Swallow evaluation indicated d/t recent extubation. Pt Identified via name and DOB. Pt is agreeable to evaluation. Family (wife) present at bedside for education. Pt is a 52 year old male to Henderson Hospital for SOB at home.
Dx: intubated 08/28-08/29; COPD exacerbation, SOB, sepsis, O2 dependent, resp distress, resp failure, elevate BNP, hypomagnesemia, hyponatremia, AKI, hypophosphatemia
Hyperglycemia d/t DM, leukocytosis, fever, HTN emergency, morbid obesity, CHF exacerbation, bil lower extremity edema, s/p left endarterectomy, elevated lactic acid, elevated CRP, PUI COVID; bronchoscopy 8/29 - mod to sev bil inflammation, mod bil white thick secretions, high position ET tube.
Significant hx includes but not limited to DM, HTN, diastolic CHF, COPD, hyperlipidemia, disc disorder, ESBL ecoli, gout, HLD, HTN, bil ICA stenosis s/p endarterectomy, EGD w/ biopsy 06/2022
Home Diet: Regular consistencies
Current Diet: NPO

Dr. Kabsuon ordered FEES for swallow evaluation.
Annie SLP assisted with feeding during FEES

Informed consent: Procedure was explained including indications for FEES, risks, benefits, and alternatives to FEES including no evaluation as all. Consent obtained- patient signed

Assessment
Findings: Pt presents with mild oropharyngeal dysphagia r/t visualized left base of tongue ?lesion, characterized by prolonged bolus preparation, reduced base of tongue retraction; resulting to trace to moderate pharyngeal residue at vallecular region/pyriform sinus region; no penetration/aspiration noted t/o this trial. Pt observed with overcompensation of surrounding pharyngeal muscles during swallow and vocalization d/t left true vocal fold fixed at paramedian position. Pt is at increased risk due to reduced pharyngeal residue awareness and ?lesion at left base of tongue at this time.

Therapeutic Exercises, precautions/safe swallow strategies, and diet modification indicated to increase swallow safety

Recommendations:
Food Consistencies: IDDSI 6 Soft and Bite Sized

Speech Therapy Forms

Liquid Consistencies: Thin Liquids
Meds: Whole with puree
Supervision/Assist: Distant supervision needed for feeding (provided initial cueing to initiate swallow strategies check on the patient frequently and assess the patient progress at least two to three times during the meal)

Aspiration precautions:
Sit upright as close to 90 degrees as possible,
remain upright 30 mins after meals
Small bites/sips
Swallow two times
Alternate solids/liquids

ST to treat for dysphagia
Consider further imaging of neck tissue
Consider f/u ENT outpatient when feasible
Pt would benefit to change of tx frequency to 2-4x/wk
Results and recommendations discussed with: Shayla RN

~~{ [Subjective/Patient Report:~~
~~Chart reviewed, consulted with Shayla RN- cleared pt for evaluation. Swallow evaluation indicated d/t recent extubation. Pt Identified via name and DOB. Pt is agreeable to evaluation. Family (wife) present at bedside for education. Pt is a 52 year old male to Henderson Hospital for SOB at home.~~
~~Dx: intubated 08/28-08/29; COPD exacerbation, SOB, sepsis, O2 dependent, resp distress, resp failure, elevate BNP, hypomagnesemia, hyponatremia, AKI, hypophosphatemia~~
~~Hyperglycemia d/t DM, leukocytosis, fever, HTN emergency, morbid obesity, CHF exacerbation, bil lower extremity edema, s/p left endarterectomy, elevated lactic acid, elevated CRP, PUI COVID; bronchoscopy 8/29 - mod to sev bil inflammation, mod bil white thick secretions, high position ET tube.~~
~~Significant hx includes but not limited to DM, HTN, diastolic CHF, COPD, hyperlipidemia, disc disorder, ESBL ecoli, gout, HLD, HTN, bil ICA stenosis s/p endarterectomy, EGD w/ biopsy 06/2022~~
~~Home Diet: Regular consistencies~~
~~Current Diet: NPO~~

~~Dr. Kabsuon ordered FEES for swallow evaluation.~~
~~Annie SLP assisted with feeding during FEES~~

~~Informed consent: Procedure was explained including indications for FEES, risks, benefits, and alternatives to FEES including no evaluation as all. Consent obtained- patient signed~~

~~Assessment~~
~~Findings: Pt presents with mild oropharyngeal dysphagia r/t visualized left base of tongue ?lesion, characterized by prolonged bolus preparation, reduced base of tongue retraction; resulting to trace to moderate pharyngeal residue at vallecular region/pyriform sinus region; no penetration/aspiration noted t/o this trial. Pt observed with overcompensation of surrounding pharyngeal muscles during swallow and vocalization d/t left true vocal fold fixed at paramedian position. Pt is at increased risk due to reduced pharyngeal residue awareness and ?lesion at left base of tongue at this time.~~

~~Therapeutic Exercises, precautions/safe swallow strategies, and diet modification indicated to increase swallow safety~~

Speech Therapy Forms

~~Recommendations:~~
~~Food Consistencies: IDDSI 6 Soft and Bite Sized~~
~~Liquid Consistencies: Thin Liquids~~
~~Meds: Whole with puree~~
~~Supervision/Assist: Distant supervision needed for feeding (provided initial cueing to initiate swallow strategies check on the patient frequently and assess the patient progress at least two to three times during the meal)~~

~~Aspiration precautions:~~
~~Sit upright as close to 90 degrees as possible,~~
~~remain upright 30 mins after meals~~
~~Small bites/sips~~
~~Swallow two times~~
~~Alternate solids/liquids~~

~~ST to treat for dysphagia~~
~~Consider further imaging of neck tissue~~
~~Consider f/u ENT outpatient when feasible~~
~~Pt would benefit to change of tx frequency to 2-4x/wk~~
~~Results and recommendations discussed with: Shayla RN~~

~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Continued intermittent therapy to address functional deficits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Education

Procedure Explanation : Indications for FEES, Risks, Benefits, Alternatives to FEES including no evaluation
~~{ [Indications for FEES, Risks, Benefits, Alternatives to FEES including no evaluation] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Consent Given : Patient Gave Consent
~~{ [Patient Gave Consent] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Teaching Method : Explanation, Video/Educational TV
~~{ [Explanation, Video/Educational TV] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Learner(s) Present : Self, Spouse, Family member
~~{ [Self, Spouse, Family member] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Barriers to Learning : None evident
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [None evident] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Document learning evaluation for Responsible Learner(s)
Speech Language/Pathology Tx Plan : Verbalizes understanding
~~{ [Verbalizes understanding] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Swallowing Strategies : Verbalizes understanding, Needs further teaching
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

Speech Therapy Forms

~~{ [Verbalizes understanding, Needs further teaching] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Plan

Planned Treatments : Dysphagia therapy
~~{ [Dysphagia therapy] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Frequency : 2-4 times per week
~~{ [2-4 times per week] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Duration : 1-2 weeks
~~{ [1-2 weeks] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
SLP Intensity Per Day : Other: as tolerated
~~{ [Other: as tolerated] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
~~{ [Continued intermittent therapy to address functional deficits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Rehabilitation Potential : Good
~~{ [Good] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Plan/Goals Established With Patient/Caregiver : Yes
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Yes] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Additional Information

Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Bed in low position, Call device within reach, Personal items within reach] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Long Term Goals

SLP Patient, Caregiver Goals : "to eat food"
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ ["to eat food"] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will tolerate least restrictive consistencies with risk of aspiration reduced as much as reasonably possible. Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will tolerate least restrictive consistencies~~

Speech Therapy Forms

	~~with risk of aspiration reduced as much as reasonably possible.] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Status :	Progressing, continue Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Progressing, continue] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Timeframe To Complete :	9/13/24 Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [9/13/24] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Short Term Goals

Other SLP Short Term Goal Grid

	Goal #1	Goal #2	Goal #3
Other :	Pt will tolerate 90ml challenge	Pt will perform compensatory	Pt will tolerate regular PO trials

Speech Therapy Forms

	vs instrumental swallow without uncompensated aspiration or penetration on thin liquids Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will tolerate 90ml challenge vs instrumental swallow without uncompensated aspiration or penetration on thin liquids] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	strategies to protect the airway and improve clearance of the bolus in 80% of trials independently Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	with no overt s/s of apsiration in 5 out o5 trials Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will tolerate regular PO trials with no overt s/s of apsiration in 5 out o5 trials] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Status :	Goal met Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Goal met] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	Progressing, continue Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Progressing, continue] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	Initial Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Initial] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Date Met :	8/30/2024 PDT Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [8/30/2024 PDT] - previously~~		

Speech Therapy Forms

	~~charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~		
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

ST Non Timed Modalities 2024

ST Treatment Start Time : 8/30/2024 12:58 PDT
~~{ [8/30/2024 12:58 PDT] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
ST Treatment End Time : 8/30/2024 13:58 PDT
~~{ [8/30/2024 13:58 PDT] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
FEES BY CINE/VIDEO RECORDING Min : 60
~~{ [60] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
ST Total Non Timed : 60
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [60] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

DOCUMENT TYPE:	Clinical Swallowing - Text
SERVICE DATE/TIME:	8/30/2024 10:49 PDT
RESULT STATUS:	Auth (Verified)
PERFORMED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 11:28 PDT)
SIGNED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 11:28 PDT)

ST Bedside Swallow Eval Entered On: 8/30/2024 11:32 PDT
Performed On: 8/30/2024 10:49 PDT by Castilo CCC-SLP, Noreen ann

General Information

Speech Orders : ST Bedside Swallow Exam - 08/29/24 19:32:00 PDT, Bedside Swallow Exam, Routine, Stop date 08/29/24 19:32:00 PDT, Stretcher, IV Yes, O2 Yes, 17433508165.0
Speech Therapy Eval and Treat - 08/29/24 19:32:00 PDT, Bedside Swallow Exam, Routine, Stop date 08/29/24 19:32:00 PDT, Stretcher, IV Yes, O2 Yes
Precautions RTF : Active Orders:
» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered

Discontinued Orders:
» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Deleted
» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb

Speech Therapy

Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued
» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled
» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed
» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued
» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued
» Isolation, Precautions: COVID-19, Discontinued

History of Swallow Studies : No qualifying data available
Level of Consciousness Rehab : Alert
Affect/Behavior Rehab : Cooperative
Basic Command Following : Intact
Orientation Assessment Rehab : Identifies self
Pain Interfering With SLP Treatment : No
Culture/Spiritual Beliefs to Incorporate : No

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Pre-Assessment
Oxygen Therapy : Nasal cannula
Tracheostomy Information : Oxygen Flow Rate: 5 L/min Performed by: Mower, Vanessa 08/30/24 07:51:00 FIO2: 30 Performed by: Stansfield RN, Erica 08/29/24 16:00:00

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Oral-Facial Exam
Facial Movement : Symmetric resting/crying
Dentition : Edentulous

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Oral Structure Grid
Labial Structure : Within functional limits
Mandible : Within functional limits
Lingual Structure : Within functional limits
Hard Palate : Within functional limits
Soft Palate : Within functional limits

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Oral Function Grid
Labial : Within functional limits
Mandible : Within functional limits
Lingual : Within functional limits
Velopharyngeal : Within functional limits

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Secretion Management : Reduced

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Upper Body Control Grid
Head Control : Upright centered position
Trunk Control : Upright centered position

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Speech Therapy

Swallowing Exam

Patient Position : Upright 90 degrees

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Liquids Swallowing Exam Adult Grid

	Ice Chips	Thin Liquid by Teaspoon	Thin Liquid by Straw
Bolus Size :		5 ml	
Total Intake :	Other: 5x	Other: 5x	3-4 ounces
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist	Speech-Language Pathologist
Oral Symptoms :	None observed	None observed	None observed
Pharyngeal Symptoms :	None observed	None observed	Cough/Throat clear after swallow
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Consistency Swallowing Exam Adult Grid

	Puree Consistency	Regular Consistency
Bolus Size :	5 ml	
Total Intake :	1-2 ounces	Other: one quarter graham cracker
Presentation :	Speech-Language Pathologist	Speech-Language Pathologist
Oral Symptoms :	None observed	Prolonged mastication
Pharyngeal Symptoms :	Wet vocal quality after swallow	Wet vocal quality after swallow
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Assessment/Recommendations

Clinical Swallowing Impression Grid

Oral Dysphagia : Liquids, Solids

Pharyngeal Dysphagia : Liquids, Solids

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

NPO Status : Except for ice chips

Clinical Swallowing Recommendations : Dysphagia treatment

Speech Therapy

Assessment Recommendations ST Daily : Chart reviewed, consulted with Shayla RN- cleared pt for evaluation. Swallow evaluation indicated d/t recent extubation. Pt Identified via name and DOB. Pt is agreeable to evaluation. Pt is a 52 year old male to Henderson Hospital for SOB at home.
Dx: intubated 08/28-08/29; COPD exacerbation, SOB, sepsis, O2 dependent, resp distress, resp failure, elevate BNP, hypomagnesemia, hyponatremia, AKI, hypophosphatemia
Hyperglycemia d/t DM, leukocytosis, fever, HTN emergency, morbid obesity, CHF exacerbation, bil lower extremity edema, s/p left endarterectomy, elevated lactic acid, elevated CRP, PUI COVID; bronchoscopy 8/29 - mod to sev bil inflammation, mod bil white thick secretions, high position ET tube.
Significant hx includes but not limited to DM, HTN, diastolic CHF, COPD, hyperlipidemia, disc disorder, ESBL ecoli, gout, HLD, HTN, bil ICA stenosis s/p endarterectomy, EGD w/ biopsy 06/2022
Home Diet: Regular consistencies
Current Diet: NPO
WBC: WNL
CXR: 8/30 2. Low lung volumes with bronchovascular crowding. Bibasilar airspace disease and/or atelectasis.
CT Head: 8/28 no acute intracranial abnormality

IMPRESSION:
Severe oropharyngeal dysphagia characterized by s/s suspected aspiration, observed continued wet vocal quality t/o this evaluation, pt reported r/t neck surgery a month ago and was told there will be swelling. However noted reduced O2 sat during swallow completion with PO trials. See Swallowing Exam Section for details.

RECOMMENDATIONS:
NPO, except ice chips with aspiration precautions and oral care TID to reduce risk of aspiration PNA
Alternative means nutrition, hydration, and medication-Consider NGT.
FEES to fully evaluate swallow function

ST to treat for dysphagia
Results and recommendations discussed with: Shayla RN
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Education
Learner(s) Present for Session : Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Document learning evaluation for Responsible Learner(s)
Speech Language/Pathology Tx Plan : Verbalizes understanding
Swallowing Strategies : Verbalizes understanding

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Long Term Goals
SLP Patient, Caregiver Goals : "to eat food"

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1

Speech Therapy

Goal :	Pt will tolerate least restrictive consistencies with risk of aspiration reduced as much as reasonably possible.
Status :	Initial
Timeframe To Complete :	9/13/24
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Short Term Goals

Other SLP Short Term Goal Grid

	Goal #1	Goal #2
Other :	Pt will tolerate 90ml challenge vs instrumental swallow without uncompensated aspiration or penetration on thin liquids	Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently
Status :	Initial	Initial
	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Plan

Planned Treatments : Dysphagia therapy
Frequency : 3-5 times per week
Duration : 1-2 weeks
SLP Intensity Per Day : Other: as tolerated
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
Rehabilitation Potential : Good
Plan/Goals Established With Patient/Caregiver : Yes

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

Additional Information

Speech Therapy

Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

ST Non Timed Modalities 2024
ST Treatment Start Time : 8/30/2024 10:49 PDT
ST Treatment End Time : 8/30/2024 10:59 PDT
ST Eval Oral/Swallow Func Minutes : 10
ST Total Non Timed : 10

Castilo CCC-SLP, Noreen ann - 8/30/2024 11:28 PDT

DOCUMENT TYPE: Inpatient SLP Daily Documentation - Text
SERVICE DATE/TIME: 8/31/2024 14:02 PDT
RESULT STATUS: Auth (Verified)
PERFORMED INFORMATION: Castilo CCC-SLP,Noreen ann (8/31/2024 15:27 PDT)
SIGNED INFORMATION: Castilo CCC-SLP,Noreen ann (8/31/2024 15:27 PDT)

ST Daily Documentation Entered On: 8/31/2024 15:28 PDT
Performed On: 8/31/2024 14:02 PDT by Castilo CCC-SLP, Noreen ann

Review/Treatments Provided
Speech Orders : Miscellaneous Speech Therapy Task - 08/30/24 11:32:00 PDT, Routine, Stop date 08/30/24 11:32:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes
Speech Language Plan : Treatment Frequency: 2-4 times per week (modified) Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
Treatment Duration: 1-2 weeks (modified) Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
Planned Treatments: Dysphagia therapy (modified) Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
Short Term Goals Reviewed : Yes
Long Term Goals Reviewed : Yes
Speech Language Goals RTF : SLP Short Term Goals

08/30/2024
Other Short Term Goals Goal #2: Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently (modified); Progressing, continue (modified)
Other Short Term Goals Goal #3: Pt will tolerate regular PO trials with no overt s/s of apsiration in 5 out o5 trials (modified); Initial (modified)
Pain Interfering With SLP Treatment : No

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Short Term Goals
Other SLP Short Term Goal Grid

	Goal #1	Goal #2	Goal #3
Other :	Pt will tolerate 90ml challenge vs instrumental swallow without	Pt will perform compensatory strategies to protect the	Pt will tolerate regular PO trials with no overt s/s of apsiration

Speech Therapy

	uncompensated aspiration or penetration on thin liquids	airway and improve clearance of the bolus in 80% of trials independently	in 5 out o5 trials
Status :	Goal met	Progressing, continue	Progressing, continue
Date Met :	8/30/2024 PDT		
	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Long Term Goals
SLP Patient, Caregiver Goals : "to eat food"

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will tolerate least restrictive consistencies with risk of aspiration reduced as much as reasonably possible.
Status :	Progressing, continue
Timeframe To Complete :	9/13/24
	Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Education
Learner(s) Present for Session : Explanation
Learner(s) Present : Self
Barriers to Learning : None evident

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Speech Education Grid
Speech Language/Pathology Tx Plan : Verbalizes understanding
Swallowing Strategies : Verbalizes understanding, Needs further teaching

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Assessment/Recommendations

Speech Therapy

Assessment Recommendations ST Daily : S: Chart reviewed, consulted with Alejandra RN- cleared pt for tx. Pt alert, agreeable treatment. Clinician discussed with Alejandra RN re: FEES findings with images in document.

O: Ordered textures: IDDSI Soft and bite size with thin liquids; WBC: WNL; CXR: 8/30 1. Interval removal of the endotracheal tube and enteric tube. 2. Low lung volumes with bronchovascular crowding. Bibasilar airspace disease and/or atelectasis.
PO trials:
Thin liquids via straw 1-3oz: no overt s/s of aspiration
Regular graham cracker one half piece: prolonged mastication, no overt s/s of aspiration, independent use of liquid wash
Compensatory/safe swallow strategies: upright in bed, small bites/sips, slow rate for PO intake, swallow two times
Education: s/s of aspiration, swallowing strategies, POC. Pt verbalized understanding and had no further questions at this time. See Also Education Grid

A: pt continues to exhibit oropharyngeal dysphagia r/t FEES on 08/30/24 noted continued wet gurgly voice prior to and after PO intake not r/t aspiration concerns at this time

P: Recommendations
Food Consistencies: IDDSI 6 Soft and Bite Sized
Liquid Consistencies: Thin Liquids
Meds: Whole with puree
Supervision/Assist: Distant supervision needed for feeding (provided initial cueing to initiate swallow strategies check on the patient frequently and assess the patient progress at least two to three times during the meal)

Aspiration precautions:
Sit upright as close to 90 degrees as possible,
remain upright 30 mins after meals
Small bites/sips
Swallow two times
Alternate solids/liquids

ST to f/u per POC
Consider further imaging of neck tissue
Consider f/u ENT outpatient when feasible
Discussed with: Alejandra RN

Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

Additional Information
Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

ST Non Timed Modalities 2024
ST Treatment Start Time : 8/31/2024 14:02 PDT
ST Treatment End Time : 8/31/2024 14:11 PDT

Patient: GARCIA, ARMANDO Admit: 8/28/2024
MRN: HHN8210503; DSH5787309; CHH7351195 Disch:
DOB/Sex: 6/2/1972 / Male FIN: HHN0005007238115
Attending: Kabsoun MD,Sara H

Speech Therapy

ST TREATMENT FOR FEED Minutes : 9
ST Total Non Timed : 9

Castilo CCC-SLP, Noreen ann - 8/31/2024 15:27 PDT

DOCUMENT TYPE:	FEES - Text
SERVICE DATE/TIME:	8/30/2024 12:58 PDT
RESULT STATUS:	Modified
PERFORMED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 14:29 PDT)
SIGNED INFORMATION:	Castilo CCC-SLP,Noreen ann (8/30/2024 14:39 PDT); Castilo CCC-SLP,Noreen ann (8/30/2024 14:29 PDT)

ST FEES Entered On: 8/30/2024 14:39 PDT
Performed On: 8/30/2024 12:58 PDT by Castilo CCC-SLP, Noreen ann

General Information

Speech Orders : FEES - 08/30/24 11:33:00 PDT, Routine, Stop date 08/30/24 11:33:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes
Miscellaneous Speech Therapy Task - 08/30/24 11:32:00 PDT, Routine, Stop date 08/30/24 11:32:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes
~~{ [FEES - 08/30/24 11:33:00 PDT, Routine, Stop date 08/30/24 11:33:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes~~
~~Miscellaneous Speech Therapy Task - 08/30/24 11:32:00 PDT, Routine, Stop date 08/30/24 11:32:00 PDT, Stretcher, Patient Has IV: Yes, Patient on O2: Yes~~
~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Precautions : Active Orders:
» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered

Discontinued Orders:
» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Deleted
» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued
» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled
» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed
» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued
» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued
» Isolation, Precautions: COVID-19, Discontinued

Speech Therapy

~~{ [Active Orders:~~
~~» Oxygen Therapy, 08/27/24 20:53:00 PDT, Routine, 2 L/Min, Nasal Cannula, Titrate to keep SpO2 greater than 90%, Ordered~~

~~Discontinued Orders:~~
~~» Restraint Nonviolent Discontinue, 08/29/24 19:32:41 PDT, Not interfering with device/treatment, Stop Date/Time: 08/29/24 19:32:41 PDT, Once, Routine, Deleted~~
~~» Restraint Nonviolent, 08/29/24 8:48:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2351, Minutes, Routine, Stop Date: 08/30/24 2..., Discontinued~~
~~» Restraint NonViolent Monitoring, 08/28/24 10:05:31 PDT, q4H, Routine, Canceled~~
~~» Restraint Nonviolent Initiate, 08/28/24 10:05:31 PDT, Not interfering with device/treatment, Stop Date/Time: 08/28/24 10:05:31 PDT, Once, Routine, Completed~~
~~» Restraint Nonviolent, 08/28/24 10:05:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2274, Minutes, Routine, Stop Date: 08/29/24..., Discontinued~~
~~» Restraint Nonviolent, 08/28/24 2:50:00 PDT, Reason: Interfering - medical treatment/healing, Soft Limb Left Upper Extremity | Soft Limb Right Upper Extremity, Not interfering with device/treatment, Constant Indicator, Duration: 2709, Minutes, Routine, Stop Date: 08/29/24 2..., Discontinued~~
~~» Isolation, Precautions: COVID-19, Discontinued~~

~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
History of Swallow Studies : Clinical Swallowing Recommendations: Dysphagia treatment Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024
~~{ [Clinical Swallowing Recommendations: Dysphagia treatment Performed By: Castilo CCC-SLP, Noreen ann 08/30/2024~~
~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Level of Consciousness Rehab : Alert
~~{ [Alert] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Affect/Behavior Rehab : Cooperative
~~{ [Cooperative] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Basic Command Following : Intact
~~{ [Intact] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Orientation Assessment Rehab : Identifies self
~~{ [Identifies self] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pain Interfering With Session : No
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [No] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pre-Assessment
Oxygen Therapy : Nasal cannula
~~{ [Nasal cannula] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Tracheostomy Information : Oxygen Flow Rate: 2 L/min Performed by: Novak RN, Shayla 08/30/24 12:00:00
FIO2: 30 Performed by: Stansfield RN, Erica 08/29/24 16:00:00
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Oxygen Flow Rate: 2 L/min Performed by: Novak RN, Shayla 08/30/24 12:00:00~~
~~FIO2: 30 Performed by: Stansfield RN, Erica 08/29/24 16:00:00~~
~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Anatomic and Structural Assessment

Speech Therapy

Dentition : Edentulous
~~{ [Edentulous] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Labial : Within normal limits
~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Lingual : Within functional limits
~~{ [Within functional limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Mandible : Within normal limits
~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Velopharyngeal : Within functional limits
~~{ [Within functional limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Base of Tongue Retraction : Reduced
~~{ [Reduced] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Glottic Gap : Within normal limits
~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Aryepiglottice Rim : Lesion, Other: ecchymosis at left side
~~{ [Lesion, Other: ecchymosis at left side] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Arytenoids : Other: left arytenoid reducced adduction
~~{ [Other: left arytenoid reducced adduction] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Aryepiglottic Folds Touch : Not tested
~~{ [Not tested] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
True Vocal Folds : Other: left fixed at paramedian position
~~{ [Other: left fixed at paramedian position] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
False Focal Folds : Other: left fixed at perimedian position
~~{ [Other: left fixed at perimedian position] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Airway Protection : Incomplete TVC Closure, Incomplete FVC Closure
~~{ [Incomplete TVC Closure, Incomplete FVC Closure] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vocal Fold Closure/Arytenoid Mobility : Reduced Adduction, Assymetry
~~{ [Reduced Adduction, Assymetry] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Secretions : Foamy, Clear
~~{ [Foamy, Clear] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pharyngeal Walls : Other: left ecchymosis ?lesion
~~{ [Other: left ecchymosis ?lesion] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Endoscopic Exam Comments : visualized left base of tongue, anterior epiglottic fold, and left aeryepiglottic fold ecchomysis ?lesion
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [visualized left base of tongue, anterior epiglottic fold, and left aeryepiglottic fold ecchomysis ?lesion] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Texture Trials
Patient Position Texture Trials : Upright at 90 degrees
~~{ [Upright at 90 degrees] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Patient: GARCIA, ARMANDO Admit: 8/28/2024
MRN: HHN8210503; DSH5787309; CHH7351195 Disch:
DOB/Sex: 6/2/1972 / Male FIN: HHN0005007238115
Attending: Kabsoun MD,Sara H

Speech Therapy

Factors Affecting Performance : No Difficulties Participating in Study
~~{ [No Difficulties Participating in Study] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Scope Passed : Right Nare
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Right Nare] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Ice Chips Texture Trials

	Ice Chips
Texture Trials :	Single ice chips Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Single ice chips] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Oral Phase :	Within normal limits Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To pyriforms Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To pyriforms] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

Onset of Swallow :	Normal (0-2 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Normal (0-2 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	III Mild: 5-25% epiglottic ligament visible Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [III Mild: 5-25% epiglottic ligament visible] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	Lateral channels Castilo

Speech Therapy

	CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Lateral channels] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Thin Liquid Texture Trials

	Thin Liquid
Texture Trials :	Tsp, Straw Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Tsp, Straw] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Oral Phase :	Within normal limits Castilo CCC-SLP,

Speech Therapy

	Noreen ann - 8/30/2024 14:39 PDT ~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To vallecular Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To vallecular] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Onset of Swallow :	Normal (0-2 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Normal (0-2 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by~~

Speech Therapy

	~~Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	Lateral channels Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Lateral channels] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Pureed Texture Trials

	Pureed
Texture Trials :	Via tsp Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Via tsp] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Oral Phase :	Within normal limits Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Within normal limits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To vallecular Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To vallecular] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Onset of Swallow :	Mild delay (3-5 seconds) Castilo CCC-SLP,

Speech Therapy

	Noreen ann - 8/30/2024 14:39 PDT ~~{ [Mild delay (3-5 seconds)] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	III Mild: 5-25% epiglottic ligament visible, IV Moderate: 25-50% epiglottic ligament covered Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [III Mild: 5-25% epiglottic ligament visible, IV Moderate: 25-50% epiglottic ligament covered] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	II Trace: 1.5% trace coating of the mucosa Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [II Trace: 1.5% trace coating of the mucosa] - previously charted by~~

Speech Therapy

	~~Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	Lateral channels Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Lateral channels] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Compensatory Strategies Trialed :	Yes Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT (Comment: liquid wash moderately effective in clearing residue consecutive swallow moderately effective in clearing residue [Castilo

Speech Therapy

	CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT]) ~~{ [Yes] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT [Comment] liquid wash moderately effective in clearing residue consecutive swallow moderately effective in clearing residue - [Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT])};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Mixed Consistency Texture Trials

	Mixed Consistency
Texture Trials :	Total trials Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT (Comment: regular graham cracker [Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT]) ~~{ [Total trials] -~~

Speech Therapy

	~~previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT [Comment] regular graham cracker - [Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT])};~~
Oral Phase :	Reduced mastication Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Reduced mastication] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Premature Spillage :	To pyriforms Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [To pyriforms] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Onset of Swallow :	Mild delay (3-5 seconds) Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Mild delay (3-5 seconds)] - previously~~

Speech Therapy

	~~charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Vallecular Residue :	III Mild: 5-25% epiglottic ligament visible Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [III Mild: 5-25% epiglottic ligament visible] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Pyriform Residue :	I None: 0% or no residue Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [I None: 0% or no residue] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Additional Residue :	None Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [None] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

Penetration-Aspiration Scale :	1. No aspiration or penetration Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [1. No aspiration or penetration] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Compensatory Strategies Trialed :	Yes Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT (Comment: liquid wash moderately effective in clearing residue consecutive swallow moderately effective in clearing residue [Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT]) ~~{ [Yes] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT [Comment] liquid wash moderately effective in clearing residue consecutive swallow moderately~~

Speech Therapy

	~~effective in clearing residue - [Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT])};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Assessment

swallowing impression

Oral Dysphagia : Liquids, Solids

~~{ [Liquids, Solids] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Pharyngeal Dysphagia : Liquids, Solids

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

~~{ [Liquids, Solids] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Contributing Factors to Dysphagia : Reduced mastication, Delayed initiation of swallow, Reduced base of tongue retraction

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

~~{ [Reduced mastication, Delayed initiation of swallow, Reduced base of tongue retraction] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Recommendations

Diet Consistency Recommendation : IDDSI 6 Soft and Bite Sized

~~{ [IDDSI 6 Soft and Bite Sized] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Liquid Viscosity Recommendation : Thin

~~{ [Thin] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Medications Swallow Recommendation : Whole in food

~~{ [Whole in food] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Supervision Swallow Recommendation : Intermittent

~~{ [Intermittent] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Positioning Swallow Recommendation : Upright 90 degrees

~~{ [Upright 90 degrees] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Endo Swallow Recommendations : Dysphagia treatment

~~{ [Dysphagia treatment] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Assessment Recommendations ST Daily : Subjective/Patient Report:
Chart reviewed, consulted with Shayla RN- cleared pt for evaluation. Swallow evaluation indicated d/t recent extubation. Pt Identified via name and DOB. Pt is agreeable to evaluation. Family (wife) present at bedside for education. Pt is a 52 year old male to Henderson Hospital for SOB at home.
Dx: intubated 08/28-08/29; COPD exacerbation, SOB, sepsis, O2 dependent, resp distress, resp failure, elevate BNP, hypomagnesemia, hyponatremia, AKI, hypophosphatemia
Hyperglycemia d/t DM, leukocytosis, fever, HTN emergency, morbid obesity, CHF exacerbation, bil lower extremity edema, s/p left endarterectomy, elevated lactic acid, elevated CRP, PUI COVID; bronchoscopy 8/29 - mod to sev bil inflammation, mod bil white thick secretions, high position ET tube.
Significant hx includes but not limited to DM, HTN, diastolic CHF, COPD, hyperlipidemia, disc disorder, ESBL ecoli, gout, HLD, HTN, bil ICA stenosis s/p endarterectomy, EGD w/ biopsy 06/2022

Speech Therapy

Home Diet: Regular consistencies
Current Diet: NPO

Dr. Kabsuon ordered FEES for swallow evaluation.
Annie SLP assisted with feeding during FEES

Informed consent: Procedure was explained including indications for FEES, risks, benefits, and alternatives to FEES including no evaluation as all. Consent obtained- patient signed

Assessment
Findings: Pt presents with mild oropharyngeal dysphagia r/t visualized left base of tongue ?lesion, characterized by prolonged bolus preparation, reduced base of tongue retraction; resulting to trace to moderate pharyngeal residue at vallecular region/pyriform sinus region; no penetration/aspiration noted t/o this trial. Pt observed with overcompensation of surrounding pharyngeal muscles during swallow and vocalization d/t left true vocal fold fixed at paramedian position. Pt is at increased risk due to reduced pharyngeal residue awareness and ?lesion at left base of tongue at this time.

Therapeutic Exercises, precautions/safe swallow strategies, and diet modification indicated to increase swallow safety

Recommendations:
Food Consistencies: IDDSI 6 Soft and Bite Sized
Liquid Consistencies: Thin Liquids
Meds: Whole with puree
Supervision/Assist: Distant supervision needed for feeding (provided initial cueing to initiate swallow strategies check on the patient frequently and assess the patient progress at least two to three times during the meal)

Aspiration precautions:
Sit upright as close to 90 degrees as possible,
remain upright 30 mins after meals
Small bites/sips
Swallow two times
Alternate solids/liquids

ST to treat for dysphagia
Consider further imaging of neck tissue
Consider f/u ENT outpatient when feasible
Pt would benefit to change of tx frequency to 2-4x/wk
Results and recommendations discussed with: Shayla RN

~~{ [Subjective/Patient Report:~~
~~Chart reviewed, consulted with Shayla RN- cleared pt for evaluation. Swallow evaluation indicated d/t recent extubation. Pt Identified via name and DOB. Pt is agreeable to evaluation. Family (wife) present at bedside for education. Pt is a 52 year old male to Henderson Hospital for SOB at home.~~
~~Dx: intubated 08/28-08/29; COPD exacerbation, SOB, sepsis, O2 dependent, resp distress, resp failure, elevate BNP, hypomagnesemia, hyponatremia, AKI, hypophosphatemia~~
~~Hyperglycemia d/t DM, leukocytosis, fever, HTN emergency, morbid obesity, CHF exacerbation, bil lower extremity edema, s/p left endarterectomy, elevated lactic acid, elevated CRP, PUI COVID; bronchoscopy~~

Speech Therapy

~~8/29 - mod to sev bil inflammation, mod bil white thick secretions, high position ET tube.~~
~~Significant hx includes but not limited to DM, HTN, diastolic CHF, COPD, hyperlipidemia, disc disorder, ESBL ecoli, gout, HLD, HTN, bil ICA stenosis s/p endarterectomy, EGD w/ biopsy 06/2022~~
~~Home Diet: Regular consistencies~~
~~Current Diet: NPO~~

~~Dr. Kabsuon ordered FEES for swallow evaluation.~~
~~Annie SLP assisted with feeding during FEES~~

~~Informed consent: Procedure was explained including indications for FEES, risks, benefits, and alternatives to FEES including no evaluation as all. Consent obtained- patient signed~~

~~Assessment~~
~~Findings: Pt presents with mild oropharyngeal dysphagia r/t visualized left base of tongue ?lesion, characterized by prolonged bolus preparation, reduced base of tongue retraction; resulting to trace to moderate pharyngeal residue at vallecular region/pyriform sinus region; no penetration/aspiration noted t/o this trial. Pt observed with overcompensation of surrounding pharyngeal muscles during swallow and vocalization d/t left true vocal fold fixed at paramedian position. Pt is at increased risk due to reduced pharyngeal residue awareness and ?lesion at left base of tongue at this time.~~

~~Therapeutic Exercises, precautions/safe swallow strategies, and diet modification indicated to increase swallow safety~~

~~Recommendations:~~
~~Food Consistencies: IDDSI 6 Soft and Bite Sized~~
~~Liquid Consistencies: Thin Liquids~~
~~Meds: Whole with puree~~
~~Supervision/Assist: Distant supervision needed for feeding (provided initial cueing to initiate swallow strategies check on the patient frequently and assess the patient progress at least two to three times during the meal)~~

~~Aspiration precautions:~~
~~Sit upright as close to 90 degrees as possible,~~
~~remain upright 30 mins after meals~~
~~Small bites/sips~~
~~Swallow two times~~
~~Alternate solids/liquids~~

~~ST to treat for dysphagia~~
~~Consider further imaging of neck tissue~~
~~Consider f/u ENT outpatient when feasible~~
~~Pt would benefit to change of tx frequency to 2-4x/wk~~
~~Results and recommendations discussed with: Shayla RN~~

~~] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT

Speech Therapy

~~{ [Continued intermittent therapy to address functional deficits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Education

Procedure Explanation : Indications for FEES, Risks, Benefits, Alternatives to FEES including no evaluation
~~{ [Indications for FEES, Risks, Benefits, Alternatives to FEES including no evaluation] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Consent Given : Patient Gave Consent
~~{ [Patient Gave Consent] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Teaching Method : Explanation, Video/Educational TV
~~{ [Explanation, Video/Educational TV] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Learner(s) Present : Self, Spouse, Family member
~~{ [Self, Spouse, Family member] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Barriers to Learning : None evident
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [None evident] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Document learning evaluation for Responsible Learner(s)

Speech Language/Pathology Tx Plan : Verbalizes understanding
~~{ [Verbalizes understanding] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Swallowing Strategies : Verbalizes understanding, Needs further teaching
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Verbalizes understanding, Needs further teaching] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Plan

Planned Treatments : Dysphagia therapy
~~{ [Dysphagia therapy] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Frequency : 2-4 times per week
~~{ [2-4 times per week] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Duration : 1-2 weeks
~~{ [1-2 weeks] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

SLP Intensity Per Day : Other: as tolerated
~~{ [Other: as tolerated] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Post-Acute Discharge Therapy Recommendation ST : Continued intermittent therapy to address functional deficits
~~{ [Continued intermittent therapy to address functional deficits] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Rehabilitation Potential : Good
~~{ [Good] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Plan/Goals Established With Patient/Caregiver : Yes
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Yes] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Additional Information

Environmental Safety Implemented : Bed in low position, Call device within reach, Personal items within reach
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [Bed in low position, Call device within reach, Personal items within reach] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

Long Term Goals

SLP Patient, Caregiver Goals : "to eat food"

Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ ["to eat food"] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Cognitive-Communication LTG Grid

	SLP Long Term Goal #1
Goal :	Pt will tolerate least restrictive consistencies with risk of aspiration reduced as much as reasonably possible. Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will tolerate least restrictive consistencies with risk of aspiration reduced as much as reasonably possible.] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Status :	Progressing, continue Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Progressing, continue] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

Timeframe To Complete :	9/13/24 Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [9/13/24] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

Short Term Goals

Other SLP Short Term Goal Grid

	Goal #1	Goal #2	Goal #3
Other :	Pt will tolerate 90ml challenge vs instrumental swallow without uncompensated aspiration or penetration on thin liquids Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will tolerate 90ml challenge vs instrumental swallow without uncompensated aspiration or penetration on thin liquids] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will perform compensatory strategies to protect the airway and improve clearance of the bolus in 80% of trials independently] - previously charted by Castilo CCC-SLP, Noreen ann at~~	Pt will tolerate regular PO trials with no overt s/s of apsiration in 5 out o5 trials Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Pt will tolerate regular PO trials with no overt s/s of apsiration in 5 out o5 trials] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Speech Therapy

		~~8/30/2024 14:29 PDT};~~	
Status :	Goal met Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Goal met] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	Progressing, continue Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Progressing, continue] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~	Initial Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [Initial] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
Date Met :	8/30/2024 PDT Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT ~~{ [8/30/2024 PDT] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~		
	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT	Castilo CCC-SLP, Noreen ann - 8/30/2024 14:29 PDT

ST Non Timed Modalities 2024

ST Treatment Start Time : 8/30/2024 12:58 PDT
~~{ [8/30/2024 12:58 PDT] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
ST Treatment End Time : 8/30/2024 13:58 PDT
~~{ [8/30/2024 13:58 PDT] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
FEES BY CINE/VIDEO RECORDING Min : 60
~~{ [60] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~
ST Total Non Timed : 60
Castilo CCC-SLP, Noreen ann - 8/30/2024 14:39 PDT
~~{ [60] - previously charted by Castilo CCC-SLP, Noreen ann at 8/30/2024 14:29 PDT};~~

Patient: GARCIA, ARMANDO
MRN: HHN8210503; DSH5787309; CHH7351195
DOB/Sex: 6/2/1972 / Male
Attending: Kabsoun MD,Sara H

Admit: 8/28/2024
Disch:
FIN: HHN0005007238115

Progress Note - Physician

DOCUMENT NAME: Progress Note-Physician
SERVICE DATE/TIME: 8/31/2024 17:09 PDT
RESULT STATUS: Auth (Verified)
PERFORM INFORMATION: Parekh DO,Parag P (8/31/2024 17:12 PDT)
SIGN INFORMATION: Parekh DO,Parag P (8/31/2024 17:12 PDT)

Subjective

Consult received
Add fluid restriction and furosemide x1
Monitor corrected Na
Withdrawals, high BP noted. Add coreg
Full eval to follow

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**L** 36.4	08/31/2024 04:17	36.8	08/30/2024 18:15	36.8	08/30/2024 18:15
Heart Rate Monitored	90	08/31/2024 13:36	79	08/31/2024 01:53	94	08/30/2024 17:30
Resp Rate	20	08/31/2024 13:36	16	08/30/2024 17:30	20	08/31/2024 07:11
SBP	**C** 188	08/31/2024 16:12	**H** 173	08/31/2024 14:00	**C** 208	08/30/2024 17:30
DBP	**H** 86	08/31/2024 16:12	79	08/31/2024 04:17	**C** 93	08/30/2024 17:30
MAP	123	08/31/2024 01:53	123	08/31/2024 01:53	138	08/30/2024 20:04
SpO2	96	08/31/2024 13:36	96	08/31/2024 00:17	98	08/30/2024 18:15
O2 Flow Rate	on 4 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Intake and Output (current encounter)

	08/28 01:14 08/31 17:09	08/31 07:00 08/31 17:09	08/30 07:00 08/31 06:59	08/29 07:00 08/30 06:59	08/28 07:00 08/29 06:59
Intake	1319.6	0	0	582.8	716.5
Output	20930	1000	4270	4860	5750
Net Total	-19610.4	-1000	-4270	-4277.2	-5033.5
Stool Count	3	0	2	1	0

Physical Exam

Results

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 127 mmol/L Low (12:28)
Potassium: 4 mmol/L (12:28)
Chloride: 96 mmol/L Low (12:28)
CO2: 25 mmol/L (12:28)
BUN: 48 mg/dL High (12:28)
Creatinine: 1.49 mg/dL High (12:28)
Glucose Level: 302 mg/dL High (12:28)
Calcium: 9.8 mg/dL (12:28)

Progress Note - Physician

Mg Lvl: 2.7 mg/dL High (12:28)

Assessment/Plan
Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Home with family care

Electronically Signed By: Parekh, Parag DO
On: 08.31.2024 17:12 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	8/31/2024 15:12 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Moreno MD,Luis A (8/31/2024 15:14 PDT)
SIGN INFORMATION:	Moreno MD,Luis A (8/31/2024 15:14 PDT)

Subjective
No acute events.
labs and vital sign reviewed
Discussed with nurse

Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Ears and nose without masses, lesions or deformities. Tympanic membranes clear bilaterally. Trachea midline.
Neck: Supple, Full range of motion.
Respiratory: Clear to auscultation and percussion. No wheezing, rales or rhonchi.
Cardiovascular: Heart sounds normal. No thrills. Regular rate and rhythm, no murmurs, rubs or gallops.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds. No pulsatile mass. No hepatosplenomegaly.
Genitourinary: No costovertebral angle tenderness.
Neurologic: Cranial nerves II-XII grossly intact. No focal neurological deficits. Deep tendon reflexes +2 bilaterally. Flexor plantar response. Moves all extremities spontaneously. Sensation intact bilaterally.
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.

Assessment:
Hyponatremia: Sodium 127
AKI (acute kidney injury) (N17.9)
Acute respiratory failure
Sepsis
COPD exacerbation
Congestive heart failure exacerbation
Hyperglycemia
Hypomagnesemia
Hyponatremia

Progress Note - Physician

Hypophosphatemia
Acute on chronic kidney disease
History congestive heart failure
History of COPD on 2 L nasal cannula
History hypertension
History of diabetes mellitus type 2
S/P L carotid endarterectomy with patch angioplasty 7/2024

Plan:
extubated
Nephrologist consulted
Monitor sodium closely
Images reviewed. 08/31
SVN treatments as needed
on rocephina and doxy
HTN urgency added labetalol, started on PO meds with metoprolol and amlodipine bp is still high
added doxazosin
awaiting FEES
Latest echocardiogram 7/2024 EF 60 to 65%
Continue to monitor and trend renal indicis
HR is high per pt going through withdrawals will restart subaxone

Chief Complaint/Reason for Consultation
Chief Complaint: SOB difficulty breathing at home. tympanic temp of 104

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)
No Known Medication Allergies (Severity: Unknown severity, Onset: Unknown)

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**L** 36.4	08/31/2024 04:17	36.8	08/30/2024 18:15	**H** 37.4	08/30/2024 16:00
Heart Rate Monitored	89	08/31/2024 13:26	79	08/31/2024 01:53	94	08/30/2024 17:00
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SBP	**H** 173	08/31/2024 14:00	**H** 173	08/31/2024 14:00	**C** 208	08/30/2024 17:30
DBP	**H** 81	08/31/2024 14:00	79	08/31/2024 04:17	**C** 93	08/30/2024 17:00
MAP	123	08/31/2024 01:53	117	08/30/2024 16:00	138	08/30/2024 20:04
SpO2	96	08/31/2024 13:26	96	08/31/2024 00:17	98	08/30/2024 16:00
O2 Flow Rate	on 4 l/min		on 2 l/min		on 2 l/min	
O2 Therapy	Nasal cannula		Nasal cannula		Nasal cannula	

Intake and Output (current encounter)

	08/28 01:14	08/31 07:00	08/30 07:00	08/29 07:00	08/28 07:00
	08/31 15:12	08/31 15:12	08/31 06:59	08/30 06:59	08/29 06:59
Intake	1319.6	0	0	582.8	716.5
Output	20930	1000	4270	4860	5750

Progress Note - Physician

Net Total	-19610.4	-1000	-4270	-4277.2	-5033.5
Stool Count	3	0	2	1	0

Physical Exam

Home Medications
albuterol (Albuterol (Eqv-ProAir HFA) 90 mcg/inh inhalation aerosol) 2 Puffs Inhalation Every 4 hours as needed as needed for wheezing
amLODIPine (amLODIPine 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
aspirin (Aspirin Low Dose 81 mg oral delayed release tablet) 81 Milligram 1 Tabs By Mouth Daily
atorvastatin (atorvastatin 40 mg oral tablet) 40 Milligram 1 Tabs By Mouth Daily for 30 Days
buprenorphine-naloxone (buprenorphine-naloxone 4 mg-1 mg sublingual film) 1 Film Sublingual 3 Times a Day dissolve under the tongue
cholecalciferol (Vitamin D3 125 mcg (5000 intl units) oral tablet) 125 Microgram 1 Tabs By Mouth Daily
dapagliflozin (Farxiga 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
furosemide (furosemide 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
glimepiride (glimepiride 2 mg oral tablet) 2 Milligram 1 Tabs By Mouth 2 Times a Day
hydrALAZINE (hydrALAZINE 100 mg oral tablet) 100 Milligram 1 Tabs By Mouth 3 Times a Day
metoprolol (Metoprolol Tartrate 100 mg oral tablet) 200 Milligram 2 Tabs By Mouth 2 Times a Day
minoxidil (minoxidil 2.5 mg oral tablet) 5 Milligram 2 Tabs By Mouth 2 Times a Day
mirabegron (Myrbetriq 50 mg oral tablet, extended release) 50 Milligram 1 Tabs By Mouth Daily do not crush or chew
nortriptyline (nortriptyline 25 mg oral capsule) 50 Milligram 2 Capsules By Mouth Every PM
olodaterol (Striverdi Respimat 2.5 mcg/inh inhalation aerosol) 2 Puffs Inhalation Daily
traZODone (traZODone 100 mg oral tablet) 200 Milligram 2 Tabs By Mouth at Bedtime

Inpatient Medications
Medications (27) Active
SCHEDULED: (24)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 5 mg Tab (amLODIPine) 10 mg 2 Tabs, OG-Tube, Daily
aspirin 81 mg Chew Tab (aspirin 81 mg oral tablet, chewable) 81 mg 1 Tabs, OG-Tube, Daily
atorvastatin 40 mg Tab (Lipitor 40 mg oral tablet) 40 mg 1 Tabs, OG-Tube, qHS
buprenorphine 2 mg sublingual Dis Tab (buprenorphine) 4 mg 2 Tabs, SubLingual, TID
cefTRIAXone 2 gm vial (Rocephin 2 g injection) 2 gm 20 mL, IV Push, qHS
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
doxazosin 2 mg Tab (doxazosin) 2 mg 1 Tabs, OG-Tube, Daily
doxycycline + Dextrose 5% in Water 100 mL (doxycycline + Dextrose 5% in Water intravenous solution 100 mL) 100 mg 10 mL, IV Piggyback, BID
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin glargine 1 unit/0.01 mL syringe (Lantus) 5 units 0.05 mL, SubCutaneous, qAM
insulin lispro HumaLOG 1 unit/0.01mL syringe (insulin lispro HumaLOG correctional scale High) 4-18 Units, SubCutaneous, Before Meals and HS

Progress Note - Physician

methylPREDNISolone sod succinate 40 mg Vial PF (methylPREDNISolone sodium succinate (Solu-Medrol)) 20 mg 0.5 mL, IV Push, q12H Interval
metoprolol tartrate 50 mg Tab (Lopressor 50 mg oral tablet) 100 mg 2 Tabs, Oral, BID
nortriptyline 25 mg Cap (nortriptyline) 25 mg 1 Caps, Oral, qPM
pantoprazole 40 mg vial (Protonix) 40 mg, IV Push, Daily
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
traZODone 50 mg Tab (traZODone) 150 mg 3 Tabs, Oral, qHS
CONTINUOUS: (0)
PRN: (3)
acetaminophen 650 mg/20.3 mL Oral Liq (Tylenol Liquid) 650 mg 20.3 mL, OG-Tube, q6H
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H Interval

Results
Recent Labs
General Chemistry

Glucose Level	302 mg/dL (High)	08/31/2024 12:28
POC Glucose	273 mg/dL (High)	08/31/2024 10:49
Sodium	127 mmol/L (Low)	08/31/2024 12:28
Potassium	4.0 mmol/L (Normal)	08/31/2024 12:28
Chloride	96 mmol/L (Low)	08/31/2024 12:28
CO2	25 mmol/L (Normal)	08/31/2024 12:28
Anion Gap	6 mmol/L (Normal)	08/31/2024 12:28
BUN	48 mg/dL (High)	08/31/2024 12:28
Creatinine	1.490 mg/dL (High)	08/31/2024 12:28
BUN/Creat Ratio	32 (High)	08/31/2024 12:28
Calcium	9.8 mg/dL (Normal)	08/31/2024 12:28
Mg Lvl	2.7 mg/dL (High)	08/31/2024 12:28
Estimated Creatinine Clearance	56.11 mL/min ()	08/31/2024 13:43
eGFR Cr	56 mL/min/1.73m2 (N/A)	08/31/2024 12:28
eGFR Pediatric	Not Reported (N/A)	08/31/2024 12:28
Calc Osmo	279 mOsmol/kg (Normal)	08/31/2024 12:28
MAR Glucose Result	273 ()	08/31/2024 11:51

General Hematology

WBC	11.79 x10e3/mcL (Normal)	08/31/2024 12:28
RBC	4.93 x10e6/mcL (Normal)	08/31/2024 12:28

Hgb	15.1 gm/dL (Normal)	08/31/2024 12:28
Hct	47.3 % (Normal)	08/31/2024 12:28
MCV	95.9 Femtoliters (Normal)	08/31/2024 12:28
MCH	30.6 pg (Normal)	08/31/2024 12:28
MCHC	31.9 gm/dL (Normal)	08/31/2024 12:28
RDW-SD	44.8 Femtoliters (Normal)	08/31/2024 12:28
RDW-CV	12.6 % (Normal)	08/31/2024 12:28
Plt	141 x10e3/mcL (Normal)	08/31/2024 12:28
MPV	10.0 Femtoliters (Normal)	08/31/2024 12:28
Neut % Auto	90.2 % (N/A)	08/30/2024 02:33
Lymph % Auto	4.8 % (N/A)	08/30/2024 02:33
Mono % Auto	3.1 % (N/A)	08/30/2024 02:33
Eos % Auto	0.0 % (N/A)	08/30/2024 02:33
Baso % Auto	0.2 % (N/A)	08/30/2024 02:33
Immature Grans %	1.7 % (High)	08/30/2024 02:33
Neut # Auto	10.77 x10e3/mcL (High)	08/30/2024 02:33
Lymph # Auto	0.57 x10e3/mcL (Low)	08/30/2024 02:33
Mono # Auto	0.37 x10e3/mcL (Normal)	08/30/2024 02:33
Eos # Auto	0.00 x10e3/mcL (Normal)	08/30/2024 02:33
Baso # Auto	0.02 x10e3/mcL (Normal)	08/30/2024 02:33
Immature Grans # Auto	0.20 x10e3/mcL (High)	08/30/2024 02:33

Abnormal Labs

General Chemistry

BUN 48 mg/dL (High) 08/31/2024 12:28
BUN/Creat Ratio 32 (High) 08/31/2024 12:28
Chloride 96 mmol/L (Low) 08/31/2024 12:28
Creatinine 1.490 mg/dL (High) 08/31/2024 12:28
Glucose Level 302 mg/dL (High) 08/31/2024 12:28
Mg Lvl 2.7 mg/dL (High) 08/31/2024 12:28
POC Glucose 273 mg/dL (High) 08/31/2024 10:49
Sodium 127 mmol/L (Low) 08/31/2024 12:28

Note: Critical results are displayed in red.

Progress Note - Physician

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**11.79** 08/31/2024 12:28	**11.93** 08/30/2024 02:33	**14.06 H** 08/29/2024 01:32	**12.75 H** 08/28/2024 03:50
Hgb	**15.1** 08/31/2024 12:28	**14.6** 08/30/2024 02:33	**13.8** 08/29/2024 01:32	**13.6** 08/28/2024 03:50
Hct	**47.3** 08/31/2024 12:28	**47.4** 08/30/2024 02:33	**41.9** 08/29/2024 01:32	**41.2** 08/28/2024 03:50
Baso # Auto	**0.02** 08/30/2024 02:33	**0.01** 08/29/2024 01:32	**0.04** 08/27/2024 20:57	
Baso % Auto	**0.2** 08/30/2024 02:33	**0.1** 08/29/2024 01:32	**0.3** 08/27/2024 20:57	
Eos # Auto	**0.00** 08/30/2024 02:33	**0.00** 08/29/2024 01:32	**0.06** 08/27/2024 20:57	
Eos % Auto	**0.0** 08/30/2024 02:33	**0.0** 08/29/2024 01:32	**0.5** 08/27/2024 20:57	
Immature Grans %	**1.7 H** 08/30/2024 02:33	**0.9 H** 08/29/2024 01:32	**1.2 H** 08/27/2024 20:57	
Immature Grans # Auto	**0.20 H** 08/30/2024 02:33	**0.13 H** 08/29/2024 01:32	**0.16 H** 08/27/2024 20:57	
Lymph # Auto	**0.57 L** 08/30/2024 02:33	**0.64 L** 08/29/2024 01:32	**1.13** 08/27/2024 20:57	
Lymph % Auto	**4.8** 08/30/2024 02:33	**4.6** 08/29/2024 01:32	**8.5** 08/27/2024 20:57	
Mono # Auto	**0.37** 08/30/2024 02:33	**0.46** 08/29/2024 01:32	**0.93** 08/27/2024 20:57	
Mono % Auto	**3.1** 08/30/2024 02:33	**3.3** 08/29/2024 01:32	**7.0** 08/27/2024 20:57	
Neut # Auto	**10.77 H** 08/30/2024 02:33	**12.82 H** 08/29/2024 01:32	**10.90 H** 08/27/2024 20:57	
Neut % Auto	**90.2** 08/30/2024 02:33	**91.1** 08/29/2024 01:32	**82.5** 08/27/2024 20:57	
Plt	**141** 08/31/2024 12:28	**148** 08/30/2024 02:33	**123 L** 08/29/2024 01:32	**109 L** 08/28/2024 03:50
Glucose Level	**302 H** 08/31/2024 12:28	**286 H** 08/30/2024 02:33	**264 H** 08/29/2024 01:32	**383 H** 08/28/2024 03:50
Sodium	**127 L** 08/31/2024 12:28	**141** 08/30/2024 02:33	**138** 08/29/2024 01:32	**130 L** 08/28/2024 03:50
Potassium	**4.0** 08/31/2024 12:28	**3.9** 08/30/2024 02:33	**3.7** 08/29/2024 01:32	**4.2** 08/28/2024 03:50
Chloride	**96 L** 08/31/2024 12:28	**110 H** 08/30/2024 02:33	**106** 08/29/2024 01:32	**98** 08/28/2024 03:50
CO2	**25** 08/31/2024 12:28	**26** 08/30/2024 02:33	**29** 08/29/2024 01:32	**27** 08/28/2024 03:50
Creatinine	**1.490 H**	**1.840 H**	**1.620 H**	**1.790 H**

Progress Note - Physician

	08/31/2024 12:28	08/30/2024 02:33	08/29/2024 01:32	08/28/2024 03:50
BUN	**48 H** 08/31/2024 12:28	**48 H** 08/30/2024 02:33	**35 H** 08/29/2024 01:32	**22 H** 08/28/2024 03:50
BUN/Creat Ratio	**32 H** 08/31/2024 12:28	**26 H** 08/30/2024 02:33	**22** 08/29/2024 01:32	**12** 08/28/2024 03:50
Mg Lvl	**2.7 H** 08/31/2024 12:28	**2.5 H** 08/30/2024 02:33	**2.6 H** 08/29/2024 01:32	**2.2** 08/28/2024 03:50
Phosphorus	**4.1** 08/28/2024 03:50	**0.7** 08/27/2024 20:57		
Calcium	**9.8** 08/31/2024 12:28	**10.2 H** 08/30/2024 02:33	**10.2 H** 08/29/2024 01:32	**9.8** 08/28/2024 03:50
Albumin. Level	**4.0** 08/27/2024 20:57			
Ur Protein	**3+** 08/27/2024 22:04			
T Bili	**1.0** 08/27/2024 20:57			
Alk Phos	**72** 08/27/2024 20:57			
AST	**20** 08/27/2024 20:57			
ALT	**24** 08/27/2024 20:57			
aPTT	**31** 08/28/2024 00:53			
PT	**11.9** 08/28/2024 00:53			
INR	**1.1** 08/28/2024 00:53			

Assessment/Plan
Diagnoses
AKI (acute kidney injury) (N17.9)
Bilateral lower extremity edema (R60.0)
CHF exacerbation (I50.9)
COPD exacerbation (J44.1)
Congestive heart failure (I50.9)
Elevated brain natriuretic peptide (BNP) level (R79.89)
Fever (R50.9)
History of pneumonia (Z87.01)
Hyperglycemia (R73.9)
Hyperglycemia due to diabetes mellitus (E11.65)
Hypertensive emergency (I16.1)
Hypomagnesemia (E83.42)
Hyponatremia (E87.1)

Hypophosphatemia (E83.39)
Leukocytosis (D72.829)
Morbid obesity (E66.01)
Oxygen dependent (Z99.81)
Respiratory distress (R06.03)
Respiratory failure (J96.90)
Sepsis (A41.9)
1. Respiratory failure (J96.90)

Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Home with family care

Electronically Signed By: Moreno, Luis MD
On: 08.31.2024 15:14 PDT

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	8/30/2024 14:06 PDT
RESULT STATUS:	Modified
PERFORM INFORMATION:	Kabsoun MD,Sara H (8/30/2024 14:10 PDT)
SIGN INFORMATION:	Kabsoun MD,Sara H (8/30/2024 19:48 PDT); Kabsoun MD, Sara H (8/30/2024 14:10 PDT)

Subjective
seen and examined no chest pain no sob discussed with RN

Allergies
Allergies (Active and Proposed Allergies Only)
No Known Allergies (Severity: Unknown severity, Onset: Unknown)
No Known Medication Allergies (Severity: Unknown severity, Onset: Unknown)

Past Medical History
Active Problems(6)
Disc disorder
DM (diabetes mellitus)
ESBL Escherichia coli
Gout
HLD (hyperlipidemia)
HTN (hypertension)

Past Surgical History
Bronchoscopy w/ Procedures (Laterality NA): 08/29/24
Endarterectomy Carotid Artery (Left): 07/26/24
EGD w/ Biopsy (Laterality NA): 06/27/22
foot sx
shoulder sx

Progress Note - Physician

Social History

Alcohol
Risk Assessment: Denies Alcohol Use; Details: Denies

Substance Abuse
Risk Assessment: Denies Substance Abuse; Details: Denies

Tobacco
Risk Assessment: High Risk; Details: Current

Family History
No Family History documented.

Objective

Vital Signs (24 hrs)	Last Charted		Minimum		Maximum	
Temp	**H** 37.8	08/30/2024 12:00	**H** 37.6	08/29/2024 19:00	**H** 38	08/29/2024 20:00
Heart Rate Monitored	99	08/30/2024 12:00	73	08/29/2024 15:00	**H** 123	08/29/2024 20:00
Resp Rate	**H** 21	08/30/2024 12:00	16	08/30/2024 03:30	**C** 35	08/30/2024 02:30
SBP	**C** 207	08/30/2024 12:00	134	08/30/2024 03:30	**C** 207	08/30/2024 12:00
DBP	**C** 96	08/30/2024 12:00	63	08/30/2024 03:30	**C** 96	08/30/2024 12:00
MAP	138	08/30/2024 12:00	91	08/30/2024 03:30	138	08/30/2024 12:00
SpO2	97	08/30/2024 12:00	94	08/29/2024 20:00	99	08/29/2024 15:00
O2 Flow Rate	on 2 l/min		on 5 l/min		on 30 %	
O2 Therapy	Nasal cannula		Nasal cannula		Ventilator	

Intake and Output (current encounter)

	08/28 01:14	08/30 07:00	08/29 07:00	08/28 07:00	08/27 07:00
	08/30 14:06	08/30 14:06	08/30 06:59	08/29 06:59	08/28 06:59
Intake	1319.6	0	582.8	716.5	20.3
Output	18160	2500	4860	5750	5050
Net Total	-16840.4	-2500	-4277.2	-5033.5	-5029.7
Stool Count	2	1	1	0	0

Mobility & Ambulation Level
PT Bed Mobility
Bed Mobility Roll Left: Rehab Contact guard assistance (08/30/24)
Bed Mobility Roll Right: Rehab Contact guard assistance (08/30/24)
Bed Mobility Supine to Sit: Rehab Contact guard assistance (08/30/24)
Bed Mobility Sit to Supine: Rehab Contact guard assistance (08/30/24)
PT Transfer Mobility
Transfer Sit to Stand Rehab: Rehab Contact guard assistance (08/30/24)
Transfer Stand to Sit Rehab: Rehab Contact guard assistance (08/30/24)

Progress Note - Physician

Transfer Bed to and From Chair Rehab: Rehab Contact guard assistance (08/30/24)

Physical Exam

Head: Normocephalic.
Eyes: Pupils are equal, round and reactive to light. Extraocular muscles intact.
Ear, Nose and Throat: Oropharynx clear, mucous membranes moist. Ears and nose without masses, lesions or deformities. Tympanic membranes clear bilaterally. Trachea midline.
Neck: Supple, Full range of motion.
Respiratory: Clear to auscultation and percussion. No wheezing, rales or rhonchi.
Cardiovascular: Heart sounds normal. No thrills. Regular rate and rhythm, no murmurs, rubs or gallops.
Gastrointestinal: Abdomen soft, non-tender, non-distended. Normal bowel sounds. No pulsatile mass. No hepatosplenomegaly.
Genitourinary: No costovertebral angle tenderness.
Neurologic: Cranial nerves II-XII grossly intact. No focal neurological deficits. Deep tendon reflexes +2 bilaterally. Flexor plantar response. Moves all extremities spontaneously. Sensation intact bilaterally.
Skin: No rashes or lesions. No petechiae or purpura. No edema.
Musculoskeletal: No cyanosis or clubbing. No gross deformities. Normal range of motion.
Lymphatics: Palpation of neck reveals no swelling or tenderness of neck nodes. Palpation of groin reveals no swelling or tenderness of groin nodes.

Home Medications
albuterol (Albuterol (Eqv-ProAir HFA) 90 mcg/inh inhalation aerosol) 2 Puffs Inhalation Every 4 hours as needed as needed for wheezing
amLODIPine (amLODIPine 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
aspirin (Aspirin Low Dose 81 mg oral delayed release tablet) 81 Milligram 1 Tabs By Mouth Daily
atorvastatin (atorvastatin 40 mg oral tablet) 40 Milligram 1 Tabs By Mouth Daily for 30 Days
buprenorphine-naloxone (buprenorphine-naloxone 4 mg-1 mg sublingual film) 1 Film Sublingual 3 Times a Day dissolve under the tongue
cholecalciferol (Vitamin D3 125 mcg (5000 intl units) oral tablet) 125 Microgram 1 Tabs By Mouth Daily
dapagliflozin (Farxiga 10 mg oral tablet) 10 Milligram 1 Tabs By Mouth Daily
furosemide (furosemide 20 mg oral tablet) 20 Milligram 1 Tabs By Mouth Daily
glimepiride (glimepiride 2 mg oral tablet) 2 Milligram 1 Tabs By Mouth 2 Times a Day
hydrALAZINE (hydrALAZINE 100 mg oral tablet) 100 Milligram 1 Tabs By Mouth 3 Times a Day
metoprolol (Metoprolol Tartrate 100 mg oral tablet) 200 Milligram 2 Tabs By Mouth 2 Times a Day
minoxidil (minoxidil 2.5 mg oral tablet) 5 Milligram 2 Tabs By Mouth 2 Times a Day
mirabegron (Myrbetriq 50 mg oral tablet, extended release) 50 Milligram 1 Tabs By Mouth Daily do not crush or chew
nortriptyline (nortriptyline 25 mg oral capsule) 50 Milligram 2 Capsules By Mouth Every PM
olodaterol (Striverdi Respimat 2.5 mcg/inh inhalation aerosol) 2 Puffs Inhalation Daily
traZODone (traZODone 100 mg oral tablet) 200 Milligram 2 Tabs By Mouth at Bedtime

Inpatient Medications
Medications (24) Active
SCHEDULED: (20)

Progress Note - Physician

albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
amLODIPine 5 mg Tab (amLODIPine) 10 mg 2 Tabs, Oral, Daily
aspirin 81 mg Chew Tab (aspirin 81 mg oral tablet, chewable) 81 mg 1 Tabs, OG-Tube, Daily
atorvastatin 40 mg Tab (Lipitor 40 mg oral tablet) 40 mg 1 Tabs, OG-Tube, qHS
buprenorphine 2 mg sublingual Dis Tab (buprenorphine) 4 mg 2 Tabs, SubLingual, TID
cefTRIAXone 2 gm vial (Rocephin 2 g injection) 2 gm 20 mL, IV Push, qHS
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
doxycycline + Dextrose 5% in Water 100 mL (doxycycline + Dextrose 5% in Water intravenous solution 100 mL) 100 mg 10 mL, IV Piggyback, BID
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin lispro HumaLOG 1 unit/0.01mL syringe (insulin lispro HumaLOG correctional scale High) 4-18 Units, SubCutaneous, q4H Interval
methylPREDNISolone sod succinate 40 mg Vial PF (methylPREDNISolone sodium succinate (Solu-Medrol)) 20 mg 0.5 mL, IV Push, q12H Interval
metoprolol tartrate 50 mg Tab (Lopressor 50 mg oral tablet) 50 mg 1 Tabs, Oral, BID
pantoprazole 40 mg vial (Protonix) 40 mg, IV Push, Daily
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
CONTINUOUS: (0)
PRN: (4)
acetaminophen 650 mg/20.3 mL Oral Liq (Tylenol Liquid) 650 mg 20.3 mL, OG-Tube, q6H
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
labetalol 20 mg/4 mL inj (labetalol) 10 mg 2 mL, IV Push, q4H Interval
morphine 4 mg/mL injection 1 mL (morphine) 4 mg 1 mL, IV Push, q4H

Results
Recent Labs
Blood Gases

pH Arterial	7.47 (High)	08/29/2024 15:09
Art pCO2	43.6 mmHg (Normal)	08/29/2024 15:09
Art pO2	84.9 mmHg (Normal)	08/29/2024 15:09
HCO3 Art	30.9 mmol/L (High)	08/29/2024 15:09
Arterial pCO2(T)	43.6 mmHg (N/A)	08/29/2024 15:09
Arterial pH(T)	7.47 (N/A)	08/29/2024 15:09
Arterial pO2(T)	84.9 mmHg (N/A)	08/29/2024 15:09
Art Base Excess	7.1 mmol/L (High)	08/29/2024 15:09
Art sO2	97 % (Normal)	08/29/2024 15:09
POC Pt Temp	37.0 DegC (N/A)	08/29/2024 15:09
POC FIO2	30 % (N/A)	08/29/2024 15:09
POC Sample	Arterial (N/A)	08/29/2024 15:09
POC Site	Radial Right (N/A)	08/29/2024 15:09
POC Allen Test	POS (N/A)	08/29/2024 15:09

Progress Note - Physician

POC Del Sys	VENTILATOR (N/A)	08/29/2024 15:09
POC Vent Mode	SPN-CPAP VS (N/A)	08/29/2024 15:09
POC Total RR	18 br/min (N/A)	08/29/2024 15:09
POC Set RR	18 br/min (N/A)	08/29/2024 04:00
POC VE	10.0 L/min (N/A)	08/29/2024 04:00
POC VT	550 mL (N/A)	08/29/2024 04:00
POC PEEP	5 cm H2O (N/A)	08/29/2024 15:09
POC PS	7 cm H2O (N/A)	08/29/2024 15:09
POC IT	0.900 Seconds (N/A)	08/29/2024 04:00

General Chemistry

Glucose Level	286 mg/dL (High)	08/30/2024 02:33
POC Glucose	247 mg/dL (High)	08/30/2024 13:44
Sodium	141 mmol/L (Normal)	08/30/2024 02:33
Potassium	3.9 mmol/L (Normal)	08/30/2024 02:33
Chloride	110 mmol/L (High)	08/30/2024 02:33
CO2	26 mmol/L (Normal)	08/30/2024 02:33
Anion Gap	5 mmol/L (Normal)	08/30/2024 02:33
BUN	48 mg/dL (High)	08/30/2024 02:33
Creatinine	1.840 mg/dL (High)	08/30/2024 02:33
BUN/Creat Ratio	26 (High)	08/30/2024 02:33
Calcium	10.2 mg/dL (High)	08/30/2024 02:33
Mg Lvl	2.5 mg/dL (High)	08/30/2024 02:33
Estimated Creatinine Clearance	45.43 mL/min ()	08/30/2024 03:15
eGFR Cr	44 mL/min/1.73m2 (N/A)	08/30/2024 02:33
eGFR Pediatric	Not Reported (N/A)	08/30/2024 02:33
Calc Osmo	304 mOsmol/kg (High)	08/30/2024 02:33
MAR Glucose Result	247 ()	08/30/2024 13:50

General Hematology

WBC	11.93 x10e3/mcL (Normal)	08/30/2024 02:33
RBC	4.84 x10e6/mcL (Normal)	08/30/2024 02:33
Hgb	14.6 gm/dL (Normal)	08/30/2024 02:33
Hct	47.4 % (Normal)	08/30/2024 02:33
MCV	97.9 Femtoliters (High)	08/30/2024 02:33
MCH	30.2 pg (Normal)	08/30/2024 02:33
MCHC	30.8 gm/dL (Low)	08/30/2024 02:33

RDW-SD	47.8 Femtoliters (Normal)	08/30/2024 02:33
RDW-CV	13.2 % (Normal)	08/30/2024 02:33
Plt	148 x10e3/mcL (Normal)	08/30/2024 02:33
MPV	10.4 Femtoliters (Normal)	08/30/2024 02:33
Neut % Auto	90.2 % (N/A)	08/30/2024 02:33
Lymph % Auto	4.8 % (N/A)	08/30/2024 02:33
Mono % Auto	3.1 % (N/A)	08/30/2024 02:33
Eos % Auto	0.0 % (N/A)	08/30/2024 02:33
Baso % Auto	0.2 % (N/A)	08/30/2024 02:33
Immature Grans %	1.7 % (High)	08/30/2024 02:33
Neut # Auto	10.77 x10e3/mcL (High)	08/30/2024 02:33
Lymph # Auto	0.57 x10e3/mcL (Low)	08/30/2024 02:33
Mono # Auto	0.37 x10e3/mcL (Normal)	08/30/2024 02:33
Eos # Auto	0.00 x10e3/mcL (Normal)	08/30/2024 02:33
Baso # Auto	0.02 x10e3/mcL (Normal)	08/30/2024 02:33
Immature Grans # Auto	0.20 x10e3/mcL (High)	08/30/2024 02:33

Abnormal Labs

Blood Gases

Art Base Excess 7.1 mmol/L (High) 08/29/2024 15:09
HCO3 Art 30.9 mmol/L (High) 08/29/2024 15:09
pH Arterial 7.47 (High) 08/29/2024 15:09

General Chemistry

BUN 48 mg/dL (High) 08/30/2024 02:33
BUN/Creat Ratio 26 (High) 08/30/2024 02:33
Calc Osmo 304 mOsmol/kg (High) 08/30/2024 02:33
Calcium 10.2 mg/dL (High) 08/30/2024 02:33
Chloride 110 mmol/L (High) 08/30/2024 02:33
Creatinine 1.840 mg/dL (High) 08/30/2024 02:33
Glucose Level 286 mg/dL (High) 08/30/2024 02:33
Mg Lvl 2.5 mg/dL (High) 08/30/2024 02:33
POC Glucose 247 mg/dL (High) 08/30/2024 13:44

General Hematology

Immature Grans % 1.7 % (High) 08/30/2024 02:33
Immature Grans # Auto 0.20 x10e3/mcL (High) 08/30/2024 02:33

Progress Note - Physician

```
Lymph # Auto          0.57 x10e3/mcL (Low)          08/30/2024 02:33
MCHC          30.8 gm/dL (Low)          08/30/2024 02:33
MCV          97.9 Femtoliters (High)          08/30/2024 02:33
Neut # Auto          10.77 x10e3/mcL (High)          08/30/2024 02:33
```

Note: Critical results are displayed in red.

CBC, BMP, Coagulation Trend (last 4 resulted)

WBC	**11.93** 08/30/2024 02:33	**14.06 H** 08/29/2024 01:32	**12.75 H** 08/28/2024 03:50	**13.22 H** 08/27/2024 20:57
Hgb	**14.6** 08/30/2024 02:33	**13.8** 08/29/2024 01:32	**13.6** 08/28/2024 03:50	**14.1** 08/27/2024 20:57
Hct	**47.4** 08/30/2024 02:33	**41.9** 08/29/2024 01:32	**41.2** 08/28/2024 03:50	**41.2** 08/27/2024 20:57
Baso # Auto	**0.02** 08/30/2024 02:33	**0.01** 08/29/2024 01:32	**0.04** 08/27/2024 20:57	
Baso % Auto	**0.2** 08/30/2024 02:33	**0.1** 08/29/2024 01:32	**0.3** 08/27/2024 20:57	
Eos # Auto	**0.00** 08/30/2024 02:33	**0.00** 08/29/2024 01:32	**0.06** 08/27/2024 20:57	
Eos % Auto	**0.0** 08/30/2024 02:33	**0.0** 08/29/2024 01:32	**0.5** 08/27/2024 20:57	
Immature Grans %	**1.7 H** 08/30/2024 02:33	**0.9 H** 08/29/2024 01:32	**1.2 H** 08/27/2024 20:57	
Immature Grans # Auto	**0.20 H** 08/30/2024 02:33	**0.13 H** 08/29/2024 01:32	**0.16 H** 08/27/2024 20:57	
Lymph # Auto	**0.57 L** 08/30/2024 02:33	**0.64 L** 08/29/2024 01:32	**1.13** 08/27/2024 20:57	
Lymph % Auto	**4.8** 08/30/2024 02:33	**4.6** 08/29/2024 01:32	**8.5** 08/27/2024 20:57	
Mono # Auto	**0.37** 08/30/2024 02:33	**0.46** 08/29/2024 01:32	**0.93** 08/27/2024 20:57	
Mono % Auto	**3.1** 08/30/2024 02:33	**3.3** 08/29/2024 01:32	**7.0** 08/27/2024 20:57	
Neut # Auto	**10.77 H** 08/30/2024 02:33	**12.82 H** 08/29/2024 01:32	**10.90 H** 08/27/2024 20:57	
Neut % Auto	**90.2** 08/30/2024 02:33	**91.1** 08/29/2024 01:32	**82.5** 08/27/2024 20:57	
Plt	**148** 08/30/2024 02:33	**123 L** 08/29/2024 01:32	**109 L** 08/28/2024 03:50	**117 L** 08/27/2024 20:57
Glucose Level	**286 H** 08/30/2024 02:33	**264 H** 08/29/2024 01:32	**383 H** 08/28/2024 03:50	**316 H** 08/27/2024 20:57
Sodium	**141** 08/30/2024 02:33	**138** 08/29/2024 01:32	**130 L** 08/28/2024 03:50	**134 L** 08/27/2024 20:57
Potassium	**3.9**	**3.7**	**4.2**	**3.9**

Progress Note - Physician

	08/30/2024 02:33	08/29/2024 01:32	08/28/2024 03:50	08/27/2024 20:57
Chloride	**110 H** 08/30/2024 02:33	**106** 08/29/2024 01:32	**98** 08/28/2024 03:50	**100** 08/27/2024 20:57
CO2	**26** 08/30/2024 02:33	**29** 08/29/2024 01:32	**27** 08/28/2024 03:50	**28** 08/27/2024 20:57
Creatinine	**1.840 H** 08/30/2024 02:33	**1.620 H** 08/29/2024 01:32	**1.790 H** 08/28/2024 03:50	**2.020 H** 08/27/2024 20:57
BUN	**48 H** 08/30/2024 02:33	**35 H** 08/29/2024 01:32	**22 H** 08/28/2024 03:50	**17** 08/27/2024 20:57
BUN/Creat Ratio	**26 H** 08/30/2024 02:33	**22** 08/29/2024 01:32	**12** 08/28/2024 03:50	**8** 08/27/2024 20:57
Mg Lvl	**2.5 H** 08/30/2024 02:33	**2.6 H** 08/29/2024 01:32	**2.2** 08/28/2024 03:50	**1.7 L** 08/27/2024 20:57
Phosphorus	**4.1** 08/28/2024 03:50	**0.7** 08/27/2024 20:57		
Calcium	**10.2 H** 08/30/2024 02:33	**10.2 H** 08/29/2024 01:32	**9.8** 08/28/2024 03:50	**10.4 H** 08/27/2024 20:57
Albumin. Level	**4.0** 08/27/2024 20:57			
Ur Protein	**3+** 08/27/2024 22:04			
T Bili	**1.0** 08/27/2024 20:57			
Alk Phos	**72** 08/27/2024 20:57			
AST	**20** 08/27/2024 20:57			
ALT	**24** 08/27/2024 20:57			
aPTT	**31** 08/28/2024 00:53			
PT	**11.9** 08/28/2024 00:53			
INR	**1.1** 08/28/2024 00:53			

Blood Glucose Trend

Glucose Level: 286 mg/dL High (08/30/24 02:33:00)
POC Glucose: 247 mg/dL High (08/30/24 13:44:00)
POC Glucose: 276 mg/dL High (08/30/24 09:50:00)
POC Glucose: 329 mg/dL High (08/30/24 05:48:00)
POC Glucose: 246 mg/dL High (08/30/24 01:54:00)
POC Glucose: 214 mg/dL High (08/29/24 21:07:00)
POC Glucose: 230 mg/dL High (08/29/24 17:36:00)

CBC (Last Within 24hrs)
WBC: 11.93 x10e3/mcL (02:33)
Hgb: 14.6 gm/dL (02:33)
Hct: 47.4 % (02:33)
Plt: 148 x10e3/mcL (02:33)

Differential (Last Within 24hrs)
- Automated -

Neut % Auto: 90.2 % (02:33)
Lymph % Auto: 4.8 % (02:33)
Mono % Auto: 3.1 % (02:33)
Eos % Auto: 0 % (02:33)
Baso % Auto: 0.2 % (02:33)
Immature Grans %: 1.7 % High (02:33)
Neut # Auto: 10.77 x10e3/mcL High (02:33)
Lymph # Auto: 0.57 x10e3/mcL Low (02:33)
Mono # Auto: 0.37 x10e3/mcL (02:33)
Baso # Auto: 0.02 x10e3/mcL (02:33)
Immature Grans # Auto: 0.2 x10e3/mcL High (02:33)

BMP, Mg, and Phos (Last Within 24hrs)
Sodium: 141 mmol/L (02:33)
Potassium: 3.9 mmol/L (02:33)
Chloride: 110 mmol/L High (02:33)
CO2: 26 mmol/L (02:33)
BUN: 48 mg/dL High (02:33)
Creatinine: 1.84 mg/dL High (02:33)
Glucose Level: 286 mg/dL High (02:33)
Calcium: 10.2 mg/dL High (02:33)
Mg Lvl: 2.5 mg/dL High (02:33)

Microbiology

Culture Respiratory		**Stain**	
Source:	Bronchial Lavage	Body Site:	
Collected Dt/Tm:	08/29/2024 13:08	Last Updated Dt/Tm:	08/29/2024 21:13

Progress Note - Physician

Rare epithelial cells
Many White Blood Cells
Many Gram Negative Rods
Rare Gram Positive Rods
Rare gram positive cocci in pairs
Rare Yeast

Culture Sputum		**Completed**	
Source:	Sputum, Suctioned	Body Site:	
Collected Dt/Tm:	08/28/2024 01:12	Last Updated Dt/Tm:	08/29/2024 16:52

Heavy Growth Mixed Upper Respiratory Flora

Culture Blood ARD		**Preliminary**	
Source:	Blood	Body Site:	
Collected Dt/Tm:	08/27/2024 21:01	Last Updated Dt/Tm:	08/30/2024 02:01

No growth at 48 hours.

Cardiology Labs
NT-ProBNP: 1242 pg/mL High (08/27/24 20:57:00)
Troponin TNIH: 105.7 ng/L Critical (08/28/24 00:53:00)
Troponin TNIH: 63.6 ng/L (08/27/24 22:04:00)
Troponin TNIH: 44.1 ng/L (08/27/24 20:57:00)

Blood Gases (Last Within 24hrs)
pH Arterial: 7.47 High (15:09)
Art pO2: 84.9 mmHg (15:09)
Art pCO2: 43.6 mmHg (15:09)
HCO3 Art: 30.9 mmol/L High (15:09)
Art sO2: 97 % (15:09)

Assessment/Plan
Diagnoses
AKI (acute kidney injury) (N17.9)
Acute respiratory failure
Sepsis
COPD exacerbation
Congestive heart failure exacerbation
Hyperglycemia
Hypomagnesemia
Hyponatremia
Hypophosphatemia
Acute on chronic kidney disease
History congestive heart failure
History of COPD on 2 L nasal cannula

Progress Note - Physician

History hypertension
History of diabetes mellitus type 2
S/P L carotid endarterectomy with patch angioplasty 7/2024

Plan:
extubated
SVN treatments as needed
on rocephina and doxy
HTN urgency added labetalol, started on PO meds with metoprolol and amlodipine bp is still high
added doxazosin
awaiting FEES
Latest echocardiogram 7/2024 EF 60 to 65%
Continue to monitor and trend renal indicis
HR is high per pt going through withdrawals will restart subaxone
Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Home with family care

Discharge Planning:
» **Discharge Planning:**
» **Discharge To, Anticipated:** Home with family care

Electronically Signed By: Kabsoun, Sara MD
On: 08.30.2024 14:10 PDT

Electronically Signed On: 08.30.2024 19:48 PDT
Kabsoun, Sara MD

DOCUMENT NAME:	Progress Note-Physician
SERVICE DATE/TIME:	8/30/2024 10:22 PDT
RESULT STATUS:	Auth (Verified)
PERFORM INFORMATION:	Al-owir MD,Bassam S (8/30/2024 10:25 PDT)
SIGN INFORMATION:	Al-owir MD,Bassam S (8/30/2024 10:25 PDT)

Garcia, A

Patient: **GARCIA, ARMANDO** **MRN: HHN8210503** **FIN: HHN0005007238115**
Age: **52 years** Sex: **Male** DOB: **6/2/1972**
Associated Diagnoses: **None**
Author: **Al-owir MD, Bassam S**

Plan
Impression and Plan: **Diagnosis**

Respiratory failure
COPD exacerbation
CHF exacerbation
Pneumonia

Progress Note - Physician

AKI
Hypophosphatemia
Hyperglycemia
Hypokalemia
HTN emergency
H/O DM , COPD, Carotid endarterectomy
CHF, and HTN

Plan:

extubated

O2

follow BAL

follow ABG`s

SVN

wean down steroids

ABX

IV Hydralazine

Amlodipine

Metoprolol

Close VS and rhythm monitoring

D/C Lasix

follow BUN/Cr

Strict I`s and O's

electrolytes per protocol

ACCU checks / ISS

DVT/GI prophylaxis

see orders
D/W RN, RT, and the pharmacist.
.

Objective
Intake and Output
Fluid Balance Primitives

8/30/2024 8:00 PDT	Stool Count	1
8/30/2024 6:00 PDT	Urine Catheter.	160 mL

Progress Note - Physician

8/30/2024 5:00 PDT	Urine Catheter.	130 mL
8/30/2024 4:00 PDT	Urine Catheter.	160 mL
8/30/2024 3:00 PDT	Urine Catheter.	140 mL
8/30/2024 2:00 PDT	Urine Catheter.	170 mL
8/30/2024 1:00 PDT	Urine Catheter.	110 mL
8/30/2024 0:00 PDT	Urine Catheter.	150 mL
8/29/2024 23:00 PDT	Urine Catheter.	150 mL
8/29/2024 22:00 PDT	Urine Catheter.	140 mL
8/29/2024 21:00 PDT	Urine Catheter.	120 mL
8/29/2024 20:00 PDT	Urine Catheter.	150 mL
8/29/2024 19:00 PDT	Urine Catheter.	140 mL
8/29/2024 18:00 PDT	Urine Catheter.	150 mL
8/29/2024 17:00 PDT	Urine Catheter.	200 mL

VS/Measurements
Inpt. Vital Signs (ST)

Vital Signs (24 hrs)	Last Charted	Minimum	Maximum
Temp	**H** 38.5	(AUG 30 04:00)	**H** 37.6 (AUG 29 19:00) **H** 38 (AUG 29 20:00)
Peripheral Pulse Rate	85 (AUG 30 07:53)	85 (AUG 30 07:53)	94 (AUG 30 01:37)
Resp Rate	20 (AUG 30 09:58)	14 (AUG 29 13:16)	**C** 35 (AUG 30 02:30)
SBP	**C** 184 (AUG 30 06:30)	134 (AUG 30 03:30)	**C** 204 (AUG 29 19:00)
DBP	**H** 86 (AUG 30 06:30)	63 (AUG 30 03:30)	**C** 94 (AUG 29 19:00)
SpO2	96 (AUG 30 07:51)	94 (AUG 29 11:00)	99 (AUG 29 13:00)
	on 5 L/min	on 30 %	
	Nasal cannula	Ventilator	

General: Alert and oriented.
HENT: Normocephalic, Oral mucosa is moist, Oropharynx Clear.
Neck: Supple, Non-tender, No carotid bruit, No jugular venous distention.
Respiratory: Breath sounds are equal, Symmetrical chest wall expansion, basilar ronchi.
Cardiovascular: Normal rate, Regular rhythm, No murmur.
Gastrointestinal: Soft, Non-tender, Non-distended, Normal bowel sounds.

Health Status
Allergies:

Allergies (2) Active	Reaction
No Known Allergies	None Documented
No Known Medication Allergies	None Documented

Current medications: Inpatient medications (ST)
Medications (22) Active
SCHEDULED: (18)
albuterol-ipratropium Inh Sol 3 mL (DuoNeb) 3 mL, NEB, RT q6H
aspirin 81 mg Chew Tab (aspirin 81 mg oral tablet, chewable) 81 mg 1 Tabs, OG-Tube, Daily
atorvastatin 40 mg Tab (Lipitor 40 mg oral tablet) 40 mg 1 Tabs, OG-Tube, qHS
cefTRIAXone 2 gm vial (Rocephin 2 g injection) 2 gm 20 mL, IV Push, qHS
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily
Consult to Pharmacy (Pharmacy Consult) 1 Each, Misc, Daily

Progress Note - Physician

doxycycline + Dextrose 5% in Water 100 mL (doxycycline + Dextrose 5% in Water intravenous solution 100 mL) 100 mg 10 mL, IV Piggyback, BID
ethanol topical (Nozin POPswab 62% topical swab) 1 amp, Topical, BID
furosemide 40 mg/4 mL vial (furosemide 10 mg/mL injectable solution) 40 mg 4 mL, IV Push, q12H
Hypoglycemia Protocol Advisor Protocol, N/A, Daily
insulin lispro HumaLOG 1 unit/0.01mL syringe (insulin lispro HumaLOG correctional scale High) 4-18 Units, SubCutaneous, q4H Interval
methylPREDNISolone sod succinate 40 mg Vial PF (methylPREDNISolone sodium succinate (Solu-Medrol)) 40 mg 1 mL, IV Push, q12H Interval
pantoprazole 40 mg vial (Protonix) 40 mg, IV Push, Daily
Protocol (Magnesium Oxide - Oral.) Protocol, N/A, q72H Interval
Protocol (Magnesium Sulfate - IV.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - Oral.) Protocol, N/A, q72H Interval
Protocol (Sodium Phosphate - IV.) Protocol, N/A, q72H Interval
Protocol (Potassium Chloride - Oral & IV.) Protocol, N/A, q72H Interval
CONTINUOUS: (1)
dexmedeTOMIDine 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL (dexmedeTOMIDine IV additive 400 mcg [0.4 mcg/kg/hr] + Premix NS Sedatives 100 mL) 100 mL, IV, 12.93 mL/hr
PRN: (3)
acetaminophen 650 mg/20.3 mL Oral Liq (Tylenol Liquid) 650 mg 20.3 mL, OG-Tube, q6H
hydrALAZINE 20 mg/mL vial (hydrALAZINE) 10 mg 0.5 mL, IV Push, q4H
morphine 4 mg/mL injection 1 mL (morphine) 4 mg 1 mL, IV Push, q4H

Problem list:
Active Problems (12)
Acute pain
At risk for falls
At risk for injury due to fall
Bowel dysfunction
Disc disorder
DM (diabetes mellitus)
ESBL Escherichia coli
Gout
HLD (hyperlipidemia)
HTN (hypertension)
Ineffective airway clearance
Sleep apnea

Assessment
Interpretation of Results
Laboratory: Fishbone Labs ST
AUG 30 02:33

141 **| H 110 | H 48 /**
_____ **H 286**

3.9 | 26 **| H 1.840 **

AUG 30 02:33

**** 14.6 **/**
11.93 _____ 148

Progress Note - Physician

/ 47.4 **\.**

Electronically Signed By: Al-owir, Bassam MD
On: 08.30.2024 10:25 PDT

Patient Information Form

Hospital Name: Henderson Hospital **Phone:** (702) 963-7510

Hospital Address: 1050 WEST GALLERIA DRIVE HENDERSON, NV 89011

Patient Demographics

Patient Name: GARCIA, ARMANDO
Medical Rec #: 8210503 **SSN:**
Marital Status: M **Gender:** M
Date of Birth: 06-02-1972 **Age:** 52
Religion: CAT
Episode ID: 5007238115
Height:
Weight:

Admit and Length of Stay Information

Unit: IMC
Admit Type: EMERGENCY **Room:** 2027
Admission Date: 08-28-2024 **Bed:** 01
Est. Discharge Date: 09-01-2024 **ALC Date:**
Pt. Functional Status Prior to Admission:
PCP:
Attending Physician: KABSOUN SARA,
Phone Number:

Diagnosis Information:

Rugs: **Primary:** Respiratory failure, unspecified, unsp **Secondary:** Fever, unspecified

Discharge Notes:

Notes:

Mode of Transportation:
Will patient receive radiation or dialysis off-site?
☐ Yes ☐ No

Payer Source:
Schedule of Treatments:

Patient Address

Living Arrangement: Select One
Facility Name:
Street: 6347 RUBYLYN AVE
City: LAS VEGAS
State/Zip: NV 891221111
Home Phone: (323)690-6321
Work Phone: (323)690-6321

Next of Kin

First Name/MI: VERONICA
Last Name: GARCIA
Street: 6347 RUBYLYN AVE
City: LAS VEGAS
State/Zip: NV 891221111
Home Phone: (323)501-5123
Work Phone:
Relation: SPOUSE
☐ Emerg. Contact ☐ POA

Emergency Contact:

First Name/MI:
Last Name:
Street:
City:
State/Zip:
Home Phone:
Work Phone:
Relation:
☐ Emerg. Contact ☐ POA

Payer Information:

Ins. Group ID#:
Primary Payer: MEDICARE **Member ID#:** 2EC2PY1UK70
Contact person at Ins. Co. (First/MI/Last): **Phone:**
Patient has met 3 consecutive, acute level of care days during this admission & may be eligible for the Medicare Extended Care Benefit. ☐ Yes ☐ No ☐ N/A ☐ Unknown

Secondary Payer: **Member ID#:** **Phone #:**
Other Payer: **Member ID#:** **Phone #:**

Income if known: ☐ Private Funds ☐ SSA ☐ SSI ☐ Pension ☐ VA ☐ Other

Patient Medicaid Eligible? ☐ Yes ☐ No **If Yes, submitted by our financial office?** ☐ Yes ☐ No
Contact person in financial office (First/MI/Last):

No Fault: Must have claim number, name of insurance company, name of insured, telephone number, and policy number. This information is needed even if no fault is exhausted. No faults must have secondary insurance information.

Case Contact

First/MI/Last: Abigail Santiago **Phone** 7023488428 **Date:** 09-01-2024